

Purpose

1. To provide person centred and safe Care to those living with dementia.

Scope

1. All workers.
2. All processes within the service.

Definition

1. Dementia is a loss of mental ability severe enough to interfere with normal activities of daily living, lasting more than six months, not present since birth, and not associated with a loss or alteration of consciousness.

Description

1. Dementia is a group of symptoms caused by gradual death of brain cells. The loss of cognitive abilities that occurs with dementia leads to impairments in memory, reasoning, planning and behaviour. While the overwhelming number of people with dementia are elderly, dementia is not an inevitable part of ageing; instead, dementia is caused by specific brain diseases. Alzheimer's disease (AD) is the most common cause, followed by vascular or multi-infarct dementia.
2. The prevalence of dementia is difficult to determine, partly because of differences in definition among different studies and partly because there is some normal decline in functional ability with age. The prevalence of dementia roughly doubles for every five years of age beginning at age 60. Dementia affects about 1% of people between ages 60 and 64, 5-8% of all people between ages 65 and 74, up to 20% of those between 75 and 84, and between 30% and 50% of those age 85 and older. About 60% of nursing home patients have dementia.
3. The cost of dementia can be considerable. While most people with dementia are retired and are not affected by income losses from their disease, the cost of Care is often enormous. Financial burdens include lost wages for family caregivers, medical supplies and drugs, and home modifications to ensure safety. The psychological cost is not as easily quantifiable but can be even more profound. The person with dementia loses control of many of the essential features of his/her life and personality, and loved ones lose a family member even as they continue to cope with the burdens of increasing dependence and unpredictability.

Policy

The service will adhere to the following principles while providing Care for people with dementia:

- »Care Planning and delivery will be person centred.
- »Care delivery will be by staff who have specialist training in dementia care, and who have access to specialist support.
- »Care delivery will focus on meeting needs and aspirations.
- »The Agency will promote dignity and respect and maintaining human rights.
- »Closely coordinated between different professionals and services across health, social Care and housing.

Procedure

Techniques and environmental changes supporting good quality dementia care:

Risk Assessment



Tools are available throughout the QCS system for personal and environmental risk assessment, and these should be used comprehensively and assiduously when providing dementia Care.

1. Within Care Planning, very detailed risk assessment should be carried out in relation to the Service User with dementia and their physical environment. In addition, risk assessment of the potential conflicts between Service Users consequent on their individual behavioural characteristics. For example, a Service User whose wandering behaviour causes them to invade the personal space, or even the belongings, of other Service Users.
1. In general, Service Users with dementia are at raised risk of:
 - » Abuse;
 - » Violent behaviour;
 - » Disruptive behaviour;
 - » Isolation;
 - » Falls;
 - » Malnutrition;
 - » Security;
 - » Tissue viability;
 - » Fast changing condition;
 - » Inability to give informed consent;
 - » Inability to participate in Care Planning; » Inability to express wishes;
 - » Communication difficulties;
 - » Depression;
 - » Accidents.

Life History

1. The use of life history (LH) research and recording is especially important in the Care of persons with dementia. A full LH record, made available to all staff providing support to the Service User, enables the carer to more fully understand behaviours of the Service User, and suggest strategies for their management.
2. The life history can be obtained from the Service User and their family. It doesn't have to be taken all at once. After you have the initial conversation, they are likely to think of more details to add. It's particularly helpful to provide a form to the family members to take away and complete, that way they can provide thoughtful responses. The information sought after should assist in conversation with the Service User in the months and years to come as well as assist in facilitating activities.
 - » Friends and Family - It is important to record the names and details of the Service Users grandparents, parents, brothers and sisters, cousins, children, grandchildren, great grandchildren, nieces and nephews, etc. Focus on details of each that will bring back happy memories for the Service User. Record the ages of children and grandchildren.
 - » School and Education.
 - » Religious activities.
 - » Hobbies.
 - » Working Life.

1. The QCS Care Plan has a Life History section.

Memory Box

2. Accompanying the life history it is useful to ask the family to put together a personal memory box for the individual. The box could contain photos, newspaper cuttings, books, ornaments. If the person was a gardener they could include favourite tools (if not a safety risk). If they were a chef or cooking fanatic they could include cooking utensils, etc. Memories relating to brothers and sisters are also very important. It is often the unexpected that brings pleasure and peace to people who's memories are fading.

ABC Charting

3. This is a technique that is widely used. The approach can be defined as:



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» “**A**” - What are the antecedents or triggers of the challenging behaviour? The idea is that all behaviours are triggered by something; they are not random. The trigger could be an environmental issue (too hot, too cold, too noisy?), an unmet need (want the WC, hungry, thirsty?) or a disease (pain, headache, not-well feeling?)

» “**B**” - Is the challenging behaviour which is causing the problem.

» “**C**” - Is the consequence of the behaviour, or the reaction of those affected by the behaviour. The way that we react to challenging behaviours can have a large impact on whether that behaviour is more or less likely to re-occur.

» “**D**” - For De-escalate and Decide. Once the situation has been diffused and calmed down, decide what you can do to prevent a similar situation in the future.

Communication

4. Communication techniques are important to the successful outcome of the process. Some tips on effective (and non-effective) communication techniques follow.

Communication Do's:

5. Position yourself to maintain eye contact and be at the person's eye level or lower;
6. Look at the person directly and make sure that you have their attention before you speak. Always identify yourself first and tell them what you are intending to do;
7. Ensure that the tone of voice used is one which conveys respect and dignity. Think about how you communicate, don't just react;
8. Use visual clues wherever possible;
9. Make sure your expectations of them are realistic. For instance, ask for only one action at a time;
10. Watch the person's body language and non-verbal communication and try to interpret it;
11. Use a calm and reassuring tone of voice and wherever possible, paraphrase what you just said;
12. Always speak slowly, but not patronisingly so, and enunciate your words clearly. If the person is hearing-impaired, manage your communication to overcome or alleviate that;
13. Talk about things which are familiar to the person;
14. Use touch if that is appropriate.

Communication Don'ts:

1. Don't talk to the person as if they were a child or use baby talk;
2. Don't use complex words or phrases or long sentences;
3. Don't glare at the person you are speaking to or otherwise visually challenge them;
4. Don't try to compete with a distracting environment; change the environment or move;
5. Don't start to speak without having first said who you are;
6. Don't break eye contact while speaking, for instance by going off and doing something in the room while carrying on speaking;
7. Don't cause more confusion and confrontation by asking for unrealistic things, such as asking the person to do more than one thing;
8. Don't ignore your own body language – be aware of it and manage it positively;
9. Don't ramble – keep to the point;
10. Don't interrupt the person unless it is absolutely necessary;

11. Don't attempt to touch the person, or invade their personal space if they are showing any fear or aggression.

Some techniques for communicating with a person with dementia:

1. Ensure that you communicate only in a quiet place that is free from distraction;
2. Be aware of the person's language and culture (a consequence of good Care Planning and Life History recording) and take these into account in your communication behaviour;
3. Be aware of the person's perception capability, attention span, intellectual level and degree of understanding (again a consequence of good Care Planning), and take that into account when communicating. Stay within the person's perception and understanding boundaries;
4. Ensure that the communication is open and conveys respect and trust. Patronising speech or talking to the person with dementia in a child-like way may either foster a sense of helplessness and dependency or trigger an angry and defensive response;
5. Pause often, making sure that the person has an opportunity to respond;
6. Make sure that sensory aids (hearing aids, spectacles) are appropriately utilised and sensory impairments (wax, cataracts) are treated.

How to make sure you are heard and seen:

7. Check that their hearing aid is on and working (if applicable);
8. Stand in front of the person where they can see you;
9. Face the person directly so they can see your facial expression and mouth;
10. Place yourself at eye level or lower when talking or listening;
11. Identify yourself by name;
12. Use the person's name.

How to make contact with the person:

1. Keep yourself and those around you calm and relaxed;
2. Touch the person gently, if they like to be touched (Care Planning again);
3. Smile and use humour.

How to make communication easy to understand:

1. Wherever you can use gestures, pictures and/or signs to explain or express things;
2. Always avoid talking over/across/about the person;
3. Always speak gently and clearly at an even pace - avoid shouting;
4. Always ask just one question at a time;
5. Always use specific names of people and places instead of pronouns: e.g.; Jim, our neighbour, or Sally, our dog, not "him" or "it";
6. Wherever you can, use a statement rather than ask a question;
7. Always wait for a response after you speak;
8. Always explain what you are going to do and what you are doing;

9. Always repeat or rephrase your message if there is no response.

Some other ideas:

1. Always allow for the time a damaged brain takes to process messages;
2. Always show your concern with reassurance and acceptance;
3. Always give praise when it's appropriate;
4. Always respond to the feelings expressed by the person;
5. When talking in a group, place the person so that the conversation is around them and they won't feel 'left out';
6. Make it easy to join in conversation by asking questions that only need a 'yes' or 'no' answer;
7. Always avoid arguments over mistaken ideas: e.g.; If the person insists they have seen a TV program a million times before even though it is a first run say: "Oh well, I don't think I've seen it before. It's interesting isn't it?";
8. Remember that touching enhances feeling of security, especially if the person is upset. Unless they respond aggressively.

Specialist Support

1. The Care Planning process should pay particular attention to the identification and involvement of external specialists, such as the CPN, in the process in order to incorporate their specialist knowledge into the service delivery.

Challenging Behaviour or Distressed Reaction

2. Ensure that challenging behaviour or distressed reaction is recorded, and reviewed as a part of the Care Planning cycle. Use the recording chart in the QCS Challenging Behaviour Policy and Procedure.
3. A checklist can be useful in helping you step through the process of changing challenging behaviour.

Reviewed by Marcus Kerridge McColl
November 2025

Depression Policy and Procedure
Crescent Community Care Services Limited

Purpose

To provide individualised safe and supportive care for people with a diagnosis of depression or people who may be depressed.

Scope

All workers working with Service Users who have been given a diagnosis of depression or who may be depressed.

Definition

1. Depression is a term in very common usage. It is a very common experience, for some lasting a short time but for others much longer. Because it is such an everyday term we need to be aware that 'something getting you down' can be a long way from the experience of living with depression. The diagnostic definition of depression includes lowness of mood and lack of pleasure, and there is a wide range of severity of symptoms of depression. The psychiatric classification system, Diagnostic and Statistical Manual of Mental Disorder volume four (DSM-IV) is widely used in mental health services in the United Kingdom, and defines depression as follows:
 - » Persistent sadness or low mood and/or;
 - » Marked loss of interests or pleasure;
 - » At least one of these, most days, most of the time for at least 2 weeks.



2. And then categorises severity as ranging through the following states depending on the number of symptoms and their impact:

- » Mild depression;
- » Moderate depression;
- » Severe depression.

3. Diagnosis of depression is undertaken by medical practitioners. Care and support staff may be working with someone who they feel is depressed. Consideration should be given to referring that person to their General Practitioner. Signs and symptoms that care and support staff should be alert to include the following:

1. **Social**

- » Withdrawing from friends and family;
- » Losing interest in things they used to enjoy;
- » Not looking after themselves properly.

2. **Physical Impact**

- » Losing appetite, or eating more than usual; §
- » Not sleeping properly;
- » Feeling anxious.

3. **Psychological**

- » Losing confidence in themselves;
- » Lacking concentration;
- » Expressing thoughts of worthlessness, and even suicide.

Description

1. As with many psychiatric diagnoses, the term depression can be a controversial one, and its interpretation can differ depending on the person's cultural background. Diagnosis of depression is more widespread in Western societies. However it is universally accepted that people can experience great feelings of sadness that can be debilitating and lead to loss of normal functioning. There are different views about what treatments or interventions work best for someone who is depressed. Depression is usually characterised by a number of factors or symptoms, which include social impact, physical impact and psychological impact or a mix of all of these. There are different types of depression which are usually more specific in their cause, such as post-natal depression, seasonal affective disorder, and bipolar affective disorder. Other people experiencing symptoms of depression may well be experiencing grief and loss.
2. There can be some obvious explanations as to why someone becomes depressed, perhaps due to distressing circumstances in their life, or perhaps there are no obvious explanations, and the causes may then be thought to be biological or genetic. The incidence of depression is very high. At least one person in five will experience depression at some point in their life. It appears to be more common in women than men. This may be because women are more willing to report symptoms of depression to their doctor.

Policy

1. There are a number of good practice principles that care and support staff should be aware of whilst providing care and support for people with depression:
 - » Being aware that a diagnosis of depression can be stigmatising, and that people may not want to admit to this;
 - » Being able to identify other sources of support including self-help groups, support groups and other local and national resources;
 - » Awareness of the principles of the Mental Capacity Act 2005. QCS policies on the MCA will provide further information when working with people who lack mental capacity to make decisions about their care;
 - » Being aware that there are different types of depression and recovery from depression is possible.

Procedure



2. The National Institute for Health and Clinical Excellence (NICE) has produced guidelines for treating and managing depression. It covers psychological treatments, medication and the services that should be made available in hospital and in the community. These guidelines are downloadable from the NICE website at www.nice.org.uk. For the majority of people who are diagnosed with depression those interventions will be delivered by their GP and primary care team. For people with the severest of depressions those interventions are likely to be carried out by specialist mental health services. However there is still a crucial role for staff working in care home and community settings in supporting these interventions. The interventions that are likely to be offered will fall under one or more of these categories:
3. **Self-Help**
 - § » Self-help groups have provided a lot of support for people with depression, and self-help group users report that shared experiences have helped them develop coping strategies. GP surgeries are a usual source for further information on local groups.
4. **Talking Treatments**
 - » These might include counselling and Cognitive Behavioural Therapy (CBT) for depression. These would be arranged via someone's General Practitioner, and in England may be provided by the NHS Improving Access to Psychological Therapies programme.
5. **Anti-depressant Tablets**
 - » There are different types of anti-depressant medication. Some of these have side effects and may take some time before they help with symptoms. The person's GP would advise on the nature of anti-depressant medication.
1. People with the severest depressions may need urgent intervention, particularly where someone is expressing suicidal intentions and in such situations the involvement of Crisis Resolution and Home Treatment (CRHT) teams may be required and access to crisis intervention can be made through referral by the person's GP.

Frameworks for Delivering Care and Support

1. All treatments and interventions should be offered as part of a care planning process based on an individual's needs. Tools in the QCS Care Management System allow for compilation of assessment of mental health needs and a care plan format to that should be reviewed on a regular basis. The majority of people receiving treatment for depression will receive this from their General Practitioner. For those with severest depression who receive care and treatment from mental health services this should be delivered as part of the Care Programme Approach framework for delivering mental health care. The support and interventions by care staff should be included in the overall Care Programme Approach care plan, and these staff may be invited to contribute to CPA care planning meetings. The person who is the subject of the CPA care plan should have their own copy. Where care staff are aware that someone is in receipt of professional mental health services they should be aware of how the care plan can be accessed, and who the person's care co-ordinator is. If not they should contact the appropriate mental health team. The contents of a CPA care plan should include:
 - » Details of who the care co-ordinator is (mental health professional);
 - » Identify the interventions and anticipated outcomes;
 - » The date of the next planned review;
 - » Crisis and contingency arrangements;
 - » Arrangements for care and support;
 - » Arrangements for the management of risk to the Service User and to others.
2. Crisis and contingency planning would be conducted by the person's care co-ordinator, but care staff should be aware of these plans. A crisis plan should identify responses to crisis situations that might arise for that person such as a worsening of their depression that cannot be managed with usual coping strategies. A contingency plan will outline a planned course of action that should follow some breakdown or failure in the person's care plan. Contact names and numbers of key professionals should form part of these plans, but a list of names and numbers on their own would not be regarded as sufficient. For crisis and contingency plans to be effective and workable, the Service User should be involved in drawing up their own plan when they are experiencing a period of wellness.

Risk Management

1. At certain times, particularly at times of crisis, people with depression may experience risk factors that might expose them to:
 - » Risk of harm to themselves, including self-neglect during periods of depression, actual self-harm, or suicide;
 - » Risk of harm and abuse by others, particularly financial or sexual exploitation as a result of feelings of worthlessness.
2. If you believe someone you are working with is at risk of these you should share that information with others you work with, and where appropriate, the person's GP. Assessment and management of people at risk is a specialist area of work that should be undertaken by health professionals, but care staff and the person themselves should be able to contribute to that. Risk management should not be seen as elimination of risk. There are risks for everyone in all walks of life, and to try and eliminate them would result in a loss of independence and choice. So risk management should be seen in the context of positive risk taking. Current policy thinking in relating to people with mental health problems can be found in the Department of Health's Best Practice in Managing Risk (2007). The Best Practice document also describes the importance of a collaborative approach to managing risk, involving the person and the whole of the care team, so that trusting relationships are developed that aid communication.

Importance of Good Communication

3. Good communication is critical in offering care and support to someone with depression. As with all communication this should be based on principles of effective communication:
 - » Good communication – providing any information in a clear and helpful way, including written and verbal information;
 - » Acknowledging the importance of developing a trusting relationship with people you work with;
 - » Discussion with the person will be conducted in a way that respects confidentiality, privacy and respect but being clear when you might need to share information. You should respect the person's confidentiality but principles of risk management may call on you to share information with other members of the care team;
 - » Being aware that the symptoms of depression may result in barriers to effective communication. Here are some examples:
4. During periods of low mood the person may not respond or be very slow to respond;
5. The person's concentration may be very limited.
6. Consider specialist support where there are additional barriers to communication.

Other Support

7. There are other ways of working that care staff and support workers can employ in offering support to someone with depression:
 - » Encouraging the person to talk about how they feel;
 - » Where possible, offer opportunities for physical exercise or activity;
 - » Offering a healthy diet of food and drink. Be aware that alcohol makes depression worse;
 - » Encouraging the person to write down what is troubling them and whether they can tackle it;
 - » Maintain hope. There are many people who have had depression and emerged from it strongly;
 - » Being alert to changes in patterns of behaviour and thinking, such as the person becoming more withdrawn, and discussing these changes with the person;
 - » Encouraging the person to be alert to their own mood, by writing things down they think and experience and note any changes.

Legal Frameworks

8. The framework for the compulsory care and treatment of people with mental disorder is governed by the Mental Health Act 1983 (which was amended in 2007).
9. Where a person lacks capacity as a result of an impairment of, or a disturbance in the functioning of, the mind or brain, to make decisions, the principles involved in making decisions for someone in their best interests are governed by the Mental Capacity Act 2005. The Mental Capacity Act 2005 also includes provision for the appointment of lasting



powers of attorney regarding financial and welfare decisions if the person should lose the mental capacity to make these decisions.

10. Policy and procedures surrounding the Mental Health Act 1983 and the Mental Capacity Act 2005 are contained within the QCS manuals.

Teamworking

1. Working with people with depression is most effective when undertaken as a team approach, involving the person, their family and friends, primary care services, mental health professionals and paid carers and support workers.
2. There are a number of principles in effective team working that should be recognised:
 - » Case recording, particularly of important decisions;
 - » Staff support and supervision;
 - » Communication and information sharing;
 - » Learning opportunities with members of the wider care team;
 - » Awareness of policies and procedures.
1. Other sections of the QCS manuals give further guidance on all of these. All of these may be identified by care staff as development and training needs.

Reviewed Marcus Kerridge-McColl
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Development Appraisal Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To assist employees in performing their jobs to the best of their abilities, maximising their levels of job satisfaction and their contributions to the organisation's objectives.
2. To identify individual employee's training and development needs required to meet the organisation's goals.
3. To highlight the potential of each individual employee to develop within their current position, or into another.
4. To ensure that employees are aware of the contribution they make to the achievement of the objectives of the organisation.

Scope

1. All employees.

Policy

1. Each employee will be appraised annually, and an exchange of views will take place between supervisors and their immediate subordinates.
2. The summary of this interview and the attached Action Plan is intended to be a fair representation of the discussion, and is referred to as a working document throughout the year.
3. Personal appraisal documentation must be stored in the individual employee's personnel file, and subject to the normal security and privacy measures applying to that file.

Procedure

1. A clear difference will be established between supervision and appraisals:
1. Supervisions are backward looking; the assumption is made that the employer has competently appointed the person being supervised, and they have the skills to do their job. Therefore the supervision interview is about how well, or otherwise, the person has used their skills in achieving the standards required, and identifying where they need to lift their performance and use the skills they have. Supervision is clearly identified as a line employee exerting



their authority and responsibility to maintain their subordinate's standards of performance. While supervision is primarily intended to identify performance shortcomings, this should be used as an opportunity to discuss support requirements needed to improve performance. Supervision should also be used to provide positive feedback where performance is adequate or exceeding the required standard.

2. Appraisals are forward looking; they are about stating what the business objectives are for the next year, as set out in the Business Plan, and identifying and clarifying the appraisee's part in achieving those objectives. Following on from that, the discussion focuses around what new skills, capabilities or resources the appraisee needs in order to meet the personal objectives which have just been agreed. If reference is made to past performance, it is only done when it is required to identify the current level of the appraisee's performance. This would be done in order to decide what development is required to move to the level of performance required by the new objectives. Appraisals are more collaborative than supervisions, the intention being for the appraiser and appraisee to co-operate on deciding how they are jointly to achieve the future objectives of their mutual employer.
1. Appraisal interviews will take place on an annual basis during a two month period which is announced well in advance. The scheduled appraisal period will follow shortly after the finalisation and publication of the annual business plan review.
2. The organisation will ensure that the business plan is available before the schedule begins, that those to be appraised have received at least that portion of the plan which relates to them, and that each employee's contribution to the achievement of that plan has been identified.
3. Appraisal interviews should be carried out by the appraisee's immediate line employee on a one-to-one basis.
4. Training or coaching will be provided to all appraisers prior to conducting their first appraisal interview.
5. New employees will be appraised in their sixth month of employment, and thereafter on an annual basis that coincides with the overall schedule.
6. Prior to appraisal, appraisers should inform the appraisee that their interview is due, and give them the appraisal preparation form together with the current Job Description and (if any) task list. The appraisee should be shown the appraisal form to ensure that they are aware of the types of questions they will be asked and can prepare for them. The appraiser will gather appropriate information relevant to the performance review, such as:
 1. Supervision records;
 2. Line manager's, or other manager's opinions;
 3. Training records;
 4. Discreet enquiries to Service Users;
 5. Discreet enquiries to co-workers.
1. By "discreet" is meant that such information gathering must be carried out in a manner which does not undermine the authority or dignity of the person being appraised, and does not give rise to a general discussion of their merits and demerits.
1. The appraisal interviews will take place from the most senior person in the organisation downwards in order to ensure that the communication of corporate objectives is made more effectively.
2. The appraisal interview should take place in private, in comfortable surroundings with no distractions, not overlooked by other people, and arrangements made to have no interruptions.
3. As the appraisal proceeds, any matters requiring action must be listed on the Action Plan located at the end of this pack, noting all agreed actions, together with agreed resources and target dates.
4. Appraisal forms will be completed by the appraiser shortly after the interview takes place, with a short follow-up meeting to review, agree and sign the form. A copy of the completed appraisal form should be given to the appraisee.
5. The completed appraisal form and Action Plan must be viewed as working documents and as such be continually referred to and reviewed throughout the year.
6. Any items on the Action Plan requiring the actions of others, or for others to be informed, should be added to the appropriate individual's or the general Action Plan.



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7. Progress against the Action Plan should be reviewed by the appraisee and their line manager at each supervision following the appraisal, and at other performance review meetings.

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Disabled Workers Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To comply with the Equality Act 2010 and good employment practice.

Scope

2. Job applicants, employees, workers, those on work experience, ex-employees. It also includes agency staff.

Policy

3. The objective of the policy is to ensure that individuals are selected, allocated work and otherwise treated solely on the basis of their relevant aptitudes, skills and abilities.
4. The organisation recognises that it has a clear legal and moral obligation towards its employees and the community at large to ensure that people with disabilities are offered equal opportunities to enter employment and progress within the organisation.
5. In order to be protected, and for us to meet our responsibilities, you need to make us aware of any disability unless it is one that is apparent.
6. In addition to complying with the Equality Act 2010 the organisation will follow procedures designed to provide for fair consideration and selection of disabled applicants and to identify / meet (satisfy) their training and career development needs.
7. If you become disabled during your employment steps will be taken to accommodate your disability by making reasonable adjustments to your employment situation. Adjustments will be made in consultation with you and may include redeployment to another post.
8. Appropriate relevant training will be provided to facilitate a smooth transition to any new role.
9. All employees have a responsibility to *avoid* discriminating against disabled workers in any of the following ways:
 1. Directly: e.g. by not appointing someone because of their hearing
 2. Indirectly: e.g. by unnecessarily specifying a height restriction for the job
 3. Association: e.g. because a partner has a disability
 4. Perception: e.g. because an employee is thought to be disabled even though they are not »
 5. Harassment: e.g. intimidating an employee because of their disability
 6. Victimisation: e.g. treating someone adversely because they have complained in the past

Guidance to the service provider STATUTORY REQUIREMENTS

1. The law requires employers to make reasonable adjustments to ensure that people within the scope of this policy (above) are not placed at a disadvantage by reason of a disability.
2. Disability has a specific definition in the Equality Act in that a person is disabled if they have (or have had) a physical or mental impairment that has (or has had) a 'substantial' and 'long-term' negative effect on their ability to do normal daily activities.
3. Long term generally means 12 months or more.

4. However certain conditions, namely HIV infection, cancer or multiple sclerosis, are included from the point of diagnosis, whether or not they affect day-to-day activities at the time.
5. Normal day-to-day activities are ones that may involve any of the following: » mobility;
 1. doing something with hands;
 2. physical coordination;
 3. continence (controlling bladder and bowels);
 4. ability to lift, carry or move everyday objects;
 5. speech, hearing or eyesight;
 6. memory or ability to concentrate, learn or understand; or » perception of the risk of physical danger.

RECRUITMENT AND SELECTION

7. You should seek to ensure that:
 1. All recruitment advertisements, job descriptions and person specifications are drafted in such a way as to clearly reflect the essential and desirable requirements of the post.
 2. Consideration is given to making reasonable adjustments to application forms and other documentation to accommodate that needs of disabled people.
 3. Applicants have a clear opportunity to ask for any reasonable adjustments that they may need during the selection process. This will enable the organisation to proactively prepare for any potential disabled employee and to include structural and or other adjustments to its development plans.
 4. The selection process does not in any way place disabled candidates at a disadvantage.
 5. Assistive technology, such as induction loops and Braille, is available during the recruitment process.
 6. That documents relating to recruitment and selection are available in an accessible format to ensure equality of opportunity for disabled candidates.
 7. The physical environment for conducting interviews is user friendly with disabled access.
 8. You make reasonable adjustments to accommodate the needs of successful disabled applicants after all necessary recruitment, health and disability checks have been completed (with specific reference to intrinsic functions of the job) and an employment offer has been accepted.
 9. Where job roles or tasks are deemed to be suitable for people with particular disabilities, such people are given preference.
 10. When a disabled person is arriving for interview they are offered any help which may be required.
 11. The usual probationary period applies to people who have disabilities unless there a cogent reasons for making an adjustment.
 12. Where adaptation of premises may be required to enable a particular disabled person to take up employment, the matter is fully investigated with relevant and appropriate advisory services and authorities.
 13. Where advice is needed on a medical condition or on the safety implications of recruiting a person with a particular disability, the relevant advisory service is contacted for guidance.

DURING EMPLOYMENT

14. It is important that all employees show consideration towards disabled colleagues. Where special arrangements need to be made to ensure health and safety, you should consider occupational health and other advice.
15. You should ensure the employees' colleagues are briefed, as may be appropriate.
16. Training and promotion opportunities need to be made available to all, regardless of disability. You may need to monitor the application of the organisation's equal opportunity policy throughout a disabled employee's employment.
17. An extended trial period may be offered to a disabled employee on transfer or promotion where it is reasonably considered appropriate.

DISABILITIES ARISING DURING EMPLOYMENT

18. If an employee becomes incapable of carrying out his or her normal duties because of a disability, consideration must be given to reasonable adjustments to the job role, working conditions, redeployment and retraining.
19. Where appropriate, you should arrange through an Occupational Health Service for an employee's capabilities to be assessed with a view to identifying suitable alternative work for that individual.

TERMINATION OF EMPLOYMENT

1. **Capability:** where an employee's performance falls to an unacceptably low standard through deterioration in his or her disability, you need to:
 1. Explore the possibility of restructuring the job to allow continuation in the role;
 2. Consider alternative job roles/employment;
 3. Consider transfer to a "holding register" until an appropriate vacancy is available. If having considered the above options, it is apparent that the employee cannot realistically continue in employment with the organisation, a decision to dismiss should be taken only by a director of the organisation following consultation with the employee, his or her representative, the occupational health advisor/disablement resettlement officer and a senior HR advisor.
 4. Generally speaking the employer's responsibility is to make reasonable adjustments so a disabled person is not at a disadvantage. Provided reasonable adjustments have been made the normal capability procedure can, and needs to be followed.
1. **Misconduct:** When a disabled employee's conduct (for reasons unconnected with the disability) is unacceptable, the organisation's normal disciplinary procedures need to be followed.
2. **Redundancy:** In a redundancy situation the particular circumstances of disabled employees can be given particular consideration. Able bodied employees do not have a specific right to protect them from an employer discriminating in favour of an employee who has a disability.
3. **Appeals:** Disabled employees who wish to appeal against a disciplinary sanction or a dismissal decision must be required to use the organisation's normal appeals procedure.

EMPLOYEE REPRESENTATIVES

4. Employee representatives, alongside management, have responsibilities for ensuring that the organisation's policy is fairly and consistently applied.
5. The organisation's policy and procedures concerning the employment of disabled people will be reviewed periodically and discussed with other parties as appropriate.

Reviewed by Marcus Kerridge McColl
December 2025

Discipline Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. Crescent Community Care Services Ltd believes that we offer our Service Users a high standard of care, arising from the cooperation of our workforce who maintain high standards of conduct, attendance and job performance.
2. The purpose of this procedure is to ensure the safe and effective operation of the business and the fair and equal treatment of all its employees to ensure continuing high standards.
3. Crescent Community Care Services Ltd views the disciplinary procedure as a positive contribution to the success of the business for both the organisation and its employees.

Scope

4. All employees.

Policy

5. No disciplinary action will be taken against an employee until the case has been fully investigated. All stages of the procedure will be implemented without undue delay.

6. At every stage in the procedure the employee will be advised of the nature of the complaint against them and will be given the opportunity to state their case before any decisions are made.
7. The employee will have the right when the 'formal procedure' is being invoked, to be accompanied by a work colleague or a Trade Union representative (as defined in the Employment Relations Act 1999) during a disciplinary interview.
8. The employee will always be informed in writing of any disciplinary action to be taken and the reason for it, indicating the specific areas of improvement required within a specific period.
9. No employee will be dismissed for a first breach of discipline except in the case of gross misconduct when the penalty will be dismissal without notice and without payment in lieu of notice.
10. The employee will have the right to appeal and should follow the Appeal Procedure.

Procedure

1. **Employee** - *This procedure is not part of your contract of employment; however, we will not depart from it without good reason.*
2. **Employer** – *consult your legal expenses insurers before beginning this process.*
3. All formal disciplinary action will be conducted by the Registered Manager or a person nominated by them. Appeals will normally be conducted by the Registered Person. We reserve the right to suspend individuals during formal disciplinary action. Any suspension will be on a paid basis and does not mean that we have prejudged the issue.
4. Before any formal disciplinary action is taken, the relevant person will carry out a full investigation to establish the facts. The investigation will normally include a meeting with you. Investigatory meetings are not disciplinary meetings and you will not necessarily be offered the right to be accompanied.
5. Even in the most serious allegations of gross misconduct (see below), a full investigation will be held. In any alleged case of gross misconduct you are likely to be suspended pending the outcome of the investigation.
6. Before any *disciplinary* meeting, you will be:
 1. Told in writing of the allegations/complaints against you, and the basis of those allegations;
 2. Given a reasonable opportunity to consider your response to that information;
 3. Offered the opportunity to be accompanied by a work colleague or a trade union representative (see right to be accompanied, below);
 4. You must take all reasonable steps to attend the meeting. At the meeting, you will be given a full opportunity to comment on the allegations, to put forward any defence or arguments you want, and to comment on what disciplinary sanction (if any) is appropriate.

Informal Warning

1. After establishing the facts, we may consider that there is no need to resort to the formal procedure, and that it is sufficient to talk the matter over with you. A note of the informal warning may be kept on your personnel file; however, they are there for background and would not normally be taken into account in the event of subsequent disciplinary procedures.
2. The purpose of an informal warning is to provide an opportunity for improvement or for the matter to be corrected without the necessity for formal disciplinary procedures to be enacted.

Formal Disciplinary Process Right to be accompanied

1. You have the right to be accompanied at any disciplinary hearing by a single companion who is either: -
 1. A work colleague; or,
 2. A full-time official employed by a trade union; or,
 3. A lay official, so long as they have been certified in writing by their union as having experience of, or as having received training in, acting as a worker's companion at disciplinary or grievance hearings.



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4. Your representative has the right to explain and sum up your case, and to respond to any views expressed at the hearing. They may not answer questions on your behalf. If your representative cannot attend on the date we have set for the interview, we will always postpone the interview for up to five days and may (at our discretion) postpone it for longer.

Stage 1: Written Warning

1. If it is decided that your conduct or performance is unsatisfactory your manager may give you a Written Warning.
2. This will state the nature of the complaint, the required standards that must be met and, where appropriate, a time limit for improvement. It will also state that further disciplinary action will follow if the required standards are not met or if there is further misconduct.
3. You will be informed of your right of appeal, and how and where this should be made. A record of the warning and related discussions will then be placed on your personnel file. It will normally cease to have effect after 6 months.
4. If your conduct is sufficiently serious, we may omit stage 1, and proceed straight to stage 2.

Stage 2: Final Written Warning

1. For more serious matters, or where you have failed to meet the required standards after being warned, you may be given a Final Written Warning. This will state the nature of the complaint, the required standards that must be met and, where appropriate, a time limit for improvement. It will also state that you will be dismissed if the standards are not met or if there is further misconduct.
2. Again, you have the right to appeal. A record of the warning and a note of all related discussions will be placed on your personnel file. A final written warning will normally cease to have effect after 12 months.

Stage 3: Dismissal

1. Where there has been gross misconduct (in which case, stages 1, and 2 may be omitted) or where you have failed to meet the required standards after due warnings have been given, you may be dismissed. In extenuating circumstances we may apply another sanction such as disciplinary transfer, disciplinary suspension without pay or demotion.
2. This will be confirmed in writing and will include details of the appeals procedure.
3. In cases of gross misconduct, the dismissal will be without notice (or pay in lieu of notice).

Dismissal without notice ('Summary Dismissal')

1. We regard certain issues as serious enough to warrant 'summary dismissal' without prior warning. These issues would constitute gross misconduct, which is the action would break the contract between us. Matters that may justify summary dismissal, include, but are not limited to:
 1. Actions which may harm the well-being of a Service User ("abuse");
 2. Acts of dishonesty where your conduct affects your ability or suitability for continued employment with us; for example, theft, fraud, the deliberate falsification of records or expenses, a relevant criminal warning or conviction, or inclusion on the DBS register;
 3. Serious insubordination or rudeness to customers or suppliers; deliberate damage to property;
 4. A serious breach of Health & Safety policies;
 5. Physical violence or aggressive behaviour;
 6. Indecent or immoral acts;
 7. Being under the influence of, or possessing, alcohol or illegal drugs during employment hours (unless you have been authorised to, and have been entertaining clients, in which case a reasonable amount of alcohol consumption is permitted);
 8. Bringing Crescent Community Care Services Ltd into serious disrepute;
 9. Any breaches of confidentiality requirements in your contract of employment, other than minor breaches;
 10. Harassment or bullying, other than minor breaches;
 11. Breaches of our Equal Opportunities Policy, other than minor breaches;
 12. Wilful misrepresentation at the time of appointment, including:
 1. Previous positions held;

2. Qualifications held;
3. Falsification of date of birth;
4. Declaration of health;
5. Failure to disclose a criminal conviction/caution within the provision of the Rehabilitation of Offenders Act.

1. Abuse of the protected disclosure provisions;
2. Serious failure to abide by the professional code of conduct which applies to your work, a copy of which was issued to you on your engagement, a revised copy of which will have been issued to you if it has changed since your engagement, and a copy of which is also available for inspection in the office;
3. Deliberate disclosure of privileged confidential information to unauthorised people;
Negligent or deliberate failure to comply with the legal requirement for The Agency's policy & procedure concerning medicines;
4. Working whilst contravening an enactment or working in such a way that is in breach of rules laid down by statutory bodies, e.g. erasure from the register of the Nursing & Midwifery Council.

1. Except in the most serious cases of gross misconduct (see below), a full investigation will be held and, if necessary, you will be suspended pending the outcome of the investigation.
2. In the event of gross misconduct, disciplinary action could take the form of a demotion, or a downgrading, with subsequent financial loss as well as loss of status, rather than a dismissal. This decision will be taken at our discretion.

Other Disciplinary Matters

1. Matters which may justify invoking the disciplinary procedure but which do not amount to gross misconduct include, but are not limited to, the following:
 1. Refusal to obey a legitimate instruction;
 2. Refusal to answer a question during the course of a properly constituted investigation;
 3. Absence without permission, or persistent absence;
 4. Poor timekeeping;
 5. Failure to report damage to our property;
 6. Failure to carry out your duties adequately;
 7. Breach of our policies, procedures and practices;
 8. Continued poor work performance, or a persistent failure to keep up-to-date with technical developments;
 9. Improper use of our equipment;
 10. Harassment or bullying (in ways that are not deemed serious enough to constitute gross misconduct);
 11. Breaches of our equal opportunities policy (in ways that are not deemed serious enough to constitute gross misconduct)
 12. Actions bringing Crescent Community Care Ltd into disrepute (in ways that are not deemed serious enough to constitute gross misconduct).

What you can expect during the Disciplinary Process

13. At each stage of the disciplinary process:
 1. You will be told of the expected standard of performance/behaviour, and the nature of the shortfall in expected standards will be identified;
 2. You will be given the opportunity to reply to any allegations made against you, and to outline any mitigating circumstances you wish to be considered;
 3. All the facts will then be considered. Occasionally we may ask you back for a further interview;
 4. You will be told of the disciplinary sanction being imposed (although this may be done in writing). This will include details of any timescale within which improvement is to be achieved and the likely consequences if there is no improvement; and
 5. You will be advised of the right to appeal.

Right to appeal

6. If you are not satisfied with a disciplinary decision, you may appeal, in writing, within five working days. Arrangements to hear the appeal will normally be made within five working days of receiving your written request. If the decision you are appealing against was a decision to dismiss you, the appeal may be heard after the dismissal has taken place.
1. All appeals must set out the grounds on which you are making the appeal.

2. We will invite you to an appeal hearing, and remind you of your right to be accompanied.
3. The outcome of the appeal will be confirmed to you in writing and will take one of three forms:
 1. The original decision will be upheld, in which case any disciplinary sanction will be confirmed;
 2. The original decision will be overruled, in which case any disciplinary sanction will be rescinded;
 3. The original decision will be substantially confirmed but a less severe sanction will be substituted for that originally imposed (usually in cases of appeals based on extenuating circumstances).
4. There is no further right of appeal.

Guidance for the Service Provider

This guidance here is not intended to supplant the ACAS Guide, which is more extensive, but it should nevertheless assist you if you are undertaking such processes for the first time or are under-confident about what may be required. It is, however, general guidance and is not a substitute for professional advice. The latter can be obtained via QCS.

1: Recommended Procedure for Disciplinary Hearing Conducting a disciplinary hearing

5. The information here is for guidance, in order to assist in a fair hearing. It cannot guarantee fairness; that will depend on how you handle or view the information that comes forward. If, elsewhere, you have more detailed or radically different procedural guidance then it may be more appropriate to follow those procedures. Public sector employees, even if brought into a private sector organisation, may have additional rights to those indicated here.

Setting the scene

6. The person whom you have invited to a disciplinary hearing is likely to be nervous. Therefore it is important to be as empathetic as you reasonably can be.
7. Introduce the people who are present and explain the reason for their presence or role. Disciplinary hearings should not be conducted alone. Typically the chair of the meeting will have an HR Adviser with them and perhaps a note taker.
8. The employee has the right to be accompanied by a Trade Union representative or work colleague. Indeed the latter in particular should be encouraged as it provides for better balance. At the outset it is beneficial to make sure that the employee has been made aware of that right and, if it has been declined, to make a note to that effect.
9. Accompanier's have the right to address the hearing but not to answer questions on the employee's behalf.
10. Outline how you intend to conduct the hearing. The employee needs to know that they will get the opportunity to put their side of the story and at what point. Usually it is best to be firm about putting the detail of the charge(s) first and asking the employee to note any points he or she may want to challenge. They can make their points later rather than interrupt you. Make clear how you see the accompanier's role. They have the right to have the opportunity to address the meeting but not to answer on the employee's behalf.

Getting started

11. The chair should then outline the reason(s) for the hearing and make sure that the employee has received any relevant paperwork or the investigation report, as appropriate.
12. If dismissal is a potential sanction this should have been flagged up previously but it is good practice for the employee to know what the possible outcome(s) could be. Without being intimidating it is valuable to clarify these possible outcomes – which must include no action being taken at all, if that should prove appropriate.

The heart of the hearing

1. Then proceed to detail the charge(s) levelled against the employee. This may mean going through the investigation report. Alternatively it may mean going through evidence, such as performance reports.
2. Once you have completed this stage allow the employee to challenge any assertions in the allegations or the report. Accompaniers will often wish to challenge the allegations, content of the report or other purported facts or evidence.



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You should be prepared for this. It is debatable whether you can refuse but, in practice, accompaniers may assert that this is part of their right to address the hearing.

3. Apart from the opportunity to challenge the report or allegations the employee should also have the opportunity to offer any explanation for their behaviour. Indeed they should be encouraged as it is better to hear any explanation now than later in a public hearing. Blame may be assigned to others by the employee and you will have to decide whether this needs to be followed up. However, all misconduct should be fully investigated first and all being well potential claims of others to blame or “framing” will have been covered in the investigation.
4. Always be prepared to adjourn if new information comes to light that might have a bearing on the fairness of any decision.
5. In addition you should seek out any mitigating circumstances. Employees will not necessarily recognise they are relevant or may be reluctant to share them. Divorce, family illness, bereavement and other life crises can all impact on employee behaviour. If they exist then you need to know about them if possible. You don’t have to consider them as relevant (if they are not) but, again, they are best in the open now, not in a subsequent Tribunal hearing.
6. Ask the employee if he or she has anything to add or indeed whether they have a different “story”.
7. Getting everything into the open is important to sound decision making. So listening and questioning are valuable skills.
8. It is good practice to give the accompanier the opportunity to have a final say on behalf of the employee.

The next step

1. Adjourn the meeting. This is not always essential but it is good practice.
2. It is also the point where a colleague, preferably an HR Adviser, can aid the deliberations. If there is a lot of material then take time.

Communicating the decision

1. You may want to advise the employee that you will let them know the decision when it has been made rather than keeping them waiting. If it is not to be at the meeting you might want to ask them how they would prefer to be informed of the decision once it is made.
2. It may be that you will wish to seek further advice and/or reflect on the meeting before making a decision.
3. But once a decision has been taken the employee must be informed of the decision and, if the decision is adverse, informed of their right of appeal, to whom that appeal should be lodged and by what timescale.
4. Written confirmation of the decision should be sent to both parties within 5 working days of the hearing.

A note on notes

1. Notes of disciplinary hearings are essential to being able to defend a decision if it is challenged. Contemporaneous notes are ones taken at the time of the hearing. If the outcome may be serious, damaging to a person’s career or would be a dismissal, then a separate trained note-taker is advisable to make such notes. This may mean three people on one side but make clear that one is just a note-taker.
2. A compromise to having a third person is to halt the proceedings as necessary to note key points. You then have some points on which to rest a case. Other notes made immediately after the meeting can be used to flesh the contemporaneous ones out provided they are made while the hearing is still fresh in the mind. Notes written up later carry much less weight.
3. The employee need not be shown the full notes; although they could be required to be disclosed, by a subsequent court order. Showing full notes, when emotions may be running high, is best avoided and, indeed, the notes might not need to be disclosed at all.
4. It would be good practice, however, to summarise the key points of the discussion on any subsequent warning form.

2: Recommended Procedure for Appeals

Conducting an appeal hearing

1. The information here is for guidance, in order to assist in a fair appeal. It cannot guarantee fairness; that will depend on how you handle or view the information that comes forward. If, elsewhere, you have more detailed or radically different procedural guidance then it may be more appropriate to follow those procedures. Public sector employees, even if brought into a private sector organisation, may have additional rights to those indicated here.
2. Generally, the appeal will be heard by someone not involved in the preceding procedural stages, if possible, and usually a more senior manager.

Setting the scene

1. Even though the employee or ex-employee (the appellant) has instigated this meeting, they are likely to be nervous. Therefore it is important to be as empathetic as you reasonably can be.
2. Introduce the people who are present and explain the reason for their presence or role. Appeal hearings should not be conducted alone. Typically the chair of the meeting will have an HR Adviser with them and perhaps a note taker.
3. The employee has the right to be accompanied by a Trade Union representative or work colleague. Indeed the latter in particular should be encouraged as it provides for better balance. At the outset it is beneficial to make sure that the employee has been made aware of that right and, if it has been declined, to make a note to that effect.
4. Accompanier's have the right to address the hearing but not to answer questions on the employee's behalf.
5. Outline how you intend to conduct the appeal hearing. The employee needs to have the opportunity to put their reasons for appeal and provide evidence to support the reasons. They should be able to make their points without interruption. Clarify the accompanier's role. The accompanier has the right to address the meeting but not to answer questions on the employee's behalf.

Getting started

1. The chair should make sure that the employee has received any relevant paperwork, as may be appropriate.
2. It is good practice for the employee to know what the possible outcome(s) could be. Without being intimidating it is valuable to clarify these possible outcomes – which must include no action being taken at all, if that should prove appropriate.

The heart of the appeal hearing

1. The chair should then invite the employee to put forward the reasons for their appeal. They may need help and encouragement to put their case, especially if they are not accompanied. It is in your interests to give them a full hearing.
2. Once you have completed this stage you may well want to query and question the employees reasons and assertions. This should be gentle cross-examination. If there are points that the employee has failed to appreciate this may be your opportunity to help them see your viewpoint.
3. Sometimes you may feel more investigation is required and, if so, you will need to adjourn.
4. Always be prepared to adjourn if new information comes to light that might have a bearing on the fairness of any decision. An appeal should not be a re-hearing but, on occasion, that will be necessary; and especially so if the original hearing was flawed on procedure.
5. As with a disciplinary hearing, getting everything into the open is important to sound decision making. So listening and questioning are valuable skills.
6. It is good practice to give the accompanier the opportunity to have a final say on behalf of the employee.

The next step

1. Adjourn the meeting. This is not always essential but it is good practice.



2. It is also the point where a colleague, preferably an HR Adviser, can aid the deliberations. If there is a lot of material then take time.

Communicating the decision

1. You may want to advise the employee that you will let them know the decision when it has been made rather than keeping them waiting. If it is not to be at the meeting you might want to ask them how they would prefer to be informed of the decision once it is made.
2. It may be that you will wish to seek further advice and/or reflect on the meeting before making a decision.
3. But once a decision has been taken the employee must be informed of the decision and, unless otherwise is the case, informed that this is the final stage of the procedure and there is no further right of appeal.
4. Written confirmation of the decision should be sent to both parties within 5 working days of the hearing.

A note on notes

1. Notes on appeal hearings are essential to being able to defend a decision if it is challenged. Contemporaneous notes are ones taken at the time of the hearing. If the outcome of the disciplinary hearing was damaging to a person's career or was a dismissal, then a separate trained note-taker is advisable to make such notes. This may mean three people on one side but make clear that one is just a note-taker.
2. A compromise to having a third person is to halt the proceedings as necessary to note key points. You then have some points on which to rest a case. Other notes made immediately after the meeting can be used to flesh the contemporaneous ones out provided they are made while the hearing is still fresh in the mind. Notes written up later carry much less weight.
3. The employee need not be shown the full notes; although they may be required, by a subsequent court order, to be disclosed. Showing full notes, when emotions may be running high, may be best avoided and, indeed, they might not need to be disclosed at all.

3: Recommended Procedure for Investigations

What if abuse is suspected?

1. Where matters of Service User abuse are suspected then there may be a requirement to involve local social services by contacting the Adult Protection Team or Safeguarding Officer, or in certain cases, the police. Indeed there may be a condition that you do so if you receive public funding. It might be, in those cases, that the whole investigation is taken out of your hands.
2. This guidance is intended for situations which can remain under your control.

So what is an investigation and to what degree should it be conducted?

1. Essentially investigation is about establishing all facts that might be relevant to the case, so far as is reasonably possible.
2. There are circumstances where the degree of investigation can be limited, particularly if the situation is not too serious. For example, perhaps someone has accepted responsibility for a careless error but nevertheless a formal warning is justified.
3. On the other hand if a serious, careless error is not admitted, or contested, and dismissal for gross misconduct is being contemplated, then thorough investigation is essential. The facts might not be as they first appear to be. Having such an investigation carried out by an experienced person can assist in establishing fairness. In any case it is preferable that the investigating officer and any subsequent disciplinary chairperson are different people.

The prime task is to gather evidence.

1. Evidence can come in a number of forms:
 1. Witness testimony



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1. This is frequently the main source of evidence. You really want such testimony from more than one source. It is also important that you are confident that each witness is independent. If they all report to the same person this may give cause for doubt. It is not unusual for employees to claim that they have been “framed”. You don’t want the first time you hear of this to be when you are in a Tribunal. So be alert to this possibility and eliminate conspiracies if you can.
2. Employees are entitled to call witnesses to a disciplinary hearing. It may therefore be wise to speak to any witnesses that the employee intends to put forward in their defence – but NOT to intimidate them!! Be careful, so you are not accused of doing so.
3. Make detailed notes of what the witnesses tell you and challenge them and what they say. You need to flush out any doubts, conspiracy or false testimony.
4. Investigatory meeting
 1. Employees should be given advance warning of an investigatory meeting. It is up to you to decide whether allowing accompaniment would hinder or help the investigation. Be clear with the employee about the status of the meeting – it is an investigatory meeting, not a disciplinary meeting or a disciplinary hearing.
 2. The purpose of the meeting is to establish facts, or at least the facts as the employee sees them. It is not the place for defence or explanation; that may come later if a disciplinary meeting is still required.
 3. Similarly you might not wish to put other evidence that you have gathered before him or her. That can come later. Nonetheless in some cases doing so can be helpful. It is a matter of judgement in each set of circumstances.
4. Signed statements
 1. If signed statements are presented to an employee then that may reduce the likelihood of him or her calling witnesses to a disciplinary meeting and thus simplify the process.
1. Unsigned statements
 2. Sometimes there is the fear, real or imagined, of the witnesses being subject to harassment, intimidation or violence from the employee against whom they are testifying. It is crucial that the disciplining officer also talks to the witnesses and challenges their statements, as discussed above. If it comes to a Tribunal then it will be the disciplining (or dismissing) officer who has to stand up in court and justify his or her decision.
3. Other evidence
 4. Photographs and even Smartphone videos can provide corroborative evidence.
 5. CCTV footage (including targeted footage) may also be used as corroborative evidence but blanket CCTV coverage must not be used as a means of monitoring employees. That could infringe human rights.
 6. Tests of sobriety are also sometimes valuable though obviously they need to be used at the immediate time. For example, can the apparently drunk employee read a simple passage of writing?

Was it the employee’s responsibility and were they trained for that responsibility?

1. An employee will often assert that a particular failing was not their responsibility and that therefore they should not carry the can.
2. An organisation chart, job description and training records all help to establish who is responsible for what and to justify a decision to dismiss if appropriate.

Have actions, that have been taken already, suggest how serious the misdemeanour might be?

1. If first line management treats the situation as minor then this can undermine the whole case. If gross misconduct is suspected then this should normally lead to suspension pending investigation. Such suspension should be “neutral”, paid and appropriate. If in doubt, seek advice.

Did the employee “know the rules”?

2. This is a secondary, but nevertheless important, point.

3. A starting point is the written particulars or written contract of employment. It is important to show the employee has received these. It is better still if he or she has signed them to say they have been read and understood.
4. Disciplinary rules are also important. These should be readily accessible to the employee. An actual copy in the employee's hand is not essential but it is important that the employee knows they exist and where they can be accessed.
5. A record of attendance at an induction can also be valuable; especially if disciplinary rules are covered at that induction.
6. Previous warnings, even if they are expired, can also underline the fact that the employee knew the rules.
7. Investigating officers should produce a report which the employee can see prior to the hearing. This will form an important part of the disciplinary hearing as will any defence or counter arguments presented by the employee.

Reviewed by Marcus Kerridge McColl

December 2025

Diversity and Equality Policy and Procedure
Crescent Community Care Services Limited

Purpose

- The purpose of the Equality and Diversity Policy and Procedure is to ensure that the principles of the Equality Act 2010 are understood and embedded into practice within Crescent Community Care Services Limited. In addition the policy will promote and formalise how the Crescent Community Care Services Limited will ensure that our practice treats people fairly, and will ensure that dignity and respect is at the heart of what we do.

The Aims of the Equality and Diversity Policy are:

- To encourage, promote and celebrate diversity in all our activities and services;
- To ensure equal access to jobs and volunteer opportunities;
- To create environments free from harassment and discrimination;
- To confront and challenge discrimination where and whenever it arises, whether it is between colleagues, or in any other area relating to Crescent Community Care Services Limited's work;
- To ensure acceptance and implementation of this policy is mandatory for all positions in Crescent Community Care Services Limited;
- To ensure, through positive action and as far as is practicable, that all Crescent Community Care Services Limited's premises and services are accessible to all;
- To ensure that employment and advancement within the organisation is not restricted because of any protected characteristics or for any other discriminatory reason.

Scope

- All employees, job applicants, volunteers, trustees, members and Service Users.

See next page for contents.

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Policy

Statement

- Crescent Community Care Services Limited strives for high standards both as an employer and as a provider of services. We recognise the need to promote diversity and wholeheartedly support a policy of equal opportunities and uphold human rights in all areas of our work and responsibilities.
- This policy will define and formalise our approach to Equality and Diversity and will recognise and publicise our commitment and our responsibilities under the Equality Act 2010.
- We will address these principles in all areas and particularly the following:
 - Communicating our expectations;
 - Recruitment and promotion;
 - Interviews and selection;
 - Training, including induction training;
 - Benefits and terms and conditions of employment;
 - Service User delivery;
 - Furthermore, the organisation will monitor the composition of its workforce and Service User base and take appropriate action if it appears that this policy is not fully effective in all areas of its operation.
- This policy provides clear guidelines for all who work with or for Crescent Community Care Services Limited to understand their responsibilities with regard to Equality and Diversity.
- Failure to follow the policy and associated procedures will lead to disciplinary or other appropriate action.
- Crescent Community Care Services Limited's aims and objectives as set out in this policy, will be achieved through training, effective monitoring, and a willingness to promote Equality and Diversity, addressing issues where they arise.



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- Our training, publications, interaction with stakeholders, and other activities, Crescent Community Care Services Limited will ensure that those we work with know our statements of policy and understand our commitment to promote Equality and Diversity.
- Crescent Community Care Services Limited will review, on an annual basis the content and implementation of its Equality and Diversity Policy and strategy. Where evidence is found of ineffectiveness, lessons will be learnt and action taken to ensure improvements are made.
- Definition of protected characteristics and what we understand by discrimination.
- The Equality Act 2010 makes it unlawful to discriminate against people with a 'protected characteristic'. These are:
 - Race, including colour, nationality, ethnic or national origin;
 - Sex;
 - Pregnancy or Maternity;
 - Disability, including physical, sensory, HIV, cancer, mental health or learning disability;
 - Age;
 - Religion or belief;
 - Sexual orientation;
 - Gender re-assignment;
 - Married or civil partnership status.
- In addition, if Crescent Community Care Services Limited provides services for Service User that are commissioned by the Local Authority or other Public Body there is a requirement under the Equality Act 2010 that there is an additional 'Public Sector Equality Duty'. This means that {Client_Name_Official} must foster good relations between people with protected characteristics and those that do not, have due regard to eliminate discriminatory practice, promote Equality and eliminate unlawful discrimination. The Crescent Community Care Services Limited will ensure these duties are met.
- Crescent Community Care Services Limited understand Discrimination can take place in a number of ways:
 - Directly; e.g. by not appointing someone because of their skin colour;
 - Indirectly; e.g. by requiring all male staff to be clean shaven (which discriminates against some beliefs);
 - Association; e.g. because an employee's partner has a re-assigned gender;
 - Perception; e.g. because an individual is thought to be disabled when not;
 - Harassment; e.g. Causing distress to a person on the basis of their religion;
 - Victimisation; e.g. treating adversely someone who has complained in the past, such as a whistleblower;
- Crescent Community Care Services Limited's Equal Opportunities ensure that policies, procedures and practice within Crescent Community Care Services Limited do not discriminate against the people within it and those who come into contact with it. It is about treating people fairly and equally.
- Diversity means that all people are valued as individuals and are able to maximise their potential and contribution to Crescent Community Care Services Limited and to the community. It recognises that people from different backgrounds, can bring fresh ideas, and a different approach to the services we provide.
- Positive Action refers to measures taken in order to assist employees or learners who have been under-represented in specific areas, to reach a level of workplace knowledge and competencies that is comparable with 'representative' employees.
- We recognise that part-time workers and workers on zero hours contracts are to be treated no less favourably than full time workers.



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- We also recognise that agency workers acquire basic rights after 12 weeks employment, meaning they are entitled to comparable terms and conditions as comparable “permanent” workers, and the same rights relating to unfair dismissal and redundancy.
- Where employees are being recruited to or promoted to any position that is exempt from the Rehabilitation of Offenders Act we may ask the following question of applicants:
 - “Do you have any convictions, cautions, reprimands or final warnings that are not protected as defined by the Rehabilitation of Offenders Act 1974 (Exceptions) Order 1975 (as amended in 2013)?”
- Applicants to exempted positions will be subject to a Disclosure and Barring Service (DBS) check. Risk assessments will be undertaken where relevant information is revealed either at interview or from a subsequent check.
- Crescent Community Care Services Limited recognise that it is unlawful to discriminate against anyone because they are either a member of a trade union or religious group.

Implementation

Expectations

- The Registered Provider has responsibility for implementing and developing the policy. The overall coordinating responsibility for equal opportunities and management of diversity is delegated to the Registered Manager (Mrs Louise Gould).
- However, Crescent Community Care Services Limited believes that all who work with or for the organisation have an individual responsibility to understand and use the policy and ensure a personal involvement in its application and to promote Equality and Diversity.
- Therefore, Crescent Community Care Services Limited expects individuals:
 - To have formally recorded that they have read and understood the policy.
 - To have undertaken training in Equality and Diversity and to understand their individual responsibilities as defined by this policy and the Equality Act 2010.
 - To cooperate with measures introduced by Crescent Community Care Services Limited to ensure equality of opportunity, diversity, non-discrimination and positive reinforcement.
 - Not to harass, abuse or intimidate any other employee, job applicant, volunteer, trustee, member, Service User, contractor/agency, stakeholder or participant in relation to any of the characteristics described in the Definition section above or for any reason whatsoever.
 - To inform the Registered Manager if they suspect discrimination is taking place in any shape or form. Or if they suspect the Registered Manager of discrimination in any shape or form they should inform the line manager of the Registered Manager.
 - To raise matters through the grievance procedure if they are the subject of discrimination.
 - To follow the Whistleblowing Policy and Procedure if they have a reasonable belief that the matter relates to (and only if it relates to) the public interest No employee or other person covered by the scope of this policy will suffer detriment where matters are raised in good faith or reasonable belief.
- Crescent Community Care Services Limited expects Line Managers:
 - To ensure that proper records of employment decisions are maintained that are consistent with this policy and regular reviews of employment practices are carried out.
 - To ensure that grievances are dealt with in a fair and consistent manner and in line with our Grievance Policy and Procedure.
 - To ensure that within their area of responsibility individuals are aware of their legal obligations under the Equality Act 2010, and of the organisation’s Equality and Diversity Policy and Procedure.
 - To ensure that the highest standards of Equality of Opportunities practice are observed in the delivery of Crescent Community Care Services Limited services and to undertake training and development opportunities to ensure that their competence is maintained at all times.



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- To provide or arrange training for staff that they are directly responsible for in Equality and Diversity and to monitor the understanding of staff, and if shortfalls in understanding are identified additional training to be provided.
- The person with responsibility for Equality and Diversity will:
 - Actively promote the benefits of employee and participant diversity, in employment, services, training and all other activities associated with the Crescent Community Care Services Limited.
 - Maintain a full awareness and understanding of the legal framework for Equalities and Diversity and ensure that all staff are aware of the implication of any changes on policies, procedures and practice.
 - Seek the views and opinions of employees, volunteers, customers and clients on the operation of the policy in his/her locality/area of responsibility, in particular to meet the diverse needs of the users of the service.
 - Offer advice and guidance to members of staff, volunteers and organisations in Crescent Community Care Services Limited's equality and diversity policy and procedures.
 - Ensure that Managers are supported in their roles with regard to the Equality and Diversity Policy and Procedure.
 - Ensure that Managers, including Registered Managers, are appraised regularly on the understanding and implementation of equal opportunities and diversity within Crescent Community Care Services Limited and the public domain.
 - Ensure that the Equality and Diversity Policy and associated documents are reviewed on an annual basis and any amendments or additions are cascaded to all employees, agencies and those who have a business relationship with Crescent Community Care Services Limited.
 - Review and approve policies, procedures and practices that impact on equal opportunities and diversity in practice.
 - Coordinate the delivery of an equality and diversity strategy and action plan to be monitored and reviewed on a regular basis.
 - Facilitate training and open discussion on equal opportunities and diversity issues as appropriate.
 - Ensure reasonable adjustments are made to working practices, equipment and premises and offer, where appropriate, additional support to trustees, staff and volunteers to ensure that they are able to play a full and active part in Crescent Community Care Services Limited's work.
 - Where new processes are planned, ensure that an Equality Impact Assessment is carried out to establish if any discrimination is likely to occur as a result of the change, and to plan any remedial actions.

Recruitment and Publicity

- Crescent Community Care Services Limited strives to ensure that all our employees, job applicants, volunteers, trustees, members and Service Users reflect the wider community.
- We will provide accessible and accurate information on vacant posts through advertisement, covering job descriptions, person specifications and interview arrangements. Wherever appropriate, vacancies will be advertised sufficiently to reach the widest possible range of candidates, either internal and/or external with positive action used where needed.
- Applicants will be informed through all recruitment material of Crescent Community Care Services Limited's commitment to equal opportunities and diversity and the existence of this policy.
- Care will be taken to ensure that 'essential' and 'desirable' requirements in Person Specifications are not discriminatory.
- Job titles will be non-discriminatory.

Interviews and Selection

- So far as reasonably possible, short-listing and interview panels will reflect the diverse makeup of Crescent Community Care Services Limited.
- Staff involved in recruitment will have appropriate equality and diversity training to ensure that they understand the implications for recruitment.

Training

- In line with this policy, Crescent Community Care Services Limited will not discriminate in the provision of training courses, promotion, mentoring, secondment or other opportunities.
- Appropriate training will be provided to enable staff and volunteers to perform their jobs effectively. The training offered will take into account the needs of all people and their 'protected characteristics'.
- Briefing on this policy will form part of the first day Induction Procedure for all staff, including senior staff and volunteers.
- Staff and volunteers are encouraged to discuss their development and training needs, through a process of regular support and annual appraisals, to include an annual skills audit of employees.

Services Users

- Crescent Community Care Services Limited will endeavour to deliver services in a way that genuinely recognise the importance of an inclusive society that brings opportunities and access, not barriers and disempowerment, to individuals.
- Crescent Community Care Services Limited will strive to ensure that our Service Users' reflect the community which Crescent Community Care Services Limited serves.
- The Crescent Community Care Services Limited contract for the delivery of the service will ensure Service Users are aware of their responsibilities to comply with the equality and diversity policy.
- All advertising or information directed to Crescent Community Care Services Limited will not imply any preferred group, unless a genuine qualification exists limiting a vacancy, such as a condition of registration or the specialism of the service or Service User Guide.
- All promotional material and information of Crescent Community Care Services Limited's will display text relating to Crescent Community Care Services Limited's commitment to equality and diversity.
- Guidelines for admission are not discriminatory and are in line with the Equality and Diversity Policy and Procedure.

Grievances

- Any staff member or volunteer who feels that they have been a victim of unlawful discrimination or unfairly treated in a way contrary to the intention of this policy should raise the issue through Crescent Community Care Services Limited's established Grievance Policy and Procedure.
- Any Service User who feels that they have been unfairly treated in a way contrary to the intention of this policy should make a complaint through Crescent Community Care Services Limited's Registered Manager, who must report any such complaint to the Registered Provider. If the complaint is about the Registered Manager, this should be made directly to the Registered Provider.
- All incidents of discrimination by Service Users should be raised with the Registered Manager, and in the event of a failure to agree satisfactory remedies, will be dealt with under the terms of the Service User agreement/contract.
- Any job applicant who believes that they have been treated unfairly and contrary to the intention of this policy should raise the issue with the Registered Manager or the Registered Provider.

Breach of Policy

- Where it appears that there may have been, or there is, a breach of the policy, the Manager will investigate the circumstances and action will be taken.
- If the breach involves the Registered Manager, the Registered Provider will carry out the investigation. This may be a delegated responsibility (delete as appropriate).
- Any member of staff found to be in breach of this policy will be subject to disciplinary action.
- Any volunteer found to be in breach of this policy will be counselled on their actions and may, where necessary, be removed from Crescent Community Care Services Limited's volunteer register.
- Any Service User found in breach of this policy will, where appropriate, be counselled on their actions and may, where

necessary, be refused future services from Crescent Community Care Services Limited.

Monitoring

- Crescent Community Care Services Limited recognises the need for a continuing commitment to genuine equal opportunities and diversity within the organisation. The effectiveness of the policy's aims and objectives can only be judged by how the policy operates in practice.
- If the Crescent Community Care Services Limited provides services for Service User that are commissioned by the Local authority or other public body there is an expectation under the Equality Act 2010 'Public Sector Equality Duty' {Client_Name_Official} should be transparent and share information about how well Equality is being promoted.
- As such the {Client_Name_Official} will establish objectives with regard to Equality and will monitor the success of these objectives and establish plans to improve the delivery of these objectives.
- The collection/analysis of data is vital in informing change and improving performance. Where appropriate, statistics on Crescent Community Care Services Limited's services will be collected and analysed in relation to Equality and Diversity.
- We will review employee turnover and seek information on reasons for leaving through exit interviews. Local and national data or statistics will be used to benchmark our performance.
- The Registered Provider and Registered Manager will annually review equality of opportunity relating to Crescent Community Care Services Limited services. Recruitment and selection procedures will be monitored and reviewed annually by the relevant Manager who will report to the Registered Provider. All aspects of personnel policies and procedures shall be kept under review in order to ensure that they align with this policy.
- In order to determine the impact of this policy it is important that a monitoring system is developed, which will measure commitment, progress and effectiveness and good practice. The Equality and Diversity Policy will be monitored and reviewed as follows:
 - The policy will be an annual agenda item at Crescent Community Care Services Limited's quality team meetings.
 - The relevant manager will undertake an annual policy review for Crescent Community Care Services Limited. All relevant parties will be encouraged to submit comments for consideration.
- If it is found that the policy is excluding or discouraging the development of staff or volunteers or restricting Service Users, the Registered Manager will take steps to amend the policy.

Reviewed Marcus Kerridge-McColl

December 2025

Double Up Home Care Policy and Procedure
Crescent Community Care Services

Purpose

1. To ensure that services are delivered to contract, and in a safe and personalised manner.
2. All care delivery requiring two or more carers in attendance.

Policy

1. The need for double up service will be subject to normal assessment and management processes, with the objective of ensuring that the service is robust, safe, and meets the Service User's needs.

Procedure

1. **Management:**



1. A competent assessor will always carry out the assessment of the need for Double up service, which will be as a result of an opportunity or need, and carried out within the normal assessment and review procedures of The Agency.
2. Double up will normally be proposed in response to moving and handling process needs, but other issues may give rise to a proposal, such as security. The objective of the double up service must be clearly identified in the Care Plan.
3. The Service management must, following the creation of a double up requirement in the Care Plan, identify and allocate the second carer using the same processes to match skills, knowledge, personality, gender and reliability which are used for primary/single carers.
4. The Service User will be fully involved with the assessment which gives rise to the need for double up and will consent to the Care Plan.
5. The care service coordinator will organise the logistics of providing a double up service, and communicate the service design including the risk factors, to all staff concerned, and the Service User and their family/advocate.
6. The service design will be fully annotated within the Care Plan for the individual Service User with the objective of ensuring that the service is stable, consistent and independent of individual service managers on duty.
7. The care service coordinator will, on notification that only one carer is in attendance on a particular visit, discuss the immediate needs of the Service User with the carer and instruct them regarding the limits of what processes they can safely carry out, and make alternative arrangements for any processes which must be carried out for the safety and comfort of the Service User.

8. **Communication and familiarisation:**

1. New carers must be introduced to the Service User at the beginning of their first attendance. The lead carer will brief any second carer who has not previously attended that Service User before the introduction, preferably before entry to the premises.
2. The lead carer has responsibility for all recording of services delivered within the Care Plan documentation, communication with service management, communication with other health professionals, and family/advocate communications.
3. The lead carer will attend all service reviews.
4. Both double up carers must ensure that they communicate primarily with the Service User during their visit, involving them in the conversation at all times, and never talking over and excluding them while discussing about matters not related to the processes in hand.

5. **Lead carer:**

1. In each double up situation, one person will be formally designated by the service management as the lead carer for that Service User. The lead person will not always be the most senior carer, but will typically be the carer who is normally permanently allocated to that Service User and who therefore best knows their detailed preferences such as preferred mode of address.
2. A lead carer is also required to ensure that moving and handling processes are not subject to misunderstandings and therefore increased risk, and that processes are agreed before use and enforced by the lead carer.

3. **Security and safety:**

1. The lead carer retains responsibility for the security of the Service User, and the premises, at all times.
2. Each carer will notify the service office immediately if the second carer unexpectedly does not attend.
3. Carers will not, under any circumstances, attempt identified double handling processes without the second carer being present and participating. In the event of non-attendance by a second carer the carer in attendance will contact the service management to discuss alternative courses of action for double handling, if any.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To meet the requirements of Section 20 of the Health and Social Care Act 2008 (Regulated Activities) Regulations 2014.
- To define the obligations of Crescent Community Care Services Limited to act in an open and transparent way in relation to Service User care and treatment.

Scope

- Any notifiable incident which occurs during the provision of a regulated activity.

Policy

- The Provider, individual or organisation, and their representatives, must make public commitments to transparency, openness and fairness; demonstrate their compliance with this standard of behaviour in daily practice and communication; support employees at all levels to follow the commitment; and not undermine their efforts to do so.
- The publication and dissemination of this policy can be regarded as a statement of commitment to transparency and openness by Crescent Community Care Services Limited.
- The Provider, individual or organisation, and their representatives have the primary responsibility for the effective execution of this policy. In normal practice, the Registered Manager may carry out the procedure set out below, unless the Provider considers that the nature of the incident means that the investigation may be compromised if the Registered Manager carries it out, in which case the Provider should delegate a representative of the Provider to manage the procedures.
- Service Users and their representatives must be informed as soon as reasonably practical of any notifiable incident.
- A notifiable incident is defined as any unintended/unexpected incident in respect of the Service User during the provision of a regulated activity, which in the reasonable opinion of a health care professional could have or has resulted in:
 - The death of a Service User or;
 - Severe harm, moderate harm or prolonged psychological harm:
 1. **Prolonged psychological harm** can be defined as psychological harm which a Service User has or is likely to experience for a continuous period of at least 28 days.
 2. **Moderate harm** can be defined as harm that requires a moderate increase in treatment, including re-admission, prolongation of care, admission to hospital, referral to hospital as an outpatient, cancelling of treatment that is otherwise needed, or transfer to another specialist facility or treatment area.
 - Moderate harm also includes significant but not permanent harm.
 3. **Severe harm** can be defined as a permanent reduction of bodily, sensory, motor, psychological or intellectual functions, including procedures carried out on the wrong person, or wrong area of the body of the right person.

Notifications should include:

- Notification that the incident has occurred.
- An apology – this is different to an admission of liability. Whilst it is appropriate to express sympathy or regret, the apology should not include any admissions of fault.
 - Note that the Complaints, Suggestions and Compliments Policy and Procedure already states that open and responsive communications must be established with a person who complains (in the context of this policy the complainant is the service itself in the first instance, because it is the service which has identified its own incident), and that an apology is the best way of maintaining open communication and taking heat out of the situation in order that it can be approached professionally, rationally and logically. Therefore, if the Complaints, Suggestions and Compliments Policy and Procedure is fully and effectively implemented, there should not be a reluctance to acknowledge a problem and apologise for its occurrence, without defining blame.
- What further enquiries will be taking place – if an internal investigation is to take place, who will be interviewed and whether external input will be obtained and when enquiries might be completed. Note that good quality assurance practice dictates that an internal investigation will always take place when an untoward incident (not just an incident as defined under this regulation) occurs, in order to provide quality improvement data for the management process. Refer to the Quality Assurance, Quality Meetings and Management Meeting Policy and Procedure documents within the QCS system.
- A factual account of the incident – it is important that the account is purely factual and should avoid expressing an opinion as to the cause of the incident or admitting any blame.



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- Confirmation of when an update will be provided – the further notification should include an apology and details of the outcome of any further investigations.
- Where for any reason the Service User cannot be contacted, or after contact declines to communicate with the service, a written record of all attempts to contact them must be kept.

Reporting a possible breach of candour:

- If any individual considers that a breach of candour has taken place, i.e. a notifiable incident has not been reported as outlined above, they must report their allegation to the Registered Manager:
 - The Registered Manager will conduct an investigation into the allegations and report the findings to the Provider for action if appropriate.
 - If the allegation concerns the Registered Manager, the individual must report the matter to the Provider directly, who will carry out the investigation and take any action which may be required.
 - If the allegation concerns the actions of the Provider, the individual must inform the Provider, and if action is not seen to be taken, the matter must be reported to the Care Quality Commission. In this event the Provider must keep the reporting individual informed of progress, and carry out corrective action, if they wish to avoid referral to the Care Quality Commission.
- If a breach of candour is found to have occurred following investigation, and that this breach was by a professionally registered person, then that person should be reported to their professional registration body for further consideration. The same action should be taken if, during the investigation, it is found that a professionally registered person had obstructed another person in their professional duty of candour.

Procedure

Awareness

- All staff must be made aware of their personal responsibility to report incidents which may be covered by this regulation, through exposure to this policy and procedure with adequate time to read and understand it. Group implementation sessions would be a useful means of achieving this understanding, supported by inclusion of the matter in the next employee supervision session to test their retained understanding:
 - Remind staff of the Accident and Incident Reporting Policy and Procedure, which overlaps to a significant extent with this policy. If the Accident and Incident Reporting Policy and Procedure is fully and effectively implemented, staff will already be aware of the need for vigilance and the need to report not just accidents or incidents, but also “near misses” i.e. incidents which may have been stopped or ceased before any harm was done, but which if not stopped in time may have caused harm to an employee or Service User. As such the Accident and Incident Reporting Policy and Procedure already goes beyond this policy scope, but does not include the requirement to notify the Service User or their representative.
 - Note that the Serious Incident Notification Policy and Procedure also overlaps with this policy, within the scope of notification to the CQC.
 - Remind staff that attempts by other staff to prevent them from reporting incidents is bullying and/or harassment, and that they should report this immediately to their manager, or if the pressure is from their manager, a representative of Crescent Community Care Services Limited.
 - Remind staff that should the incident appear at first to be on the borderline of reportable and non-reportable. It should be reported anyway.

Reporting, Investigation and Communication

- All staff must report incidents defined in this policy in a way which generates an understandable and permanent record, for instance using the attached forms even if a verbal report has been already been made. The report must be made to the person on duty and in charge of the service at the time of the incident, who in turn must formally report it to the Registered Manager if they are not the same person, as soon as is practical, with regard to the seriousness of the incident. For example, a serious incident, may require immediate notification to the Registered Manager or their delegate even if they are not on duty at the time. A less serious incident may only require to be brought to their attention when they are next on duty. If in any doubt, report immediately.
- The Registered Manager or delegate should:
 - Carry out an initial assessment of whether the report does include details of a notifiable incident under the regulations. If the conclusion is yes, or borderline, continue with this procedure. If it is considered that the report does not constitute a notifiable incident, discuss that conclusion with the Service User and the staff member making the report. If they agree with the conclusion, follow normal incident reporting procedures as set out in the Accident and Incident Reporting Policy and Procedure. If they do not agree and continue, after consultation, explanation and negotiation, to hold the opinion that the incident is reportable under the regulations, then proceed with this procedure.



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- Inform the Provider or their representative of the incident report, and agree with them who is the most appropriate person to continue the procedure. If the Provider takes over the role, the Provider should continue the process using the following procedure.
- Prepare and deliver as soon as is practical a notification of the incident (or borderline incident which may not yet have been fully defined) to the Service User and their representatives. While this can be done verbally, and may need to be done in that way if circumstances dictate, such as the incident being very obvious at the time and needing to be acknowledged on the spot, good practice would be to gather at least outline information and record it, and quickly develop a notification statement with some thought and consideration behind it. This could be done in rough on the notification record attached simply in order to clarify matters and ensure all available data has been considered, then transferred to a more formally formatted copy later. It is important to retain all records permanently, including rough notes, written notes on verbal exchanges, and written notes on discussions with the Service User and representatives. The latter could usefully be recorded on the Family and Service User discussion record sheet from the Care Plan.
- Include an apology that the incident occurred, without apportioning blame in any way.
- Inform the Service User and representative of sources of support and information which may be of assistance to them. Offer them support from within the organisation if possible, for instance by allocating to them staff other than those in whom they may have lost confidence as a result of the incident. External resources such as advocacy services should also be considered and offered if appropriate.
- Prepare a list of people to be interviewed in order to gather data statements; also any other information which may assist an investigation, and include that information in the notification.
- Inform the Service User and representative of the likely progress of the investigation and the estimated date of the final conclusion, or any proposed interim stage. If this is done separately to the initial informing of the incident, include the apology made in that document.
- Identify and inform any other notification requirements, for instance CQC, Health and Safety Executive, GPs, Care Commissioners, Pharmacist, Adult Abuse Team, DBS, insurers (who will require notification as soon as possible and who may become involved with investigations), Environmental Health, and/or regional or national management if appropriate to the organisation.
- Prepare a list of people to be interviewed in order to gather data statements; also any other information which may assist an investigation.
- Carry out the interviewing and data gathering identified, adding any further sources which may be identified as the investigation proceeds.
- Having gathered all the evidence you think you need, carry out an investigation and reach a conclusion. Record your deliberations and how the various factors were weighed and balanced, and keep the record. Seek outside or independent advice if you feel that you are lack the skills or knowledge to effectively weigh the factors and reach a conclusion you are confident of.
- During the process this far, if possible avoid identifying individual person involved by name. The purposes of the investigation are to establish if the incident actually happened, define its nature, gather facts about the processes around the incident, and identify causes where possible. At this stage no blame can be attached to any person or process because the facts are not yet known, therefore to identify individuals by name may be unfair to them.
- Prepare a statement to be given to the Service User and representative stating the outcome of the investigation. Remember that the underlying objective of the regulation is that the service be open, honest, fair, and responsive in its response to the incident. The final statement should include a more specific apology than the generic one issued with the first notification, because the causes of the incident should have been established. However, it is very advisable to discuss this area with your insurers, who will have competing concerns to your own obligations under the legislation, will not wish you to admit liability. If a conflict does arise between the obligations under legislation and the insurer's views, the legislation must be followed.

If a Breach of Candour is found to have taken place

- Follow the policy set out above. Use the accident/incident reporting form attached to make the report in a full and accountable manner.

Reviewed Marcus Kerridge-McColl

October 2025

Purpose

1. To ensure that there is effective communication in an emergency situation

Scope

2. All employees and Service users / families

Policy

3. The Registered Manager has a 24 hour on call responsibility, except for where there is a specific arrangement made to the contrary and is clearly communicated to all members of staff and Service users
4. The On Call Care Manager, or the Duty Manager may be contacted out of hours and they will have access to the Registered Manager or Directors if guidance is required.

Procedure

1. If advice is required then contact the On Call or the office
2. In the event of any emergency including carer illness, nonattendance of work or for Service Users and their families the Office or the On Call must be informed immediately
3. Any care worker unable to access a Service users property must call the office or On Call immediately
4. The On Call will always have a Duty Manager that they can gain advice from
5. If the Duty Manager is unable to offer advice or support then they may contact the Registered Manager and or the Directors
6. All relevant telephone numbers are kept in the office and by the On Call Care Manager
7. The On Call Care Manager telephone number for out of hours is 07958 789359
8. The On Call Care Manager and the office also have a list of other emergency numbers including the Emergency Services, Duty Social Workers and a list of all the Service users GP's

Reviewed Marcus Kerridge-McColl
September 2025

Entry to Service Users' Premises and Safekeeping of Keys Policy and Procedure
Crescent Community Care Services Limited

Purpose

To comply with best practice safety requirements and protect from security problems.

Scope

All premises entered by Crescent Community Care Ltd staff members, or for which Crescent Community Care Ltd holds keys.

Policy

1. The security of access to Service Users' homes is of paramount importance, and should never be compromised.
2. Strict procedures must be followed by all agency staff having access to Service Users' premises, their keys or their access codes.
3. Employees of The Agency must not take keys directly from a Service User without the knowledge of the office.
4. Keys should never carry a home address or telephone number; this is for the safety of Service Users.
5. If keys are lost, the loss must be reported to the manager immediately.

Procedure

1. All visits to Service Users' homes will be planned in advance, and clearly logged on the on-site attendance documentation and notified to the administration office.
2. Any changes or exceptions to the above policy will be notified in advance and recorded as specified.
3. Carers will not hold keys to Service Users' homes unless by prior arrangement with the Registered Manager.
4. Holding of Service Users' keys will be on an individual basis, not a matter of regular practice, and supported by a needs assessment which will be kept in both the on-site records and the service administration office.
5. A receipt will be issued for keys held.
6. Keys will be kept secure in a key safe in the service administration office, and will be issued to the Carer attending the Service User's home as required, and returned when not required. Issue and return will be recorded.
7. The address will not be fixed to the keys. Use a code, such as the Service User file reference number, or a specific code held in that file.
8. On cessation of service, or change of circumstances which no longer require keys to be held, the keys will be returned and a receipt obtained.
9. Carers attending a Service User's home will at all times wear their identification badge.
10. Carers attending a Service User's home will ring the doorbell or knock so as to announce their presence on opening the door.
11. If the Service User's home has an entry code, all reasonable Care must be taken to ensure that the code is kept confidential, and the administration officer informed immediately if there are any concerns about the security of the door entry procedure, whereupon administration must make immediate arrangements for a change of code.
12. Entry codes will be changed at random intervals not less frequently than every three months, and in every case where there is a change of Carer.
13. If Carers are unable to gain access to the home, or access is given but treatment is refused, they should contact the administration office immediately to inform their line manager and leave a note that they have attended. The line manager will contact the family to inform them of the issue of access and the registered manager will decide if any further action is required, including the need to notify social services or the emergency service, based on risk assessment and knowledge of the family. This may be referred to as a Non-Access Policy by some authorities.
14. Time for Care visits must be strictly observed and if the Carer is delayed for any reason the family must be notified.

Reviewed by Marcus Kerridge McColl
November 2025

Fair Access, Diversity and Inclusion Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To promote equality of opportunity, diversity, inclusion in employment and the delivery of goods and services regardless of sex, marital or civil partnership status, gender reassignment, race, colour, nationality, ethnic or national origins, sexual orientation, disability, religion or belief, political affiliation and age and to overcome discrimination due to visible and non-visible differences. The purpose is also about respecting and valuing other people.

Scope

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2. The Fair Access, Diversity and Inclusion Policy applies throughout Crescent Community Care Services Ltd including:
 1. Recruitment and Employment practices;
 2. Training and development;
 3. Service and product procurement (contractor / supplier standards).

4. For employees, compliance with this policy is a condition of employment within Crescent Community Care Services Ltd services.
5. People who use or seek Crescent Community Care Services Ltd services shall be protected by the policy. Equally they shall be expected to respect the rights of others under the terms of the policy.

Policy

6. Crescent Community Care Services Ltd upholds the right of people who use its services, its employees and volunteers to be treated fairly and without discrimination at all times. Crescent Community Care Services Ltd also further upholds the right of those people to express their concerns or complaints and for them to be heard and dealt with appropriately.
7. Crescent Community Care Services Ltd as an employer and service provider shall take all reasonable and practical measures to ensure its practices are free from discrimination and inclusive and accessible to all.
8. Crescent Community Care Services Ltd wishes to set minimum standards in accordance with the law and the requirements of its regulators and funders, and will ensure that service delivery, employment and the supply of goods and services will be carried out in line with these standards. Those standards are set out in the accompanying sections of this policy.
9. In summary, standards shall:
 1. Promote 'equality of opportunity';
 2. Address and tackle discrimination in proactive and positive ways;
 3. As far as is practically possible, actions under this policy will be proactive rather than reactive in nature.

Aims of policy Service Users

4. There shall be a clearly worded statement in an accessible format or with an easy read version (delete as appropriate) which shall be made available and posted clearly in all services and which provides a summary of the Policy Statement.

Learning and development

5. This policy sets out the basis upon which the employees shall be expected to demonstrate their learning and development.

Public reference

6. Crescent Community Care Services Ltd is required to show publicly that it has adopted a policy covering Fair Access, Diversity and Inclusion, more commonly known as 'equal opportunities'. This policy document and its summary demonstrates Crescent Community Care Services Ltd's public commitment.

Responsibilities

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1. **Marcus Kerridge McColl** is responsible for promoting Equal Opportunities both in the home and in the community settings.
2. Individual members of the **Crescent Community Care Services Ltd** Management Team are responsible for the detailed implementation of actions under this policy within their areas of responsibility and for ensuring that the review of this policy is carried out in a timely manner in consultation with relevant experts in the field.
3. **Marcus Kerridge McColl** is the lead member of the **Crescent Community Care Services Ltd** Management Team for the implementation of this policy.
4. **Marcus Kerridge McColl** is specifically responsible for ensuring that monitoring and performance data, relating to the recruitment, employment and development of **Crescent Community Care Services Ltd** employees, is updated and made available at quarterly intervals to senior managers, kept/stored securely and disseminated only in accordance with laid down guidelines and the Data Protection Act.



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5. All managers of **Crescent Community Care Services Ltd** who are responsible for press, publicity and marketing strategy will ensure that any actions arising from the policy are addressed in external statements and will reflect as one of the key tenets of Crescent Community Care Services Ltd services.
6. All managers will:
 1. Embody best practices at all times in promoting Fair Access, Diversity and Inclusion within **Crescent Community Care Services Ltd** services and workplace settings;
 2. Ensure that new employees during the first day of induction are made aware of this policy and its application within their role, day-to-day work and contact with clients, Service Users and all stakeholders;
 3. Consider the need for Personal Development Planning and regular briefings to address any matters under this policy;
 4. Actively address with individual staff members (through formal and informal supervision and appraisals) as well as teams, any issues arising from this policy.
1. All Staff must at all times abide by this policy. Breach of this policy may result in serious consequences.

§

Legal and regulatory context

2. Key groups of people and some types of discrimination that are dealt with under statute include:
 1. Equal pay;
 2. Gender Issues;
 3. Race (includes culture and language);
 4. Disability (physical and mental health, learning disability, sensory impairment etc);
 5. Religious beliefs or non-belief;
 6. Sexual orientation;
 7. Gender Realignment (Recognition of);
 8. Age;
 9. Mental Incapacity;
 10. Housing (accessibility).

Shortened policy statement

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1. An abbreviated and easy to read summary of this policy is produced in the Appendices and shall be displayed wherever **Crescent Community Care Services Ltd** is responsible for delivering services.
2. It is important that Service Users, employees and volunteers are reminded of their rights and responsibilities, in their contact with or for **Crescent Community Care Services Ltd**, and that breaches of this policy will be taken very seriously. Swift action will be taken to address any breaches.
3. Employees must note that only Shortened Statements which have received **Crescent Community Care Services Ltd** senior management approval may be displayed.

Standards

Individual and personal behaviour

1. The culture of **Crescent Community Care Services Ltd** is demonstrated through the standard of behaviour and attitudes shown by employees towards others. The culture of any organisation is also demonstrated by the style of leadership adopted by managers and other senior people.
2. Recognizing and respecting our clients, Service Users and employees through the promotion of this policy creates a cohesive team and equips Crescent Community Care Services Ltd to meet business and competitive challenges full on.

Learning about ourselves and each other

1. **Crescent Community Care Services Ltd's** philosophy embodies a culture which values people. In recognising the importance of practices that ensure fairness of access, diversity and inclusion, it is equally important to recognise that all individuals carry their own learning and experiences that have shaped the way that they understand and respond to others.



CRESCENT

2. Cultures which are based upon blame and persecution, as well as the misconstrued and excessive use of the term 'political correctness', are far from suited to supporting ongoing learning. Learning about ourselves and others depends upon a culture which values reflection, positive comment, transparency and constructive feedback as essential, albeit at times challenging.
3. Challenging each other and our own attitudes and behaviour, without fear of condemnation, and learning from those experiences, helps everyone to ensure that people are valued and understood; this will promote a better understanding of what is necessary to ensure probity, accountability, inclusiveness and fairness within **Crescent Community Care Services Ltd** services and workplaces.

Listening and consulting

1. The culture of **Crescent Community Care Services Ltd** is also demonstrated in the way it responds to the needs and expectations of people who use, or who may use **Crescent Community Care Services Ltd** services. It follows that consulting and listening is a vital part of promoting inclusion and fair access to all. Developing a culture that is responsive to external factors makes it possible to understand, plan and deliver services that are sensitive to the needs of the widest possible range of need in the community. By the same token, information about **Crescent Community Care Services Ltd** services must be presented in a way that is informative, helpful, timely, credible and user friendly.

Workplace / team value statements

2. **Crescent Community Care Services Ltd** has adopted a culture whereby work teams are expected to adopt its particular value statements, in addition to those of **Crescent Community Care Services Ltd** as a whole. It is important, in order to promote consistency, that when setting and reviewing value statements this policy is considered as an intrinsic part of that process.

Service Users

3. In responding to the needs of Service Users, **Crescent Community Care Services Ltd** will ensure fair access to its services. **Crescent Community Care Services Ltd** will make sure that everything it does is transparent, regularly reviewed and updated in order to meet the changing needs of current and future Service Users.
4. Information, with staff support as necessary, will be made available to help to explain the Equal Opportunity Policy and any other key policy or procedure. This information shall be clear, concise and 'fit for purpose'.
5. We will take into consideration any communication or language needs of clients, Service Users, and other stakeholders in all future service planning and development.
6. **Crescent Community Care Services Ltd's** complaints policy will be made available to all Service Users, parents/carers, advocates, friends and befrienders and the public as we welcome complaints in order to maintain our standards and to improve service delivery.
7. Service Users will be encouraged by staff at **Crescent Community Care Services Ltd** to self-advocate by making a complaint themselves if required or will be supported to access Advocacy Services to do so.
8. Service Users are asked to respect the rights of others. There is a Safeguarding Policy (CR03, CR04) in place to protect them from abuse.
9. **Crescent Community Care Services Ltd** will ensure that all buildings for which it is responsible will be accessible, safe and welcoming and will ensure that the environment is conducive for the purposes for which it is intended.
10. **Crescent Community Care Services Ltd** will work and consult with other organisations and groups in a fair open and professional manner at all times.

Service User inclusion

1. Service User inclusion is pivotal to the planning and delivery of all services so that they are relevant, of high quality, and sensitive to the needs of people who use them. Crescent Community Care Services Ltd therefore acknowledges that Service Users are 'experts by their experience'.

Suppliers



CRESCENT

2. **Crescent Community Care Services Ltd** will ensure that procurement and supplier selection includes an assessment of their ability to act consistently with this policy.
3. **Crescent Community Care Services Ltd** recognises that the people who use its services have different needs but the right to the same quality of service. All people who use our services have the right to benefit from them without being subjected to direct or indirect discrimination or abuse from employees, contractors or other persons.
4. The process of selection and procuring of suppliers will be undertaken in a way that is fair and non-discriminatory and shall be subject to monitoring, audit and regular review.

Employment (includes volunteers)

1. **Crescent Community Care Services Ltd** provides equal pay without discrimination as defined under the Equal Pay Act 1970.
2. **Crescent Community Care Services Ltd** encourages flexible work practices to reflect work life balance or ways of working to allow a diverse range of people into employment, and to help employees manage disabilities, illnesses or religious observances that occur during their employment.
3. Guidelines will be issued to managers on the promotion & monitoring of standards. Any contraventions of Equal Opportunities standards will be fairly dealt with under robust and established systems in place.
4. In order to review the effectiveness of the policy and to progress our diversity strategy, **Crescent Community Care Services Ltd** will maintain an up-to-date employee database that provides information which can be used to review and monitor equal opportunities within the workforce. This information will be kept separate from any other employee files or information stored and will be kept according to data protection rules and regulations.

Recruitment (includes volunteers)

All new posts will be thoroughly evaluated for the opportunity of taking positive action, involving the use of statutory provisions of the Equality Act 2010 and the Disability Discrimination Act 2005 to deal with the under-representation of groups in **Crescent Community Care Services Ltd's** workforce.

1. The selection process for the recruitment of new staff and volunteers will be clear and transparent, based solely on relevant criteria.
2. Recruitment procedures and recruitment training will aim to promote equal opportunities and avoid the possibility of discrimination from application through to selection.
3. **Crescent Community Care Services Ltd** will aim to ensure that staff and Service Users representing a diversity of people will carry out the interview/selection process to reinforce user involvement.
4. Recruitment documentation will state **Crescent Community Care Services Ltd's** commitment to Equal Opportunities and anti- discriminatory practices within its service delivery.

Training

1. **Crescent Community Care Services Ltd** has a training policy that affects all employees. **Crescent Community Care Services Ltd's** ability to deliver excellence in its service quality is dependent upon a skilled and competent workforce for which learning and continual improvement is a fundamental principle. It follows that principles of merit and equality are central to such a strategy.
2. That policy shall set out the processes for all employees undertaking and benefiting from Learning and Development opportunities.
3. Eligibility in accessing learning and development opportunities is dependent upon need and will 'fit' with each individual's job description. Access to any formal or informal learning opportunities is carried out in line with the training plan; **Crescent Community Care Services Ltd's** training policy and the individual's Personal Development Plan. In addition, access shall be based upon the need to address issues of equality among **Crescent Community Care Services Ltd** user groups.



CRESCENT

4. All Induction and Personal Development Plans shall positively address any instances where inequality may prevent someone's ability to participate fully, and will identify necessary measures to maximise outcomes for each individual.
5. All managers will assist new staff in developing their understanding of this policy during their induction period. This will include the standards of expected behaviour and confidentiality in relation to others.
6. All members of staff will be given ongoing opportunities to discuss and further develop their understanding and practice in relation to **Crescent Community Care Services Ltd's** Fair Access, Diversity and Inclusion Policy.
7. Providers, consultants and participants involved in training activities should adhere and actively engage in implementing the Policy.

Methods of complaint

1. **Crescent Community Care Services Ltd** is required to adopt a range of formal procedures which are designed to protect Service Users and employees from discrimination and victimisation. They include:
 1. Complaints, Suggestions and Compliments Policy and Procedure – for Service Users/members of the public/staff;
 2. Harassment Policy and Procedure – for staff;
 3. Grievances Policy and Procedure – for staff and former members of staff;
 4. Whistleblowing Policy and Procedure – for staff and volunteers.

Appendix

FAIR ACCESS, INCLUSION & DIVERSITY POLICY SHORT STATEMENT Service Users

Crescent Community Care Services Ltd recognises that the people who use its services have different needs but the right to the same quality of service. All people who use our services have the right to benefit from them without being subjected to direct or indirect discrimination or abuse from other persons.

Reviewed by Marcus Kerridge McColl
December 2025

Falls Management Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To apply best practices in the management of falls.

Scope

2. All falls.

Policy

3. All falls by Service Users, however apparently trivial, will be:

» Immediately recorded in the Care Plan daily records for that Service User. » Immediately relayed to the Manager.

4. The Manager will ensure that:
 - » All staff are trained to record and report all falls.
 - » All reports of a fall result in an immediate dynamic review of the falls plan of Care for the affected Service User, according to the formats in the Care Plan pack.
 - » The Care Plan review is holistic.

Procedure

1. The Manager will conduct an immediate dynamic review of the falls plan of Care for the affected Service User, according to the formats in the Care Plan pack.
2. The Care Plan review must be holistic, that is, the review must look at other risk factors and assessments in order to identify and assess possible reasons for falls.
3. The formats for a comprehensive falls assessment are contained in the Care Plan pack.

Reviewed by Marcus Kerridge McColl
November 2025

Family Leave Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To comply with the Employment Relations Act 1999 and the Employment Act 2000.

Scope

2. All employees.

Policy

3. The organisation recognises that at various stages in employees' working lives, domestic and family commitments and responsibilities potentially reduce or hinder their ability to fulfil work responsibilities. Where such conflicts cannot be resolved, this could result in the company losing employees. To prevent such loss of skilled experienced workers, and to help reduce anxiety and stress amongst our workforce, this organisation not only acknowledges those potential conflicts but also offers practical help through its family leave.

Procedure

Statutory Paternity Leave



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4. Fathers and partners of mothers are entitled to a paid paternity leave. Partner means a person who lives with the mother and the child in an enduring family relationship, but is not the mother's parent, grandparent, sister, brother, aunt or uncle. Parental rights extend to partners in same-sex couples.
5. Fathers and partners are entitled to two weeks statutory paid paternity leave. This must be taken as a single period of one week or two consecutive weeks, and must be taken within 56 days of the birth or 56 days of the first day of the EWC, whichever is later. Odd days cannot be taken, or two one-week blocks.
6. The pay is called paternity pay (birth) to distinguish it from the pay available to adoptive parents. The weekly rate is either £100 or 90% of the employee's normal weekly earnings, whichever is lower.
7. To be eligible for statutory paternity leave and pay the employee must satisfy the following conditions:
 1. The employee must have had 26 weeks of employment with the employer by the end of the 15th week before the EWC;
 2. They must have average weekly earnings of at least £77; and
 3. They must be the child's father and have responsibility for the child's upbringing or, if they are not the child's father, they must be married to, or be the partner of, the child's mother and must have the main responsibility (apart from any responsibility of the mother) for the child's upbringing.
4. Either before or during the 15th week before the EWC or as soon as is practicable after this, the employee must notify the employer as to how much paternity leave they intend to take, the EWC, and the date when they want the leave to begin. This date can be the date of the actual birth, or a date that is a specified number of days after the birth, or a predetermined date which is later than the first day of the EWC. This notice must be in writing.

Adoptive Parents Leave

1. Pay and leave are available for the same period and at the same pay as statutory maternity entitlements for one adoptive parent when a child is matched or placed for adoption, and for the same period and at the same rate as statutory paternity entitlements for the other parent.
2. The maternity-equivalent leave and pay are called ordinary (and, if applicable, additional) adoption leave and adoption pay. The paternity-equivalent leave and pay are called paternity leave (adoption) and paternity pay (adoption).
3. Where a married couple adopt jointly, they can decide who takes the adoption leave and pay and who takes the paternity leave and pay (adoption). (At the moment, only married couples can adopt jointly). Where there is one adoptive parent, they take the adoption leave and pay, and their partner, if they have one, takes the paternity leave and pay (adoption). The definition of partner is the same as for paternity leave (see above).
4. Adopters and partners must have been employed by their employers for at least 26 weeks ending with the week when an approved adoption agency notifies the employee that they have been matched with a child for adoption, or the week when the child is placed for adoption. Adoption pay and leave are not available when the child is known to the adopter, for example a stepchild or foster child. Special rules apply where a child is adopted from overseas.
5. To be entitled to adoption pay or paternity pay (adoption) the employee must have average weekly earnings of at least £77. Employers can recover statutory adoption payments from the Inland Revenue in the same way as SMP.

Parental Leave

1. Employees who have at least one year's continuous service will be entitled to 18 weeks parental leave upon the birth of their child. This leave may be taken at any time up to the child's fifth birthday. This right also applies to employees who have acquired formal responsibility for a child, e.g. guardians.
2. Employees who have at least one year's continuous service who adopt a child will also be entitled to 18 weeks' leave. The child must be under the age of 18. The right to parental leave lasts for five years from the date on which the child is placed for adoption.
3. Parents of disabled children can take parental leave up until the child's eighteenth birthday.
4. Employees who work part-time will receive a pro-rata amount of leave.



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5. An employee may not take parental leave in periods of less than one week (although parents of disabled children are allowed greater flexibility).
6. An employee may not take more than four weeks' leave per year per child.
7. Applications for parental leave must be made in writing to their manager and employees may be required to provide evidence of their responsibility for the child and the child's age.
8. Employees must give at least 21 days' notice of an intention to take parental leave and should specify the dates.
9. It may be necessary to postpone parental leave for up to six months (unless it is being taken immediately after a child is born or adopted) where the business could not cope or would be harmed by an employee's absence. Postponement will be confirmed in writing no later than 7 days after the employee's notice to take leave. This will state the reason for the postponement and set out the new dates of parental leave.
10. Employees will remain employed during the parental leave period. Certain contractual terms continue to apply. In particular, the employer's obligation of trust and confidence, terms concerning notice, redundancy compensation, disciplinary and grievance procedures. Also the employee's obligation of good faith and any express term prohibiting Disclosure of confidential information or participation in a competing business.
11. Payment will not be made for any parental leave taken.
12. Where both parents are employed by the company, each will be entitled to leave but both employees may not take parental leave at the same time.
13. Employees returning from parental leave of four weeks or less will be reinstated in their previous jobs. In extenuating circumstances parental leave may be extended at the company's discretion. Employees returning from parental leave of more than four weeks will be reinstated in their previous jobs wherever possible or, if not offered similar jobs which have the same or better status, terms and conditions as the previous jobs.
14. Note: Since January 2002 there are three major changes in parental leave entitlements:
 1. Parental leave is extended to parents of children born or placed for adoption between 15 December 1994 and 14 December 1999. The leave must be taken by 31 March 2005 (or up to the child's 18th birthday, if the child is entitled to disability living allowance);
 2. Parental leave is generally available only to parents with at least one year's continuous employment with their current employer. However, for parents of a child born or placed for adoption between 15 December 1994 and 14 December 1999, employment of at least one year with a previous employer can count. The year's continuous employment must have been between 15 December 1998 and 9 January 2002. The employee must give the current employer notice of the period of employment with the previous employer, and must provide such proof as the current employer reasonably requires;
 3. For all children who were born or placed for adoption on or after 15 December 1994 and are entitled to disability living allowance, the period of parental leave is extended from 13 weeks to 18 weeks, to be taken before the child's 18th birthday.

Compassionate Leave

1. Employees will be entitled to 5 days unpaid compassionate leave where a member of their immediate family dies, is seriously ill or in severe distress.
2. Additional days of unpaid leave, up to a maximum of 10 days, may be granted at the discretion of the Registered Provider.
3. Employees should contact the Registered Provider as soon as possible if they wish to make use of this provision.

Urgent Family Leave

1. All employees are entitled to up to 5 days unpaid absence per year as family leave, in addition to annual leave and other absence entitlements.

2. Family leave may be granted for pressing and important family reasons such as the complete breakdown of childcare arrangements.
3. Requests for family leave must be made to the Registered Provider. Wherever possible, such requests should be made in advance. Where the reason for the request is family illness, a medical certificate may be required. Not more than 5 days' family leave may be taken at a time.

Reviewed by Marcus Kerridge McColl
December 2025

Fire Safety Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To comply with statutes, regulations and quality standards. §
2. The Regulatory Reform (Fire Safety) Order 2005.

Scope

1. All establishments owned or controlled by Crescent Community Care Services Ltd. §
2. All Service Users wherever located
3. All employees

Policy

INTRODUCTION

1. We recognise that fire prevention is an important obligation for all organisations, including ours, and that fire has the potential to present significant risks to our health and safety (see Fire Safety Guidance).
2. It is our policy to:
 1. Assess the risks of fire at our premises and implement appropriate control measures;
 2. Ensure good housekeeping to minimise the risk of fire;
 3. Provide means of detection and early-warning in case of fire;
 4. Inspect and/or test fire safety equipment at appropriate intervals;
 5. Provide and maintain safe means of escaping from the premises in the event of a fire;
 6. Maintain all equipment and installations intended for fire-detection and fire-fighting;
 7. Implement a procedure for the action to be taken in the event of a fire;
 8. Train and instruct staff in fire safety including the carrying out of fire drills;
 9. Keep records of all fire safety matters (see Fire Records Section);
 10. Ensure that all visitors are made aware of the fire precautions and emergency arrangements;

11. Identify people with any disability or impairment who may require assistance in the event of a fire;
12. Comply with the requirements of the Regulatory Reform (Fire Safety) Order 2005 where one is in force;
13. Train staff in assisting Service Users in moving to a safe place.

Procedure

General

1. Carry out a fire risk assessment.
2. Repeat the fire risk assessment at least once a year, and always when there are significant changes to:
 1. The building;
 2. The processes carried out within the building;
 3. The use of the building;
 4. The number of people accommodated within the building;
 5. The dependency level of the people accommodated within the building;
 6. The equipment within the building;
 7. The level of training and skills, including language skills, of the staff using the building. §
1. Identify persons at risk who may need assistance to evacuate or in moving to a safe place.
2. Ensure that there is a method for giving warning of fires and that it can be heard in all areas of the establishment;
1. Ensure that the establishment can be evacuated safely in the dark or in the event of a power failure.
2. Display fire evacuation notices and ensure that all escape routes and exits are adequately signed.
1. Ensure that all escape routes and exits are free from obstruction and all firefighting facilities are readily available. §
2. Maintain fire detection, warning systems and installations.
3. Train staff, carry out fire drills and keep records.

Actions required prior to Evacuation

1. The Registered Manager or Person in charge to assess the need for evacuation ensuring the safety of Service Users, visitors and staff.
2. The evacuation plan is reviewed taking into account the evacuation procedures for individuals.
3. Staff to be briefed on the reason, method, route and onward plan prior to evacuation.

Actions required during Evacuation

1. The Registered Manager or Person in charge should ensure that a list is available at all times as to the people who have been evacuated and those still in the building
2. The Service User and staff register to be held by the person in charge.
3. Designated personnel should be established to man the external doors in the case of Fire Alarm activation ensuring that no Service Users leave the premises unaccompanied and that no inappropriate persons are allowed to enter the premises.

4. In the event the emergency services are involved – the lead will be taken from the senior officer on site.

Actions required after evacuation

1. The Registered Manager or Person in Charge should collate all of the information regarding the whereabouts of the Service Users and staff. Should it be necessary to contact relatives or next of kin, the Registered Manager or Person in Charge should ensure that all Service Users are in a specified location before any contact is made.
2. Relatives of Service Users and staff to be contacted and notified of their whereabouts.
3. In the event of a full evacuation of the premises, a designated person is to ensure the security of the building during this time.
4. Staff that were not on shift at the time should be notified of the location of the Service Users and what their location of work will be until it is declared safe to return to the building.
5. Notify Social Services and other organisations as necessary with the location of Service Users.
6. The CQC is to be notified as per on the Statutory Notification form, of the events leading up to, actions taken and current situation with regard to the evacuation, this can be done by telephone but must be followed up with an written notification

Actions required prior to any return to the premises

1. The Registered Manager or Person in Charge is to confirm that the premises are safe to return to having taken instruction from the emergency services personnel involved.
2. Any transport to return the Service Users and staff to the premises is arranged.
3. All areas of the premises to be checked for safety and cleanliness prior to any return.
4. Relatives to be contacted and kept informed.
5. Contact to be made with Social Services and other organisations advising of the return to premises
6. Contact the CQC or use a notification to confirm the premises are safe and Service Users can be returned.

Arson

Arson is a serious threat to homes, shops, offices, storage buildings, and hospitals. All buildings are at risk. Much of the arson is associated with vandalism and burglaries. If small fires have been started on your own or neighbouring premises they could be a warning of worse to come – inform the police and the fire brigade.

1. **Security:**
 1. Keep the number of entry points to the minimum compatible with safe means of escape in the event that a fire breaks out;
 2. Perimeter fences, walls and gates need to be strong and high enough to keep out intruders;
 3. Doors and windows must be in good repair and locked when not in use;
 4. Locks and padlocks must be of good quality;
 5. Keys must be distributed only to a restricted number of people;
 6. Gaps under doors must be kept small;
 7. Letter boxes should have metal containers fitted on the inside;
 8. Stored material of any kind should be kept away from perimeter walls or fences where it could be set alight.

9. **Employees:**

1. Warn staff about the threat from arson;
2. They should challenge anyone who should not be on the premises and report any suspicious activities;
3. Vet new employees;
4. Keep an eye on contractors.

5. **Visitors:**

1. Control the access and movement of visitors.

2. **Fire protection:**

3. Fixed and portable fire-fighting equipment must be regularly maintained and protected against sabotage attempts.

4. **End-of-day (onset of darkness or night shift) checks:**

5. Ensure that:

1. The building is secured by a named individual at the end of each working day; ☒

2. **Doors and windows** are secure;

3. No combustible material is left lying around;

4. No unauthorised people are on the premises;

5. Alarms are switched on;

6. External lighting is switched on;

7. Flammable liquids are locked in the proper store.

Common causes of fire and how to guard against them

Electricity

Neglect and misuse of electrical wiring, fittings and equipment can easily cause fires in the workplace. Overheating of electrical circuits, poor wiring connections, use of unauthorised electrical appliances, multi-point adaptors and problems with the use of extension leads are all hazards which frequently result in fires starting. Therefore:

1. Ensure that all electrical circuits and equipment in the workplace are inspected and tested on a regular basis. (Remember, there is a requirement to do this under the Electricity at Work Regulations 1989).
2. Have any faults in wiring or fittings repaired promptly by a qualified electrician.
3. Switch off electrical equipment when it is not in use and particularly at the end of the day's work.

Rubbish and Waste Materials

Rubbish and waste materials that are left to accumulate can easily contribute to the spread of fire; they are also a place for malicious fires to be started. Make sure that you remove all waste materials from the workplace on a regular basis and place them in a suitable container located in a safe position outside the building. Ideally this container will be of metal construction and fitted with a lockable lid. Arrange for the container to be emptied regularly.

Do not burn rubbish on bonfires, even if it is thought safe to do so. They can easily get out of control and spread fire to nearby buildings or structures.

Smoking

Smoking is still a major cause of fires in buildings. You should:

1. Consider having a 'No Smoking' policy in your establishment. But remember to make provision for those Service Users who like to smoke. This will be a designated area or room where sufficient glass or metal ashtrays are provided. A fire extinguisher should be provided in or near the room and any seating should have only small amounts of padded upholstery.
2. Prohibit smoking in storage areas. If the public visit your premises ensure that they understand that this is a no-smoking area.
3. Check designated rooms at the end of the working day to ensure that no smoking material has been left burning.

Cooking

Many small businesses have kitchens where staff may prepare food themselves. These facilities are similar to domestic kitchens and cooking hazards may still arise:

1. Avoid undertaking deep fat frying unless a thermostatically controlled pan is provided. Even then it would be wise not to leave the pan unattended.
2. Ensure that combustible materials such as cloths, towels and loose fitting clothing (especially sleeves) are kept well clear of hobs.
3. Toasters and microwave ovens should not be sited in office areas; they should only be available in kitchens.

Heating Appliances

Portable heaters can often be hazardous in the work environment, especially if placed too close to combustible furniture, fittings or materials. Convector heaters are safer than radiant fires. If you do need to use heaters:

1. Ensure that they are securely guarded and properly fixed to prevent them from being knocked over.
2. Place them well away from any materials which could easily ignite.
3. Never stand papers or books on them or drape clothing over them.
4. Do not allow ventilation grilles to become obstructed.
5. Clean portable heaters on a regular basis.

Combustible Materials

If combustible items, such as packing materials, glues, solvents, flammable liquids or gases, are used or stored in the workplace, it is recommended that:

1. The amounts brought into the premises should be kept to a minimum and sufficient for the day's work only.
2. The bulk supplies of such materials should be locked in a secure store, preferably outside the main premises.

Hazardous Materials

If you use paints, solvents, adhesives, chemicals or gas cylinders, keep them in separate storage areas and well away from any sources of ignition. Gas cylinders, even when empty, can explode when exposed to heat. Remember that the Highly Flammable Liquids and Liquefied Petroleum Gases Regulations 1972 may apply to your operations.

Fire drills

1. To ensure that all employees know how to leave the premises in the event of fire, repeated practice is desirable. Fire drills should be held at regular intervals and preferably every 2 months or whenever there is a substantial change in staff levels or turnover. All staff, from all shifts should participate in a fire drill. Employees should be trained:

1. To recognise the fire alarm when it sounds;
 2. To act in accordance with the evacuation plan;
 3. To leave the premises quickly by the nearest possible route;
 4. To go to the designated assembly point;
 5. To assemble for roll call.
-
1. Departmental managers (or their equivalent) should make sure that their departments are completely evacuated.
 2. Management should evaluate performances during fire drills and in particular should investigate the causes of any delays in evacuation and take steps to make sure delays are eliminated.

Records

1. To ensure compliance with the regulations
1. The service is required to maintain accurate and up to date records of all Fire drills undertaken.
2. The training statistics must be updated for all staff to ensure fire safety training has been undertaken and a plan of ongoing training and drills formulated.

Reviewed by Marcus Kerridge McColl
December 2025

Food and Nutrition Policy and Procedure
Crescent Community Care Services Limited

Purpose

To support the values of Crescent Community Care Services Ltd.

Scope

All Service Users.

Policy

1. All Service Users will be assessed for their food preferences and their wishes will be documented in the Care Plan.
2. All Service Users who have a particular nutritional need will have a full assessment of nutrition and all information regarding the Service User's nutrition will be documented in the Care Plan.

Procedure

1. Line Managers/Area Coordinators/Review & Assessment Officers will be trained in nutritional assessments and reviews.

2. Service User nutritional assessments will be carried out, and where any special arrangements or needs are identified, a Care Plan will be prepared.
3. Where a Service User is unable to eat appropriately for their correct nutritional balance, a referral to the GP will be made.
4. Specialist advice will be obtained from appropriate professionals such as GPs, community nurses and community dietician.

Reviewed by Marcus Kerridge McColl
November 2025

Food Hygiene Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To comply with statutes, regulations and quality standards. §
2. To prevent cross-contamination.

Scope

3. All carers also engaged in food preparation.

Policy

1. Crescent Community Care Services Ltd will follow good practices to comply with Food Hygiene Regulations and the requirements of the Environmental Health Officer.
1. In a residential setting, access to food preparation areas will be controlled to exclude as far as is possible persons who may unnecessarily increase risk.

Procedure

1. The kitchen must not be used as a thoroughfare.
2. Access must be limited to staff associated with food preparation.
3. The kitchen and equipment must be maintained at a high level of cleanliness at all times.
4. Smoking is not allowed in food preparation areas.
5. Pets are not allowed where food is prepared.
6. Staff working in the food service areas must maintain a high standard of hygiene (see Personal Hygiene Policy and Procedure).
7. Food waste must be regularly removed from the kitchen for disposal.
8. There must be suitable separation between raw and cooked items stored in a refrigerator or any other sterile facility.
9. The dishwashing machine should be used whenever possible.
10. A lid should not normally cover waste storage containers within the catering area, as this promotes cross contamination by handling. Storage facilities outside of the catering area must have a lid.
11. Food placed into temperature-controlled storage must be dated, and similar food rearranged so that the oldest is presented for use first.



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12. Rice and pasta must not be reheated. Other reheated food must be checked with a temperature probe to confirm that the body of the food has reached normal cooking temperatures.
13. On removal of stored food, a check should be made to ensure that the oldest dated item is used first, checking that it is still "in date" from both food hygiene and food quality viewpoints.

Reviewed by Marcus Kerridge McColl
December 2025

Grievances Policy and Procedure
Crescent Community Care Services

Purpose

1. The Grievance Policy is a framework for resolving a formal grievance which an employee has concerning any aspect of his/her employment.
2. The basic aim of the grievance procedure is to try and arrive at a mutually satisfactory solution of the grievance as quickly as possible. Employees are therefore encouraged to deal with the problem informally in the first instance with their manager. If it is not possible to resolve a grievance informally, employees should then formally raise the matter without unreasonable delay with a manager who is not the subject of the grievance. This should be done in writing and should set out the nature of the grievance.
3. The objective of a grievance procedure is to provide a recognised channel through which a grievance can be brought to the attention of management by providing the right for an employee to have their grievance heard, investigated and, if proved justified, remedied.

Scope

1. All employees.

Policy

2. The organisation recognises that from time-to-time employees may wish to seek redress for grievances relating to their employment.
3. In this respect, the organisation's policy is to encourage free communication between employees and their managers in order to ensure that questions and problems arising during the course of employment can be aired and, where possible, resolved quickly and to the satisfaction of all concerned.

Procedure

This procedure is not part of your contract of employment; however, we will not depart from it without good reason.

4. If there is any matter relating to your work, or the people with whom you work, with which you are unhappy, you should use the following grievance procedure.
5. If your grievance relates to an equal opportunities matter, it may be easier to deal with all complaints under one procedure rather than using both the grievance procedure and our equal opportunities complaints procedure. We will advise you of which procedure is most appropriate.
6. At any formal meeting, you have the right to be accompanied by a single companion who is either
 1. A work colleague; or,
 2. A full-time official employed by a trade union; or,
 3. A lay official, so long as they have been certified in writing by their union as having experience of, or as having received training in, acting as a worker's companion at disciplinary or grievance hearings.
1. Your companion has the right to explain and sum up your case, and to respond to any views expressed at the hearing. They may not answer questions on your behalf. If your companion cannot attend on the date we have set for the



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interview, we will always postpone the interview for up to five days and may (at our discretion) postpone it for longer should it become necessary.

Informal Stage

2. Where possible, the employee should first discuss on an informal basis, their grievance with their manager who is usually in the best position to help
3. On hearing the employee's grievance, the manager shall reach a decision within five working days or as soon as reasonably practical. If a delay to a decision is unavoidable, the manager will provide the employee with the reason why and the anticipated timescale expected to reach a decision

Formal Stage:

1. If it is not possible to resolve a grievance informally the employee should raise the matter formally and without unreasonable delay with a manager who is not the subject of the grievance. The grievance should clearly set out the facts in writing of the exact nature of the complaint or problem and the redress being sought. You should state that this comes under the formal stage of the grievance procedure.
2. The grievance will be investigated and the manager will arrange as soon as possible a mutually agreed date for a meeting where the employee will be given an opportunity to state their grievance. During the meeting the employee will be allowed to explain their grievance and how they think it should be resolved.
3. During this meeting, the employee may also be accompanied by a companion if they wish. To exercise this right, the employee must first make a reasonable request in advance to the manager. What is reasonable will depend on the circumstances of each case however, it would not normally be reasonable for employees to insist on being accompanied by a companion whose presence would prejudice the hearing or where it is not sensible logistically for a companion to attend for geographical reasons.
4. If the meeting is likely to take longer to arrange then this will be communicated to the employee giving the reason for the delay.
5. Consideration may also be given to adjourning the meeting for any further investigations as it may be necessary for us to make further enquiries with other people about your grievance, and possibly to have a second meeting with you.
6. The grievance meeting will be recorded in writing and the signature of the employee will be obtained as verification to the accuracy of the statement. After the meeting, we will write to you with the outcome of your grievance. The letter will include a right to appeal if you are not satisfied with the outcome.

Appeal:

1. If after receiving the grievance decision the employee is still aggrieved, they may inform a more senior manager in writing that they wish to appeal stating the grounds on which the grievance should be reviewed. This appeal must be submitted in writing within five working days of receiving the last decision. After this period, the right to appeal will be lost.
2. Following receipt of the employee's notification of their wish to exercise their right to appeal, the manager will arrange the appeal hearing for as soon as possible on a mutually agreed date. The employee again will have the right to request a companion at the meeting.
3. The decision of the Appeal will be conveyed in writing within ten working days. Where this is not possible, the employee will be given an explanation for the delay and notified when a response can be expected.
4. The decision of the Appeal shall be final and binding.

Overlapping Grievance and Disciplinary Cases:

1. Where an employee raises a grievance during a disciplinary process then depending on the circumstances, the disciplinary process may be temporarily suspended in order to deal with the grievance. Where the grievance and disciplinary cases are related, it may be appropriate to deal with both issues concurrently.

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Harassment Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To comply with the following:
 1. Equality Act 2010.
 2. Best employment practice.
 3. See also the Disabled Workers Policy and Procedure for further information on the effect of the Equality Act 2010.
 4. 2010.
 5. See also the Equality and Diversity Policy and Procedure for further information on the effect of equality and diversity regulations.
 6. Note: In particular that detailed procedures and requirements in relation to all aspects of equality, diversity, discrimination and harassment are embedded throughout all of the relevant policies in this management system, and that all procedures are designed to lead users of the system away from discriminatory behaviour and towards best practice behaviour.

Scope

7. All employees.

Policy

1. This organisation deplores all forms of **harassment, including sexual or racial harassment and harassment based on disability, age, gender, gender reassignment, religion or belief and sexuality and** seeks to ensure that the working environment is conducive to all its employees.
2. The following procedure informs employees of the type of behaviour that is unacceptable and provides employees who are the victims of any form of harassment with a means of redress.
3. Implementation of the policy is the duty of all staff, particularly managers and supervisors. All employees must comply with this policy. (The organisation encourages and welcomes the support of the recognised trade union(s) in seeking to eradicate harassment of all forms from the workplace).
4. **Harassment is unwanted conduct related to a relevant protected characteristic, which has the purpose or effect of violating an individual's dignity or creating an intimidating, hostile, degrading, humiliating or offensive environment for that individual (Equality Act 2010)**
5. The relevant protected characteristics are:
 1. Race, including colour, nationality, ethnic or national origin;
 2. Sex;
 3. Disability;
 4. Age;
 5. Religion or belief;
 6. Sexual orientation;
 7. Gender re-assignment.

Procedure

1. Sexual harassment as in other forms of harassment at work is unlawful. Both the organisation and the perpetrator may be held liable for such unlawful actions, and may be required to pay damages. Sexual harassment can reduce the effectiveness of the organisation by creating a threatening environment, and increasing stress, sickness absence and high staff turnover. Employees have the right to work in an environment free from sexual intimidation and harassment.



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2. Racial harassment at work is also unlawful and, as with sexual harassment, both the organisation and the perpetrator may be held liable. It also creates a threatening environment and all staff members have the right to work in an atmosphere free from racial intimidation.
3. Intentional racial or sexual harassment is also a criminal offence punishable by imprisonment or a fine.
4. Incitement to racial hatred is also a criminal offence punishable by imprisonment or a fine.
5. It is important for employees to note that the other types of harassment covered in this policy are also taken very seriously by the organisation and will be dealt with in the same manner. Harassment may consist of a single act or a series of acts.

EXAMPLES OF HARASSMENT

1. Sexual harassment takes many forms, from relatively mild sexual banter to actual physical violence. Employees may not always realise that their behaviour constitutes sexual harassment but they must recognise that what is acceptable to one person may not be acceptable to another. Sexual harassment is unwanted behaviour of a sexual nature by one employee towards another. Examples of harassment include:
 1. Insensitive jokes and pranks;
 2. Lewd comments about appearance;
 3. Unnecessary body contact;
 4. Displays of sexually offensive material, e.g. pin-ups;
 5. Requests for sexual favours;
 6. Speculation about a person's private life and sexual activities;
 7. Threatened or actual sexual violence;
 8. Threat of dismissal, loss of promotion, etc. for refusal of sexual favours.
9. Racial harassment and incitement to racial hatred can also take many forms; from relatively minor abuse to actual physical violence. Examples of harassment include:
 1. Insensitive jokes related to race;
 2. Pranks;
 3. Deliberate exclusion from conversations;
 4. Abusive, threatening or insulting words and behaviour;
 5. Displaying abusive writing and pictures.
 6. Displaying or disseminating material, or acting in a manner, which is likely to influence others to engage in racial discrimination or hatred.
7. The examples above are not exhaustive. Items 1(g), 1(h) and 2(d) could well amount to gross misconduct, and lead to summary dismissal, but other items may also constitute gross misconduct depending on the circumstances of the case in question.
8. Harassment related to other protected characteristics may also take similar forms to those outlined above with a similar impact on the organisation and employees as stated.

THE ENVIRONMENT

1. The organisation prohibits the display of sexually offensive material, e.g. pin-ups and posters or by emails or attachments, and will if necessary ensure that work-places are inspected and offending material removed.
2. The organisation expects all managers and supervisors to ensure that this policy and procedure is adhered to at all times.
3. The organisation recognises the sensitive nature of complaints of sexual harassment. Employees who wish to discuss such complaints in confidence should contact the Registered Manager.

TAKING ACTION AGAINST HARASSMENT

1. If an employee believes he or she has been the subject of harassment they should act promptly.
2. Write down a brief description of the unwanted behaviour or keep a diary of incidents as they occur.

INFORMAL REMEDY

1. Employees who believe they have been subjected to sexual or racial harassment are advised, if appropriate to speak informally to the person whom the employee thinks is responsible for the behaviour to make it clear to their harasser that the behaviour is unacceptable and must stop.
2. The employee should make notes summarising the date and content of the discussion. If an employee is unable to do this verbally then a written request (explaining the distress that the behaviour is causing) handed to the harasser may be effective. The person named above can assist employees in taking such action.

FORMAL PROCEDURE

1. Where informal methods fail and the behaviour persists or concerns a specific incident, the employee must report the matter to their line manager.
2. If the employees' line manager is the alleged perpetrator/harasser, the member of staff should report the matter to their line manager's manager.
3. In some cases, the employee may feel more comfortable speaking to another line manager. A formal grievance should be raised and the employee is advised to seek assistance, as above, in doing so. The complaint should be made in writing, and where possible, state:
 1. The name of the alleged perpetrator/harasser;
 2. The nature of the harassment;
 3. Date(s) and time(s) when harassment occurred;
 4. Name(s) of witness(es) to any incidents of harassment;
 5. Any action already taken by the complainant to stop the harassment.
 6. The complaint/grievance should be sent, in confidence, to the Registered Provider.
1. Immediately after a complaint of harassment has been received, action will be taken to separate the harasser from the complainant; this may involve temporary transfer of the alleged harasser to another department, or suspension with pay until the complaint has been resolved.
2. The senior manager handling the complaint will carry out a thorough investigation as quickly as possible to establish whether or not there is a case to answer, maintaining confidentiality at all times. All employees involved in the investigation are expected to respect the need for confidentiality. Failure to do so will be considered a disciplinary offence and may lead to disciplinary proceedings under the organisation's Disciplinary and Grievance procedures.
3. Copies of statements made by witnesses will be made available to both the alleged perpetrator/harasser and the complainant. Witnesses will be encouraged to appear at the complaint hearing if requested by either party. It is acknowledged that some witnesses may be reluctant to do so. In these circumstances the manager will, if necessary, adjourn the hearing in order to ask supplementary questions of witnesses in private.
4. The complainant may, if he or she wishes, be supported throughout the procedure and hearing by a work colleague. Alternatively, he or she may be supported by certified Trade Union Representative or employed Trade Union Official.
5. The employee accused of harassment will also have the equivalent right to be accompanied at the hearing. Where the manager concludes that harassment has taken place, he or she will ensure that the perpetrator has every opportunity to defend or explain his or her actions, in accordance with the organisation's disciplinary procedure.
6. The severity of the penalty imposed upon an employee against whom an allegation of harassment has been proven will be consistent with those detailed in the disciplinary procedure (e.g. gross sexual harassment will normally result in summary dismissal).
7. Where a lesser penalty is appropriate (e.g. a written warning) this may be coupled with action to ensure that the victim is able to continue working without embarrassment or anxiety. After discussion with the victim, the manager may order the transfer of the harasser to a different work area, or arrange for the amendment of working schedules or rotas to minimise contact between the two employees. If the alleged victim so wishes, his or her own transfer may be arranged, subject to practical limitations. The result of the hearing will be confirmed in writing to both employees.
8. If the complainant is not satisfied about the way in which his or her complaint has been handled, he or she may ask for it to be reconsidered by the Registered Provider under the relevant stage of the organisation's grievance procedure.

9. Requests for reconsideration of the complaint should be made within five working days of the first hearing. The decision of this second hearing will be sent, in writing, to both parties and will be final.
10. An employee who receives a warning or is dismissed for harassment may appeal against the penalty in accordance with the organisation's appeals procedure.
11. An employee who brings a complaint of harassment will not suffer victimisation for having brought the complaint. However if the complaint is found to be vexatious, frivolous and untrue and has been made in bad faith, then disciplinary action may be taken against the complainant.
12. For further details of procedures, please refer to the organisation's Disciplinary and Grievance procedures.

Reviewed by Marcus Kerridge McColl
December 2025

Hate Crime Policy & Procedure
Crescent Community Care Services Limited

Purpose

1. To comply with the Criminal Justice Act 2003 & Crime & Disorder Act 1998

Scope

2. All Employees
3. All Service Users
4. Any person who has any forms of dealing with the Service
5. Any person targeted criminal offence that is potentially a hate crime

Policy

1. The definition of a hate crime is a crime whereby the perpetrator targets the victim due to his / her racial group, religion, sexual orientation. Disability, ethnicity, class, nationality, age, gender, gender identity or political affiliation.
2. The Act compels a Court to consider certain issues when considering a sentence
3. These issues are
 1. That at the time of committing the offence, or immediately before or after doing so the offender demonstrated towards the victim of the offence hostility based on
 1. The sexual orientation (or presumed sexual orientation) of the victim
 2. A disability (or presumed disability) of the victim
 2. That the offence is motivated (wholly or partly)
 1. By hostility towards persons who are of a particular sexual orientation
 2. By hostility towards a person who has a disability or a particular disability
4. Crescent Community Care Services Limited will follow Policies and Procedures that are intended to identify hate crimes and report them when they occur, and will train all workers to avoid and identify these incidents

Procedure

1. Policies to be followed are
 1. Anti Bullying Policy & Procedure
 2. Harassment Policy & Procedure



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2. In any incident where bullying or harassment is observed which involves verbal abuse, physical abuse, damage to property or any combination of these, then the incident must be reported to the Registered Manager or Directors immediately
3. If the incident has any features of a hate crime then this behaviour must be clearly documented in a report
4. The Registered Manager or Directors will review the information and decide within the guidelines of the Policies & Procedures and consider the reporting of the case to the Authorities for consideration of prosecution

Reviewed Marcus Kerridge-McColl
October 2025

Hazard Reporting Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To comply with statutes, regulations and quality standards.

Scope

2. All establishments.

Policy

Introduction

3. We recognise that staff members have an invaluable monitoring role within the workplace in helping to identify hazards (see Hazard Reporting and Recording Guidance) before they cause an injury or accident. In addition, staff members also have a legal duty to report conditions that might cause harm.

It is our policy to:

4. Have an effective system for the reporting of hazards found by staff in their workplace. §
5. Ensure all reported hazards are dealt with expediently and efficiently.
6. Check that action has been taken following receipt of a hazard report.
7. Train staff to verbally report the following circumstances immediately:
 1. Discovery of a fire;
 2. Ineffective, defective or missing guards;
 3. Damaged or ineffective personal protective equipment or clothing;
 4. Faulty equipment that cannot be operated safely;
 5. Insufficient training or information to carry out your work safely;
 6. Insufficient information on the use and handling of a hazardous substance;
 7. Spillage of a hazardous substance;
 8. Potential incident or dangerous occurrence;
 9. Complete check-lists for hazard spotting at prescribed intervals.

Procedure

1. Locate Hazard Record forms in prominent accessible areas.
2. Inform employees of the Hazard Records locations and how to complete them.
3. Employee identifies hazard.
 1. Employee contacts Supervisor immediately, and employee completes form.
 2. Supervisor decides on action to take.
 3. Senior Manager monitors reports and actions.

Reviewed by Marcus Kerridge McColl
November 2025

Health & Safety Policy and Procedure

Crescent Community Care Services Limited

Purpose

1. To comply with statutes, regulations and quality standards.
2. The purpose of this policy is to ensure that the organisation, its employees and others experience a safe environment, and that statutory obligations are met.

Scope

1. This policy applies to all employees, all Service Users and all visitors to the premises of Crescent Community Care Services Ltd, and any premises in which their employees work.

Policy

2. **Crescent Community Care Services Ltd** recognises that they have a responsibility to ensure that reasonable precautions are taken to provide and maintain working conditions which are safe, healthy and comply with all statutory requirements and codes of practice relating to the organisation's particular activities.
3. **Crescent Community Care Services Ltd** will, so far as is reasonably practicable, pay particular attention to:
 1. The provision and maintenance of plans and systems of work that are safe and healthy.
 2. Arrangements for ensuring safety and the absence of risks to health in connection with the use, handling, storage and transport of articles and substances.
 3. The provision of information, instruction, training and supervision such as to ensure the health and safety at work of employees and others.
 4. The control of the place of work maintaining it in a safe condition.
 5. The provision of a safe means of access to and egress from the place of work.
1. This policy will be reviewed at least annually.

Procedure

Organisation and Responsibilities

2. **Executive**
 1. The Registered Provider is responsible for safety in Crescent Community Care Services Ltd and will monitor the safety policy on a regular basis.
 2. The Registered Provider will be sufficiently apprised of health and safety matters to ensure that sufficient resources are available to provide any health and safety equipment, clothing, information and training for employees in order (as far as is reasonably practicable) to achieve and maintain a high standard of safety proficiency.
3. **Safety Officer**
 1. The Safety Officer is the Registered Manager, unless indicated otherwise by a notice on the main staff notice board of the establishment.
 2. The responsibilities of the Safety Officer are to:
 1. Maintain safety records; ☒
 2. Investigate accidents;
 3. Provide accident statistics;
 4. Keep a watching brief on changing safety legislation.
 1. The Safety Officer reports directly to the Registered Provider.

2. Full investigations of accidents will be carried out by the Safety Officer with a view to the prevention of future occurrences.
3. The Safety Officer is responsible for ensuring that the organisation's obligations with respect to assessment, control and monitoring of hazardous substances are met.
4. The Safety Officer is responsible for recording of accidents in accordance with RIDDOR (Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013), by:
 1. Ensuring that the Accident Reporting Policy and Procedure is followed, and that all accidents are recorded, using the form attached to that policy;
 2. Ensuring that all accidents which result in absence from work for more than seven days (not including the day of the accident) are reported within 10 days to the Health and Safety Executive.

RIDDOR 2013 reporting requirements

3. Reportable incidents under RIDDOR 2013 are:
 1. Death and injuries where:
 1. The accident is work-related;
 2. It results in an injury of a type which is reportable.
 3. The types of reportable injuries are:
 1. Death;
 2. Specified injuries, which are defined by the HSE as:
 1. Fractures, other than to fingers, thumbs and toes;
 2. Amputations;
 3. Any injury likely to lead to permanent loss of sight or reduction in sight;
 4. Any crush injury to the head or torso causing damage to the brain or internal organs; Æ
 5. Serious burns (including scalding) which:
 1. Causes significant damage to the eyes, respiratory system or other vital organs;
 2. Covers more than 10% of the body.
 6. Any scalping requiring hospital treatment;
 7. Any loss of consciousness caused by head injury or asphyxia;
 8. Any other injury arising from working in an enclosed space which:
 9. Leads to hypothermia or heat-induced illness;
 10. Requires resuscitation or admittance to hospital for more than 24 hours.
11. The following occupational diseases are reportable:
 1. Carpal tunnel syndrome;
 2. Severe cramp of the hand or forearm;
 3. Occupational dermatitis;
 4. Hand-arm vibration syndrome;
 5. Occupational asthma;
 6. Tendonitis or tenosynovitis of the hand or forearm;
 7. Any occupational cancer;
 8. Any disease attributed to an occupational exposure to a biological agent.

9. Dangerous occurrences require reporting, for example:
 1. The collapse, overturning or failure of load-bearing parts of lifts and lifting equipment;
 2. Plant or equipment coming into contact with overhead power lines;
 3. The accidental release of any substance which could cause injury to any person.
4. For full details of reporting requirements, go to the HSE web site at: <http://www.hse.gov.uk/riddor/reportable-incidents.htm>
5. Contacts for the HSE, including for online reporting are at: <http://www.hse.gov.uk/contact/contact.htm>

Department Manager (where this differs from the Registered Manager).

1. Where there are no designated Department Managers, the Registered Manager fulfils this role.
2. Department Managers have the responsibility to provide leadership and to promote responsible attitudes towards health and safety.
3. Each manager will:
 1. Ensure that each new employee is given induction training, including the precautions and procedures appropriate to their specific jobs. All new members of staff will be shown the location of first aid boxes, fire exits and firefighting equipment;
 2. Ensure that all subordinates are aware of the health and safety policy (a copy is on the organisation notice boards);
 3. Keep up to date with health and safety matters applicable to the operations of the organisation;
 4. Investigate all accidents with the assistance of the Safety Officer, with a view to prevention of a further occurrence;
 5. Ensure that good housekeeping standards are applied;
 6. Review periodically all new and existing equipment with reference to mechanical and operational safety and, in particular, the location of all equipment bearing in mind all health and safety factors;
 7. Carry out regular safety checks and audits.

Supervisors

1. Supervisors have the responsibility to provide leadership and to promote responsible attitudes towards health and safety. Supervisors must ensure that all tasks carried out in their sections are performed with the utmost regard for the health and safety of all those involved.
2. Accidents must be reported immediately to the Department Manager or Registered Manager.
3. Particular regard will be paid to:
 1. Equipment and its usage to ensure that they are safe and do not endanger health;
 2. Provision of safety arrangements for the handling, storage and movement of materials, equipment and substances;
 3. Supplying sufficient information, instructions, training and supervision such as to enable employees to avoid hazards and contribute positively to their own health and safety at work;
 4. Inspecting, on a regular basis, equipment such as lighting, passageways, fire alarms, fire escapes, fire extinguishers, first aid facilities and work practices, in order to ensure their efficiency and maintenance.

Employees

1. All employees have a responsibility to do everything they can to prevent injury to themselves, their fellow employees and others affected by their actions or omissions at work.
2. They are expected to follow company procedures in particular, to report any incidents which have or may have led to injury or damage. To neglect this responsibility can lead to prosecution by the Health and Safety Executive.
3. Any employee who is faced with a conflict between the demands of safety and their job should raise the matter immediately with the Supervisor.

Administrative Arrangements

1. The following statements are an overview, and most areas for health and safety management are amplified by further policies and procedures elsewhere in this management system.

2. **Risk Assessment**

1. The Safety Officer will regularly review all areas in use by the organisation, or in which its workers work, to:

1. Identify risks;
2. Assess the risk;
3. Evaluate the risk;
4. Eliminate the risk where appropriate;
5. Introduce control measures to reduce risks, to a reasonable level, where appropriate;
6. Develop or locate, and arrange delivery of appropriate training to reduce risk, to a reasonable level, where appropriate.

7. The Safety Officer will carry out a generic risk assessment whenever workers:

1. Begin work in a new area;
2. Begin work in a new building, or building type, in an existing area;
3. Have or raise an issue in an area or building which they are already working in.

4. The Safety Officer will carry out a risk assessment on new equipment brought into the establishment. §

1. **Reporting Accidents**

1. In the event of an accident causing injury you must ensure that the injured person is being cared for, and send immediately for a supervisor or first-aider.
2. DO NOT MOVE THE INJURED PERSON.
3. Report the full details to the Department Manager who will record the incident in the accident book.
4. The record will be regularly inspected by the Safety Officer. The accident will be reported to the inspecting authority as and when necessary.
5. Any “near miss” incident which occurs should also be reported to your immediate supervisor who will be responsible for making a report to the Department Manager.
6. All accidents will be investigated by the Department Manager and the Safety Officer.
7. A report will be made to the Registered Provider, via the Management Meeting, who will ensure that necessary action is taken to prevent recurrence.

8. **First Aid**

1. During the induction programme employees will be shown the location of the nearest first aid box to their work area.
2. The organisation will ensure that sufficient employees are trained as first aid specialists to provide coverage on all shifts.



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3. The identity of designated first aid specialists will be noted by clear notices complying with the recommended format displayed at all work stations and staff areas throughout the establishment.
4. **Fire**
 1. Fire exits must be kept clear from obstruction.
 2. All employees must know their evacuation route and assembly point in case of fire.
3. **IF YOU DISCOVER FIRE:**
 1. Immediately operate the nearest fire alarm call point.
 2. WITHOUT INCREASING PERSONAL RISK, try to put out the fire, if possible, with the nearest appropriate fire appliance provided, by directing the hose or extinguisher to the base of the flame.
1. **IF YOU HEAR THE FIRE ALARM:**
 1. The senior person on duty will be responsible for calling the Fire Services.
 2. Report immediately to the staff assembly point, which is by the fire alarm panel in your building.
 3. **DO NOT USE THE LIFT.**
 4. **DO NOT DELAY FOR PERSONAL BELONGINGS.**
 5. Follow the instructions of the person in charge, who is fully in charge of all staff and persons on the premises until the Fire Brigade arrives.
 6. If told to leave the building, do not re-enter the building until instructed by your senior supervisor or the Fire Brigade.

Organisation Code of Safe Practice 5

7. **Good Housekeeping – General**
 1. Undue hurrying and forgetfulness cause many accidents. Do not run down steps. Use hand rails going up or down stairs.
 2. Watch out for someone coming round a blind corner or opening doors quickly.
 3. Never read while walking.
 4. Ensure that floor areas are well lit and kept clear of obstruction.
 5. Where floors are wet through spillages or cleaning, the area must be protected using a recognisable wet floor sign until the area has dried. The sign must be removed to storage as soon as possible after the area has dried.
6. **Good Housekeeping – Offices**
 1. Leaving a lower filing drawer open causes many trips and falls. Please make sure they are closed.
 2. Electrical, computer and telephone cords must not be allowed to lie uncovered on the floor and should be taped down, since they are major tripping hazards.
 3. Spilled coffee or soft drinks, tracked-in rain, leaves or snow, should be cleaned up immediately.
 4. Pointed objects such as pencils, pens, letter openers, files and the like must be used carefully to avoid puncture wounds.
 5. Horseplay, including throwing paper clips, shooting rubber bands, tossing objects out of windows, is unacceptable behaviour, and may be the subject of disciplinary procedure.
6. **Electrical Equipment**
 1. Electrical equipment is normally safe, provided it is properly installed and regularly inspected.



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2. Always remember that water and liquids are conductors of electricity, and be aware that their association with faults caused by, for example, damaged cables, flexes, plugs and sockets, the overloading of circuits and fuses, etc. would make the shock more severe.
3. Therefore you should:
 1. NEVER touch electrical equipment with wet hands, move any portable electrical equipment without disconnecting it from the mains, make electrical repairs or do other electrical work unless you are an authorised person;
 2. KEEP electrical supply cables and flexes away from wet areas or from where they will be damaged by being walked over or knocked when moving equipment about;
 3. ALWAYS switch off all equipment when not required, unless continuous operation is necessary;
 4. Disconnect electrical equipment at night by removing the plug from the socket, again unless continuous operation is necessary and/or instructed otherwise;
 5. Report defective equipment to the Registered Manager.
6. **Moving and Handling**
 1. Staff must not carry out moving and handling operations unless the operation has been assessed for risk, an opinion has been formed and recorded by an appropriate person, a recommended handling technique identified, and the technique communicated to all staff.
 2. Moving and handling form part of the induction training where general guidelines are given on the prevention of back injury and the importance of risk assessment of both individual lifting/handling operations and environmental consideration.
 3. Staff who find themselves alone with a Service User should never attempt to lift/move a Service User who has been assessed as requiring two people to perform such an operation. Advice should be sought immediately and the Service User should be made comfortable/safe until assistance arrives.
4. **Basic Food Hygiene**
 1. All employees who have contact with food in the establishment, or enter food preparation areas, will be suitably trained in basic food hygiene.
 2. Basic food hygiene training is incorporated in the induction training for all employees.
 3. Employees normally working in food preparation will complete a recognised Basic Food Hygiene qualification as soon as practicable after initial employment, or produce proof of a recent qualification.
 4. Catering supervisors and cooks will complete the Intermediate Food Hygiene Certificate as soon as practicable after initial employment, or produce proof of a recent qualification.
5. **Transmittable Diseases**
 1. Transmittable diseases form part of the induction training.
 2. When performing hands-on personal Care with Service Users, full protective measures (gloves, aprons etc) should be taken in order to eliminate any risk of cross-infection.
3. **Hot Water Bottles**
 1. Hot water bottles should not be used except in exceptional circumstances and only if the Service User insists.
 2. Hot water bottles must never be used on Service Users with dementia.
 3. If a bottle must be used, follow the procedure below:
 4. Hot water bottles may be used for Service User comfort, but will be covered by a protective cover and filled in a manner which is safe for staff, and eliminates the risk of scalding in the event of failure of the bottle.
 5. Check that the hot water bottle has a protective cover over the bare rubber inner.



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6. Mix water to the highest temperature which it is possible to bear on bare skin in a jug. Pour that water into the bottle and seal.
7. Press the bottle and check the stopper for leakage.
8. When placing the bottle against the Service User, wait long enough for the Service User to report that the temperature and position is comfortable before leaving.
9. **Infection Control**
 1. Infection control training is incorporated in induction training.
2. **Prevention of cross-infection**
 3. To prevent cross-infection, ensure that:
 1. Any infection a Service User has does not spread to others;
 2. The Service User is not exposed to potential sources of infection in his/her surroundings; &E
 3. Others do not bring infection to The Agency.
4. **Notifiable diseases**
 1. The Health Services and Public Health Act 1968, the Public Health (infectious Diseases) Regulations 1988 and subsequent amendments require certain infectious diseases to be notified to the 'proper officer' of the Local Authority.
 2. The responsibility for the notification of the listed disease(s) rests with the Doctor attending the Service User.
 3. The Local Authority has the power to stop work in order to prevent the spread of infection, including food borne infections (Food Hygiene (General) Regulations 1970).
 1. Diseases notifiable under the Public Health (Control of Disease) Act 1984: Cholera, Plague, Smallpox, Relapsing Fever, Typhus, and Food Poisoning.
 2. Diseases notifiable under the Public Health (Infectious Diseases) Regulations 1988: Acute encephalitis, acute poliomyelitis, anthrax, diphtheria, dysentery, leprosy, leptospirosis, malaria, measles, meningitis, meningococcal, septicaemia, mumps, ophthalmia, neonatorum, paratyphoid fever, rabies, rubella, scarlet fever, tetanus, tuberculosis, typhoid fever, viral haemorrhagic fever, viral hepatitis, whooping cough and yellow fever.
 3. Most outbreaks will present non-specific symptoms; serious sepsis or epidemic wound infections.
 4. Any member of staff suspecting an outbreak of the notifiable disease should make their suspicions known to the Registered Manager who will inform the appropriate Doctor.
5. **Rules to prevent the spread of infection**
 1. All staff should adhere to the following:
 1. Avoid infection by careful control of coughing and sneezing, i.e. use tissues / handkerchief; &E
 2. Appropriate hand washing;
 3. Use of disinfecting hand rubs;
 4. Avoid wearing jewellery, except for wedding rings;
 5. Keep hair short or tied back;
 6. Wear clean uniform clothing, and do not travel to and from work in that clothing;
 7. Report any signs of infection to the appropriate person;
 8. Keep toilets and commodes scrupulously clean using correct disinfectant agents; &E
 9. Correct handling of food to prevent food borne illness;
 10. Take Care when dealing with pets. Always hand wash or use the hand rub after contact. Š
11. **Staff skin awareness**
 1. All cuts and abrasions should be covered with a waterproof plaster (blue coloured if working in food areas). Early detection and prompt reporting of infection is particularly important.

2. Any staff member with a skin infection must take advice from a doctor before continuing to work. All skin infections must be reported to the Registered Manager.
3. **Staff sickness**
 1. Staff with diarrhoea and vomiting should not attend work but ring to report sick. Should the condition persist it may be necessary to provide a specimen of faeces and not return to work until medical clearance by a GP is given. It cannot be emphasised strongly enough that young children and the elderly are particularly vulnerable to infection, and every attempt should be made to minimise any risk of infection.
2. **Skin Infections**
 1. Report to your manager any Service Users who have a rash or unaccountable marks on his/her body.
 2. Where scabies or shingles are suspected:
 1. The manager must request a visit from the GP;
 2. Staff should wear a plastic apron and wear gloves for any direct contact;
 3. All linen must be placed in the appropriate bag and the appropriate laundry procedure followed for contaminated laundry.
4. **Blood borne viruses**
 1. Any Service User may be a carrier of a blood borne virus. There are blood-borne viruses other than hepatitis B, other hepatitis and HIV/AIDS. Appropriate precautions must therefore be taken with all Service Users and particularly with body fluids.
 2. Always assume that blood and other body fluids are infected. All accidents, facial, particularly eye, or wound contact with infected body fluids must be recorded as an incident.
 3. Accident avoidance measures should include common sense precautions to avoid accidents and injuries, particularly when using sharps, whether the Service User is known to be infected or not. All accidents must be reported.
 4. Body fluid handling and spillage procedure should be as follows:
 1. Use no-touch techniques when dealing with blood or other body fluids. Wear gloves and plastic aprons as appropriate. Masks and goggles are not normally needed;
 2. Care staff wearing disposable gloves and plastic aprons should wipe up body fluid spillages immediately;
 3. Use appropriate disinfectant agents on carpets;
 4. Use no-touch techniques or gloves when disposing of anything contaminated with blood, e.g. dressings.
 1. Avoid contamination with saliva. If saliva contamination to eyes, a cut or an open wound occurs, wash liberally with water and inform the manager immediately.
2. **Outbreak control measures**
 3. An outbreak of gastroenteritis is indicated by the occurrence of UNEXPLAINED diarrhoea and/or vomiting in two or more Service Users. (Remember that there are also non-infective causes of diarrhoea and vomiting). The recommended action in such cases is as follows:
 1. Staff should inform the manager who should then contact the appropriate GPs;
 2. A specimen of faeces should be made available for testing, if required;
 3. Wear plastic apron and protective gloves when in contact with excreta;
 4. Dispose of faeces carefully and disinfect bedpans/commodes using disinfectant;
 5. If possible, place the Service User in a single room, with their own toilet facilities such as a commode;
 6. Any Service User with, or suspected of having, gastroenteritis should have their own sink/bowl for washing;



7. All crockery and cutlery should be soaked in a bowl of disinfectant for 30 minutes before being removed from the room of the Service User to the kitchen;
8. Place all contaminated linen into a coloured bag and keep separate from any other linen;
9. Wear a protective apron and gloves when sluicing contaminated linen. To sluice any contaminated linen, leave the linen to soak in disinfectant for 30 minutes before removing to laundry;
10. Wash hands thoroughly after attending the Service User and before going to any other task.
11. The manager should notify the local health authorities when the occurrences are unexplained. §
12. **Emergency Situations**
13. In case of being faced with emergency situations such as relating to gas, electricity, water, fire or medical issues, stay calm, assess the situation, and raise alarm by contacting 999, depending on the emergency. Emergency situations will form part of your induction programme.
14. **Major injuries**
1. Fracture of the skull, pelvis and any bone in the arm or leg, but not bones in the hand or foot.
2. Amputation of a hand or foot or of fingers, thumbs or toes where the bone or a joint is completely severed.
3. Loss of sight in an eye or a penetrating injury or a chemical or hot metal burn to an eye.
4. Injury requiring medical treatment or loss of consciousness due to electric shock.
5. Loss of consciousness due to lack of oxygen.
6. Decompression sickness.
7. Acute illness believed to be the result of exposure to a pathogen or infected materials.
8. Any other injury that results in the person being admitted to hospital for more than 24 hours.
9. Any incident in which a dangerous substance being conveyed by road and involved in a fire or where there is an uncontrolled release or escape of dangerous substances.
10. Any incident whereby breathing apparatus malfunctions in such a way as to deprive the wearer of oxygen.
11. Any incident in which plant or equipment comes into contact with overhead power lines exceeding 200 volts.
12. Prescribed diseases and certain poisoning.
13. Some skin diseases including: occupational asthma, farmers lung, pneumoconiosis, asbestosis and mesothelioma.
14. The following infections: leptospirosis, hepatitis, tuberculosis, and anthrax, any illness caused by a pathogen.
15. **COSHH**
1. COSHH forms part of your induction training and are incorporated into the individual Service User accommodation risk assessment, this forms part of the Service Users Care Plan.
2. For the purpose of COSHH, a substance is considered as hazardous if one or more of the following criteria are met:
 1. Substances listed as very toxic, harmful, corrosive or irritant;
 2. Substances for which maximum exposure limit (MEL) is specified in the COSHH schedule; ☒
 3. A micro-organism hazardous to health;
 4. Substances airborne as concentrations of dust;
 5. Any other substances, which create comparable hazards.
6. **Safety Rules for the use of household cleaning agents**
1. Handle all household cleaning agents with Care. Remember they contain powerful chemicals.

2. Always wear protective clothing (overalls, rubber gloves).
3. Always read the instructions on the label of the product to be used.
4. If unsure of the product or it is thought that the chemical is in the wrong container, DO NOT USE.
5. NEVER MIX chemicals, especially bleach and toilet cleaner.
6. Make sure that the ventilation is adequate. DO NOT use chemicals in a confined space.
7. NEVER SMOKE whilst using chemicals. Smoking is not permitted in the homes of Service Users.
8. Store all chemicals in a cool dry place after use.
9. Store all chemicals out of reach of children but not on high shelves. Keep away from heat.
10. NEVER place chemicals in other containers. If a container is broken, discard it with its contents.
11. AEROSOLS must be:
 1. Kept away from heat;
 2. Never punctured;
 3. Never used near a naked flame or heat;
 4. Avoid breathing the vapour - used in a well ventilated room.
1. Be careful when throwing away chemicals. Be sure they are in a safe condition and that no one else will be harmed by them. Never throw away metal scouring pads with discarded batteries – they can smoulder and cause a fire.
1. **IF AFTER USING HOUSEHOLD CHEMICALS WITHIN THE WORKPLACE A FEELING OF DROWSINESS OR OF BEING GENERALLY UNWELL DEVELOPS, CONTACT YOUR DOCTOR IMMEDIATELY AND THEN INFORM THE MANAGER/CARE CO-ORDINATOR.**
2. **Safe systems of work**
3. To help give a better picture regarding the health and safety of employees in the work place, a list of the common areas where risks and hazards occur is shown on the following forms. It shows the areas/appliances that may present a hazard or risk, the types of accident/injury they may cause and the appropriate action that should be taken by Care staff.

Working at Heights/Reaching etc:

1. Avoid working at height where possible.
2. Use work equipment or other measures to prevent falls where they cannot avoid working at height.
3. Where they cannot eliminate the risk of a fall, use work equipment or other measures to minimise the distance and consequences of a fall, should one occur.
4. Risk-assess all situations whereby working at heights is unavoidable.
5. Provide suitable training for those working at heights.
6. Provide suitable equipment.

Employees and other workers on site will:

1. Not work at heights without ensuring that the Registered Manager has authorised the action, after carrying out a risk assessment, and the employee has been trained to work at height and has appropriate equipment for doing so.
2. Not attempt to obtain items which are beyond your reach. If you cannot reach – get a ladder or stepping stool. Be sure that the ladder is in a safe condition.
1. Do not use chairs, open drawers, or any makeshift device for climbing.

2. Do not climb up the shelves themselves. Do not overreach on the ladder. It is safer to get down and move the ladder.

Smoking

1. Smoking is only allowed in designated areas.

Floors

1. Floors must be kept free of obstruction.
2. Spillages of fluids must be immediately mopped up, and wet floors clearly marked.
3. Damage to floors must be reported immediately.

Stairs

1. Stairs must be kept clear of obstruction.
2. Inflammable materials will not be stored in a stairwell.
3. Damage to stairs must be reported immediately.

Lighting

1. Non-functioning lighting must be reported immediately.

Doors

2. Doors must not be obstructed from closing.
3. Damage to fire doors must be reported immediately.

Lifts

1. Damage to, or malfunction of, lifts must be reported immediately.
2. Lifts will not be used during a fire alarm.

Gas

1. Damage to gas installations, or a smell of gas, must be reported immediately.
2. Cases of headache, unusual tiredness and muscular weakness experienced in rooms containing a gas appliance must be reported immediately.

Health and Safety: Grievance Procedure

1. This procedure relates only to occupational health and safety problems, disputes or grievances.
2. In the event of the above, employees should either orally or in writing refer the matter to the Department Manager.
3. If employees are dissatisfied with the outcome or in the event of there being a real danger of death, serious injury or health risk and there is insufficient time to eliminate excessive danger, staff should immediately report to the Registered Provider who will investigate and determine what action should be taken.
4. After the investigation, the employee will be informed that either:
 1. The organisation has so far as reasonably practicable eliminated the danger and employees must resume normal working; or
 2. The organisation does not consider that the matter constitutes a grave risk to health or safety, and employees must resume normal working; or

3. The organisation will undertake further investigation and may, if necessary, obtain expert opinion.
1. Employees will then be suspended on full pay or be transferred to alternative work whilst the investigation takes place.
2. Refusal to resume normal working when instructed will be a breach of organisation discipline. The matter will then be dealt with under the organisation's normal disciplinary procedure.

Reviewed by Marcus Kerridge McColl
November 2025

Health and Safety Training Policy and Procedure
Crescent Community Care Services

Purpose

1. To comply with statutes, regulations and quality standards.

Scope

2. All establishments.

Policy

Introduction

3. Training is a vital part of our strategy to effectively manage health and safety issues within our business. When carried out effectively, it can change our staff's perceptions of risks and result in significant improvements in health and safety performance. So preparing our staff to work safely and reducing accidents and damage to our premises and equipment is important. It is also a general factor in motivating staff, so that improvements are often found in overall commitment and work performance, and ensures that staff members are competent and confident when carrying out their work. It is our legal responsibility to provide adequate Health and Safety Training (see Guidance).
4. It is our policy to:
 1. Identify the health and safety training needs associated with our work activities.
 2. Provide the following health and safety training for our staff:
 1. Induction training for new starters;
 2. Training on our health and safety policies and procedures;
 3. Work activity training relevant to the member of staff, including the use of any equipment; Æ
 4. Training required by specific legislation;
 5. Training on fire and emergency procedures including alarm raising;
 6. Training on the recognition, handling and use of hazardous substances;
 7. Awareness training for management staff;
 8. Refresher training where identified in our training needs analysis.
 1. Keep records (see training records) of all staff training and related documents;
 2. Ensure staff are aware of their legal obligation to co-operate and to put into practice any new instruction or guidance given.

Procedure

1. Establish the work content of a job.
2. Specify the level of competence required for each task. Consider the individual characteristics which may be needed.
3. Assess current competence of individual staff against this analysis.
4. Identify staff training needs.

5. Identify and if necessary train trainers for each topic. §
6. Identify external training providers.
7. Add trainers to approved trainer list.
8. Develop training packages if necessary.
9. Arrange training dates.
10. Deliver training.
11. Assess effectiveness of training.
12. Amend training and redeliver if necessary.
13. Review training needs to ensure that all areas are covered.

Reviewed by Marcus Kerridge McColl
November 2025

Human Rights Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To comply with statutory obligations and best practices on human rights.

Scope

2. All activities of Crescent Community Care Services Ltd.

Policy

3. Crescent Community Care Services Ltd will endeavour to comply with best practices in the protection of the human rights of its Service Users and its employees.
4. The QCS compliance management system was originally developed in the early 1990s, when Homes for Living in was first published, and the whole system was designed to uphold the principles of HAFLI:
 1. Privacy;
 2. Dignity;
 3. Independence;
 4. Rights;
 5. Fulfilment;
 6. Choice.
7. These principles were also incorporated into the Care sector QCF Diploma awards also developed at that time.
8. A guiding principle of the development of the QCS system, and its subsequent development, was a recognition of research, which indicated that carers who felt that they lacked control over their employed time and actions, due to their rights being infringed by their employer, would attempt to regain that control by denying the rights of the Service Users whom they were supporting. The system was therefore designed to apply the same principles to employees as the employer expected the employees to apply to the Service User.
9. Subsequently the Human Rights Act 1998 codified the principles of human rights. This however did not change the importance of the six principles as applied to care and support of vulnerable people, but in some instances extended or defined the principles.

10. The QCS system has been developed and extended to include the specific requirements of the Human Rights Act within the Care sector.

Procedure

1. The QCS policies which support human rights are (main items only):
 1. Service Users' Handbook;
 2. Witnessing of Documents Policy and Procedure;
 3. Access to NHS Services Policy and Procedure;
 4. Deprivation of Liberty Safeguards Policy and Procedure;
 5. Mental Capacity Act 2005 Policy and Procedure;
 6. Mental Health Regulations 1983 Policy and Procedure;
 7. Participation Policy and Procedure;
 8. Principles of customer Care / philosophy of care;
 9. Privacy Policy and Procedure;
 10. Restraint Policy and Procedure;
 11. Risk Taking Policy and Procedure;
 12. Advocacy Policy and Procedure;
 13. Safeguarding Policy and Procedure;
 14. Choice Policy and Procedure;
 15. Confidentiality Policy and Procedure;
 16. Consent to Examination or Treatment Policy and Procedure;
 17. Harassment Policy and Procedure;
 18. Equality and Diversity Policy and Procedure;
 19. Anti-Bullying Policy and Procedure;
 20. Complaints, Suggestions and Compliments Policy and Procedure.
21. The Articles of the Human Rights Act are:
 1. Article 1: This article is introductory and not included in the Human Rights Act;
 2. Article 2: Right to life;
 3. Article 3: Prohibition of torture;
 4. Article 4: Prohibition of slavery and forced labour;
 5. Article 5: Right to liberty and security;
 6. Article 6: Right to a fair trial;
 7. Article 7: No punishment without law;
 8. Article 8: Right to respect for private and family life;
 9. Article 9: Freedom of thought, conscience and religion;
 10. Article 10: Freedom of expression;
 11. Article 11: Freedom of assembly and association;
 12. Article 12: Right to marry;
 13. Article 13: This is not included in the Human Rights Act;
 14. Article 14: Prohibition of discrimination.
15. A more detailed explanation of these Rights is available on the website below:
<http://www.equalityhumanrights.com/your-rights>
16. Of particular interest in a Care setting are:
 1. Article 8: Right to respect for private and family life.
 1. This Right states that everyone has a right to respect for their private and family life, their home and correspondence. People have the right to live their own life and have personal privacy.
 1. Article 9: Freedom of thought, conscience and religion.
 2. This Right protects people's rights in relation to their thoughts and beliefs. The state is not permitted to interfere with a person's right to hold a particular belief.
 3. Article 10: Freedom of expression.

4. This Right states that everyone has the right to express their views and receive opinions without interference from a public authority.
5. Article 14: Prohibition of discrimination.
6. This Right states that discrimination cannot occur on the grounds of particular attributes such as a person's sex, race, or religion. It also protects people from discrimination on the grounds of, for example, disability or marital status. A public authority should ensure policies and decisions do not involve any form of discrimination on any ground.

Reviewed by Marcus Kerridge McColl
November 2025

Illegal Possession of Drugs on Premises Policy and Procedure Crescent Community Care Services Limited

Purpose

1. To guide employees and Service Users in dealing with illegal drugs, while protecting Service User confidentiality and the obligations of Crescent Community Care Services Ltd.

Scope

2. All workers, Service Users and visitors.

Policy

3. Crescent Community Care Services Ltd will proactively manage issues arising from the use of illegal drugs on clients premises.

Legal Framework

4. The principal legislation covering the misuse of Controlled Drugs in the UK is the Misuse of Drugs Act 1971. The Act makes unlawful the production, supply and possession of drugs subject to control, except in certain permitted circumstances, such as when prescribed by a doctor.
5. Section 8 of the Act states that: 'a person commits an offence if, being the occupier or concerned in the management of any premises, he knowingly permits or suffers any of the following activities to take place on his premises...'
 1. Producing or attempting to produce a Controlled Drug...
 2. Supplying or attempting to supply a Controlled Drug...
 3. Preparing opium for smoking...
 4. Smoking cannabis, cannabis resin or prepared opium...
1. The intention is to ensure that occupiers and managers of premises are vigilant to ensure that Controlled Drugs are not misused on their premises. If illegal drug use by Service Users, visitors, or staff is known to be taking place on Crescent Community Care Services Ltd's premises, failure to inform and co-operate with the police could expose managers to prosecution.

Procedure

2. **Known or suspected possession or use of illegal drugs by a Service User**



1. Any member of staff who observes or suspects that a Service User is using or in possession of an illegal drug must report their suspicions to the Manager immediately.
2. **Note that a Service User's property cannot be searched unless the Service User is present and gives express permission, and a second member of staff witnesses the search.**
3. The Manager and another employee witness should advise the Service User that Crescent Community Care Services Ltd does not permit or tolerate the use or possession of illegal drugs on premises owned or managed by them. The Service User should be invited to voluntarily hand over any illegal drugs in his/her possession for destruction. A record of this conversation must be made in the Service User's records, with due regard to "need to know" confidentiality.
4. In subsequently reviewing the Service User's care needs it may be appropriate for the clinical team to offer the Service User the opportunity of a referral to the Alcohol and Drug Advisory Service (ADAS).
5. **Refusal of a Service User to surrender an illegal drug**
 1. **Crescent Community Care Services Ltd has no authority to remove or retain any substance without the Service User's express permission.**
 2. In the event that the Service User refuses to surrender an alleged illegal drug, the Manager should consult with the Registered Provider to decide whether, in the wider public interest, the Police should be informed of their suspicions. The Service User must be informed that this action may be taken.
3. **Handling of an illegal drug surrendered by a Service User**
 1. The alleged illegal drug should be placed in a secure envelope, labelled "suspected illegal drug surrendered to (name of person creating envelope), on (date), (time), and that entry signed and witnessed. The fact of the surrender and storage arrangements should be noted in the Service User's records, with due regard to "need to know" confidentiality.
 2. Place the envelope in a Controlled Drug cabinet.
 3. Inform the Police to arrange removal if the matter has been reported to the Police, or request the Pharmacist to remove and destroy it. Pharmacists are empowered to remove and destroy illegal drugs.
 4. The Manager should consult with the Registered Provider to decide whether, in the wider public interest, the Police should be informed of the situation. The Service User must be informed that this action may be taken.
5. **Supplying or dealing in illegal drugs**
 1. Where the quantity or circumstances of the observation, suspicion or surrender of illegal drugs gives rise to the suspicion that there was an intention to supply drugs to others, the Police must be informed.
2. **Visitors**
 3. There is no duty of confidentiality in respect of visitors. Any visitor suspected of or known to be in possession of illegal drugs, they must be told to leave the premises immediately.
 4. Any repeat of the possession on the premises, or reluctance to leave, must immediately lead to reporting to the Police.
5. **Staff**
 1. Any observation or suspicion of the use of illegal drugs on the premises by staff must be reported to the Manager, who will deal with the matter according to disciplinary procedures and the alcohol and drugs misuse policy.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

1. To ensure that Service Users and employees receive immunisation when a risk assessment indicates that they would benefit.

Scope

1. All Service Users and all employees.

Policy

2. Each Service User's GP will be contacted to obtain their opinion as to what immunisation is available, and if they recommend that the Service User is immunised.
3. If the recommendation is positive, the Manager will consult with the Service User, and if they consent to the immunisation, liaise with the GP to arrange for the immunisation.
4. All employees will be risk assessed for each available immunisation and recommended to ask their GP to immunise them if the risk assessment indicates that they would benefit.

Procedure

Service Users

1. Arrange with each Service User's GP when to request an annual review of that Service User's immunisation status.
2. At the agreed date, contact the GP and arrange for an assessment, followed by immunisation if recommended, and the Service User agrees.

Employees

1. Pre-employment:
 1. During the pre-employment process, assess the risks of:
 1. Protecting the employee and their family from workplace infections;
 2. Protecting Service Users, including those who may not respond well to their own immunisation;
 3. Protecting other employees;
 4. Allowing for the efficient and robust running of the Service.
 5. Do not use immunisation as a substitute for good practice - only consider immunisation if the risk analysis indicates that the risk cannot be adequately controlled by safe working practices alone.
- §
6. Government recommendations for all healthcare workers is:
 7. That routine vaccinations are up to date for all workers, including:
 1. Tetanus;
 2. Diphtheria;
 3. Polio;
 4. MMR.
 5. In addition, direct healthcare workers who have close contact with Service Users, or are involved with handling materials emanating from Service Users, are recommended to have the following additional immunisations, unless the individual risk assessment indicates otherwise:
 1. BCG, for those in close contact with medically vulnerable people;
 2. Hepatitis B and Hepatitis C for those who work with bodily fluids;
 3. Influenza, for those directly working with medically vulnerable people;
 4. Varicella for vulnerable workers who are in direct contact with Service Users.



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5. Employees should be referred to their GP for the immunisation, with a covering letter detailing the need, if required by the GP.
6. For a detailed analysis of the requirements, read the publication:
7. Immunisation of healthcare and laboratory staff - <https://www.gov.uk/government/publications/immunisation-of-healthcare-and-laboratory-staff-the-green-book-chapter-12>
8. For information about how to deal with an exposure incident see:
9. Sharps and Needlestick Policy and Procedure.
10. <http://www.hse.gov.uk/biosafety/blood-borne-viruses/how-deal-exposure-incident.htm>

Reviewed by Marcus Kerridge McColl
November 2025

Induction Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. When new employees are recruited, relocated or transferred, the organisation will aim to help them adjust to the new situation and gain the required knowledge and competencies as quickly and easily as possible so that they become integrated and productive members of the organisation from the earliest possible opportunity.

Scope

2. All new, relocated and transferred employees.

Policy

3. When a job has been offered, or a relocation or transfer agreed, an induction “programme” will be drawn up for use during the early part of the employee’s employment covering local environmental matters, employment information, and job specific knowledge.
4. The programme will meet the standards of the **Health and Social Care Act 2008**.
1. All new members of staff, including trainees and all staff under 18, will successfully complete an induction programme, within 12 weeks of appointment. Induction resources can be found on the Skills for Care website.
2. New employees may not begin work until a negative check has been received, and when they begin work they will be supervised at all times by an experienced and checked employee during the period before their criminal record check is received and employment confirmed. During the period before a satisfactory criminal record check is received, new staff will not work alone with access to Service Users.

Procedure

Users of the QCS Online System (only):



CRESCENT

3. On the first day of employment, new employees will be coached to use the PC linked to the QCS system, and given a personal ID and password.

Organisations using the paper based QCS system:

4. The induction programme will consist of the compulsory items listed in Recruitment Pack C – Job Start and Induction, together with any specific skills identified during recruitment as required but not present, or which need aligning with this employment situation.
5. Before the new employee begins work, the manager, or their delegate, will ensure that a full induction programme is specified, a venue for the induction arranged, and sufficient uninterrupted time is available for adequate one-to-one training.
6. When the new employee first reports for duty, the Manager or a person delegated to the task and given sufficient time and knowledge to carry out the task, will greet them and introduce them to colleagues. An experienced and Police-checked member of staff will be allocated to the new employee as a “buddy” or “shadow”, with the intention of providing a point for informal support during the introductory period. Where possible, the staff rota will be organised so that the new employee and “buddy” or “shadow” will be working together as much as possible, and alternative supervision arrangements made where the “buddy” or “shadow” is not on duty.
7. On the first day of employment, the Recruitment Pack C – Job Start and Induction Pack will be completed, and the appropriate section of the Induction Checklist used to plan and control induction training.
8. For catering staff, the Manager must establish before the recruit begins work that they have appropriate food hygiene qualifications and skills and, if not, require them to complete the “Food Hygiene Workbook”, and read the “Industry Guide to good Hygiene Practice: catering Guide” before beginning working
9. Items in the programme will be demonstrated to the employee, and when the employee is assessed as having understood each individual item the demonstrator will sign off the demonstration and assessment columns. At the completion of induction, a review of understanding will be carried out and signed off if satisfactory.

Reviewed by Marcus Kerridge McColl
December 2025

Infection Control Policy and Procedure
Crescent Community Care Services Limited

Purpose

To maintain prevention and control of cross-infection.

Scope

All employee and others who enter the premises, such as agency workers, visitors and contractors.

Policy

1. Crescent Community Care Services Ltd will arrange for the prevention of infection, toxic conditions or spread of infection in the community.
2. All work spaces will be kept clean, hygienic and free from offensive odours throughout.
3. All employees will be trained in effective hand washing.
4. Infected materials and/or clinical waste will be kept in appropriate (yellow) bags and stored separately from foodstuffs and food preparation areas.
5. Employees who report an infectious condition must not be allowed back on duty until medical clearance is given.
6. Employees known to have an infectious condition will be reported to the relevant authorities if appropriate.
7. A register must be maintained of all known causes of infection, from which likely causes of infection may be traced and/or identified.

8. For outbreaks of infectious disease which, in the opinion of the medical practitioner involved, is sufficiently serious to warrant notification of the CQC, such notification will be carried out within 48 hours.
9. Staff will wear barrier clothing when dealing with waste materials and when in close physical contact with the Service User. The barrier clothing will be removed and disposed of before leaving the premises. New barrier clothing will always be used for each Service User.
10. Staff will carefully wash their hands on entry to and exit from a Service User's premises, following the detailed instructions given.

Procedure

1. The carer will summon a doctor if an infectious disease is suspected.
2. The Community Health Physician must then be notified.
3. If the Service User shares a house with others, they should, if possible, be accommodated in a single room, with separate sanitary facilities, for the duration of the infection.
4. Staff will explain to the Service User the reasons for taking precautions, and ask for co-operation and participation.
5. Relatives should be kept fully informed of the Service User's condition and progress, and should be advised not to bring in anything valuable or expensive which cannot be disinfected.
6. Aseptic techniques must be used at all times.
7. Sterile disposable equipment should be used.
8. Strict personal hygiene must be observed by all staff.

The following is repeated from the Health and Safety Policy:

Prevention of cross-infection

9. To prevent cross-infection, ensure that:
 - Any infection a Service User has does not spread to others.
 - The Service User does not suffer from potential sources of infection in their surroundings.
 - Others do not bring infection onto the premises.

Notifiable diseases

1. Certain cases of infectious disease, whether confirmed or suspected, must be notified by the resident's GP to the proper officer of the local authority (who may be part of the local HPU team) under the Health Protection (Notification) Regulations 2010.
2. The responsibility for the notification of the listed disease(s) rests with the doctor attending the Service User.
3. Notification of infectious diseases' is the term used to refer to the statutory duties for reporting notifiable diseases in the Public Health (Control of Disease) Act 1984 and the Health Protection (Notification) Regulations 2010.
4. Diseases notifiable to local authority proper officers under the Health Protection (Notification) Regulations 2010 include:
 - » Acute encephalitis;
 - » Acute infectious hepatitis;
 - » Acute meningitis;
 - » Acute poliomyelitis;
 - » Anthrax;
 - » Botulism;
 - » Brucellosis;
 - » Cholera;
 - » Diphtheria;
 - » Enteric fever (typhoid or paratyphoid fever); » Food poisoning;

- » Haemolytic uraemic syndrome (HUS);
- » Infectious bloody diarrhoea;
- » Invasive group A streptococcal disease;
- » Legionnaires' disease;
- » Leprosy;
- » Malaria;
- » Measles;
- » Meningococcal septicaemia;
- » Mumps;
- » Plague;
- » Rabies;
- » Rubella;
- » Severe Acute Respiratory Syndrome (SARS); » Scarlet fever;
- » Smallpox;
- » Tetanus;
- » Tuberculosis;
- » Typhus;
- » Viral haemorrhagic fever (VHF);
- » Whooping cough;
- » Yellow fever.

5. Most outbreaks will present non-specific symptoms; serious sepsis or epidemic wound infections.
Any member of staff suspecting an outbreak of the notifiable disease should make their suspicions known to the
6. Registered Manager who will inform the appropriate doctor.

Records on outbreak

7. A record should be kept of the following information on residents, with suspected or confirmed infections or infectious disease:
 - » Name, age/date of birth;
 - » GP's name and address;
 - » Date of onset of symptoms and cessation of symptoms; » Type of symptoms;
 - » Samples taken and sent;
 - » Diagnosis;
 - » Source of infection, if known;
 - » Contacts - family, staff, visitors;
 - » Outcome;
 - » Whether the case was notified/ reported to the Proper Officer/HPU and date of reporting. §
1. Information should also be kept for any staff members that develop similar symptoms.

Rules to prevent the spread of infection

2. All staff should adhere to the following:
 - » Avoid infection by careful control of coughing and sneezing, i.e. use tissues / handkerchief;
 - » Appropriate hand washing;
 - » Use of disinfecting hand rubs;
 - » Avoid wearing jewellery, except for wedding rings;
 - » Keep hair short or tied back;
 - » Wear clean uniform clothing, and cover that clothing with barrier clothing while dealing with each Service User, disposing of it before leaving the premises;
 - » Hand wash on entry and exit of the premises, and as appropriate during stay on premises;
 - » Report any signs of infection to the appropriate person;
 - » Keep toilets and commodes scrupulously clean using correct disinfectant agents;

»Correct handling of food to prevent food borne illness;

»Take care when dealing with pets. Always hand wash or use the hand rub after contact.

Staff skin awareness

1. All cuts and abrasions should be covered with a waterproof plaster (blue coloured if working in food areas). Early detection and prompt reporting of infection is particularly important.
2. Any staff member with a skin infection must take advice from a doctor before continuing to work. All skin infections must be reported to the Registered Manager.

Staff sickness

3. Staff with diarrhoea and vomiting should not attend work but ring to report sick. Should the condition persist it may be necessary to provide a specimen of faeces and not return to work until medical clearance by a GP is given. It cannot be emphasised too strongly that young children and the elderly are particularly vulnerable to infection, and every attempt should be made to minimise any risk of infection.

Skin Infections

4. Report to your manager any Service Users who have a rash or unaccountable marks on their body.
5. Where scabies or shingles are suspected:
 - » The manager must request a visit from the GP.
 - » Staff should wear a plastic apron and wear gloves for any direct contact. Face masks and eye protection may be required if there is a risk of blood and body fluid splash into the eyes, nose and mouth.
 - » All linen must be placed in the appropriate bag, and the appropriate laundry procedure followed for contaminated laundry.

Blood borne viruses

1. Any Service User may be a carrier of a blood borne virus. There are blood-borne viruses other than hepatitis B, other hepatitis and HIV/AIDS. Appropriate precautions must therefore be taken with all Service Users and particularly with body fluids.
2. Always assume that blood and other body fluids are infected. All accidents, facial, particularly eye, or wound contact with infected body fluids must be recorded as an incident.
3. All accidents must be reported.
4. Body fluid handling and spillage procedure should be as follows:
 - »Use no-touch techniques when dealing with blood or body fluid. Wear gloves and plastic aprons as appropriate. Masks and goggles are not normally needed.
 - »Care staff should clean body fluid spillages immediately, wearing disposable gloves and plastic aprons. Face masks and eye protection may be required if there is a risk of blood and body fluid splash into eyes, nose and mouth.
 - »Use appropriate disinfectant agents on carpets.
 - »Use no-touch techniques or gloves when disposing of anything contaminated with blood, e.g. dressings.
1. Avoid contamination with saliva. If saliva contamination to eyes, a cut or an open wound occurs, wash liberally with water and inform the manager immediately.

Outbreak control measures

2. An outbreak of gastroenteritis is indicated by the occurrence of UNEXPLAINED diarrhoea and/or vomiting in two or more Service Users. (Remember that there are also non-infective causes of diarrhoea and vomiting). The recommended action in such cases is as follows:
 - »Staff should inform the manager who should then contact the appropriate GPs.
 - »A specimen of faeces should be made available for testing, if required.

- »Wear plastic apron and protective gloves when in contact with excreta.
- »Dispose of faeces carefully and disinfect bedpans/commodore using disinfectant.
- »In a shared house, place the Service User in a single room, where possible, and with their own toilet facilities, such as a commode.
- »Any Service User with, or suspected of having, gastroenteritis should have their own sink/bowl for washing.
- »All crockery and cutlery should be soaked in a bowl of disinfectant for 30 minutes before being removed from the room of the Service User to the kitchen.
- »Place all contaminated linen into a coloured bag and keep separate from any other linen.
- »Wear a protective apron and gloves when sluicing contaminated linen. To sluice any contaminated linen, leave the linen to soak in disinfectant for 30 minutes before removing to laundry. Face masks and eye protection may be required if there is a risk of blood and body fluid splash into eyes, nose and mouth.
- »Wash hands thoroughly after attending the Service User and before going to any other task.

3. The manager should notify the local health authorities when the occurrences are unexplained.

National Colour Coding Scheme

4. National Colour Coding for cleaning materials and equipment ensures that these items are not used in multiple areas, therefore reducing the risk of cross-infection.
 - » Red: Bathrooms, washrooms, showers, toilets, basins and bathroom floors;
 - » Blue: General areas including wards, departments, offices and basins in public areas;
 - » Green: Catering departments, ward kitchen areas and patient food service at ward level;
 - » Yellow: Isolation areas.

Waste Management

5. Any waste that is deemed to be infectious or hazardous is considered to be hazardous waste and should be consigned for treatment/disposal at suitably licensed/permitted facilities.

Segregation of Waste

Clinical waste

6. All clinical waste should be secured in an approved way and identified with a coded tie or label to indicate source of waste (see table below). Bags should not be closed by an overhand knot. Good practice is to 'swan neck' the bags by twisting the top and then turning it over on itself. The bag should then be secured with tape and tie. Bags should not be more than 3/4 full.
7. Areas where clinical/hazardous waste is produced should have foot-operated bins for waste stored in bags.

Non-clinical Waste or Domestic Waste

8. Other general waste (food waste, non-contaminated household materials) unsuitable for recycling, should be disposed of in black refuse bags.

Reviewed by Marcus Kerridge McColl
November 2025

Lifts and Hoists Policy and Procedure
Crescent Community Care Services

Purpose

1. To comply with statutes, regulations and quality standards.

Scope



CRESCENT

2. All establishments with lifts and hoists owned or used by Crescent Community Care Services Ltd, or installed in Service User's accommodation.

Policy

INTRODUCTION

3. Various types of lifts and hoists are used by The Agency, ranging from passenger lifts to fixed and mobile hoists. All share a common feature in that if they are not properly maintained and subjected to a thorough inspection by a competent person, failure of the lift/hoist may result in serious injury and even death.
4. It is our policy to:
 1. If appropriate, provide lifts and hoists that are suitable for their intended use, or complete a referral to the Local Authority's Community Equipment team to provide suitable lifts and hoists;
 2. Ensure that all lifts and hoists, including slings and fastenings, are regularly inspected and maintained;
 3. Ensure that all lifts and hoists are thoroughly examined by a competent person in accordance with statutory requirements;
 4. Train members of staff in how to use the lifts and hoists.

Procedure

1. Prepare an inventory of lifting appliances and equipment.
2. Arrange maintenance contracts; these will be the responsibility of The Agency if the lift or hoist is owned by them, or the responsibility of the owner if not owned by them e.g. Service User's own equipment. It is the responsibility of The Agency to exercise due Care and attention even if the equipment is not owned by them.
3. Arrange statutory insurance inspections.
4. Arrange and deliver training for all members of staff who may use the equipment.
5. Carry out visual in-house checks.
6. Check safety devices e.g. passenger alarms, interlocks, brakes, slings.
7. Set up reporting system for defects.
8. Monitor and review.

Reviewed by Marcus Kerridge McColl
December 2025

Management and Prevention of Violence at Work Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To comply with statutes, regulations and quality standards.
2. **Crescent Community Care Services Ltd** will promote awareness and preventative measures to reduce the risk of violence towards workers.



3. Employees who deal directly with others in the course of their work may increasingly face aggressive or violent behaviour. They may be sworn at, threatened, or even attacked; serious acts of violence are however relatively infrequent. The Health and Safety Executive's definition of work related violence is:

1. "Any incident in which a person is abused, threatened or assaulted in circumstances relating to their work".

Scope

1. All workers.

Policy

2. Acts of violence can cause pain, distress and even disability or death. Physical attacks are obviously dangerous but serious or persistent verbal abuse or threats can also damage employees' health through anxiety or stress. The organisation considers violence towards employees and the risk of violence to be a serious matter and recognises and accepts that it has both a moral and legal responsibility to prevent or reduce the risk of violence at work through:

1. The introduction of measures to reduce the risk of violence at work;
2. Full support for any employee who has been subject to abuse, threats or assault in the course of their work;
3. The reporting of all incidents of violence through the Accidents Reporting Procedure;
4. The full investigation of all reported incidents and the reporting of all assaults to the Police;
5. Serious incidents of violence by employees or students involving assault on another person being dealt with in accordance with the Disciplinary Procedure.

1. Persistent or isolated examples of behaviour amounting to racial or sexual harassment will be dealt with in accordance with the Harassment Procedure and/or Disciplinary Procedures.

2. In accordance with the Health and Safety Policy, Managers are responsible for the implementation of any measures introduced in accordance with the spirit of this policy within their area of control. The policy should be referred to in conjunction with other Policies, in particular:

Working Alone Policy and Procedure

De-Escalation Policy and Procedure

1. Managers must ensure that a review of all working activities is undertaken and that an assessment is made of those work activities that might result in members of staff and others being exposed to violence e.g. working practices and procedures and/or physical aspects of premises. Appropriate controls should be introduced.

2. **Assessment**

1. Effective measures for dealing with violence at work need to be based on proper analysis of the problems and careful consideration of the alternatives that are available. Issues to be considered include: ☒

1. Physical aspects of premises;
2. Working practices and patterns; ☒
3. Staff training;
4. Contact with the police.

1. **Physical aspects**

1. The general design and physical environment of buildings can sometimes be improved to reduce the likelihood of violent incidents, and can help reduce the numbers of unauthorised visitors wandering through the building.

2. **Working practices and patterns**

3. Particular activities or jobs can create the potential for violence. Sometimes activities can be altered so that they are carried out in a way which reduces the risk e.g. working in pairs or providing a means of summoning help. Where activities or jobs cannot be altered or adapted, specific precautions or techniques may help reduce risks. Specific precautions to be considered include dealing with angry Service Users or the public, training in interpersonal skills, and precautions for home visits and off-premises activities.

4. **Staff training**



CRESCENT

5. Employees who may be exposed to violence need to be aware of the risks they face and be capable of recognising the potential for danger, either to themselves or to others, through staff training.

Procedure

6. Contact with the Police:

7. Where circumstances require it, assistance may be summoned from the Police by dialling 999.

1. Risk Assessments must be reviewed at intervals to ensure the effectiveness of the measures introduced for the control of violence.

2. Training:

3. Appropriate training for employees will be included in the Staff Development and Training Programme arranged by the Head of Personnel Services.

1. Reporting of Incidents:

2. Any incidents of violence, as defined in paragraph 1, must be reported to the Health and Safety Officer using the Accident (Personal Injury) Report in accordance with the Accidents and Hazards Investigation and Reporting Procedure. Certain acts of physical violence must be reported to the Health and Safety Executive in accordance with the Reporting of Injuries, Diseases and Dangerous Occurrences Regulations and this will be undertaken by the Health and Safety Officer.

Reviewed by Marcus Kerridge McColl
December 2025

Maternity Leave Policy and Procedure
Crescent Community Care Services Limited

Purpose

To comply with the:

1. Employment Relations Act 1999;
2. Employment Act 2002;
3. Equality Act 2010;
4. Statutory Maternity Pay (General) Regulations 1986;
5. Statutory Maternity Pay (Medical Evidence) Regulations 1987; §
6. Maternity and Parental Leave Regulations 1999.

Scope

1. All employees.

Policy

2. The organisation's policy is to comply with both the letter and spirit of the law on maternity rights. To this end, its aim is to inform all female employees, regardless of marital status, of your entitlement to statutory maternity rights and to ensure that those rights are understood by employees who qualify. "You" in this policy applies to you if you are a female employee.



CRESCENT

3. This policy and procedure covers the basic (sometimes called statutory) rights that employers must provide for new and pregnant mothers who work for them.
4. This policy also covers the basic allowance that relates to antenatal rights for fathers and partners.

Summary of employment rights if you are pregnant.

1. You are entitled to:
 1. Paid time off for ante-natal care;
 2. 52 weeks' maternity leave, regardless of how long you have been with us;
 3. Statutory maternity pay for 39 weeks, provided you have worked for us for 26 weeks by the 15th week before your baby is due. You may qualify for maternity allowance if you do not qualify for SMP;
 4. Continuation of the contract of employment, except for pay;
 5. Return to the same employment after ordinary maternity leave. After additional maternity leave you are entitled to return to the same job unless that is not reasonably practicable, in which case the right is to return to an appropriate similar job;
 6. Protection from detrimental treatment or dismissal, including redundancy, on any grounds partly or wholly related to pregnancy;
 7. 18 weeks' unpaid parental leave until the child's 18th birthday;
 8. Dependants' leave, where appropriate.
9. Partners of pregnant women and adopting parents may be entitled to paternity leave.

Maternity leave and your employment contract

1. The first 26 weeks of maternity leave are deemed ordinary maternity leave. Your employment contract continues during ordinary (and additional) maternity leave.
2. The only element of your contractual rights that does not continue is your wages or salary. You are entitled to receive the same basic statutory rights such as paid annual leave – which you can continue to accrue while on maternity leave.
3. Additionally, as your employer, we must not discriminate against you while you are on any part of the maternity leave.

Starting maternity leave

1. The earliest that you can start the maternity leave is 11 weeks before the expected week of childbirth, which is when you are around 29 weeks pregnant. To ensure that your rights are upheld, you should use the due-date given on the MAT B1 pregnancy certificate that your midwife or GP will give to you.
2. In order to work out when the 11th week before the expected week of childbirth falls, you should find the Sunday before the date which the certificate states the baby is due, or the due date if it is a Sunday, and count back 11 Sundays from that date. The earliest date on which you can start the leave is that Sunday. As long as it does not fall before the 11 weeks before the baby is due, it is generally up to you as to when your maternity leave begins.
3. You can even choose to work right up to the date that the baby is born.
4. You should note that the start of the maternity leave might be triggered automatically if:
 1. You have a pregnancy-related illness or absence in the last four weeks of your pregnancy. In this case you must inform us that you have an illness that relates to the pregnancy and we can insist that you start your maternity leave even if you are off sick for only one day. However, we may allow you to carry on working until you had planned to start your leave, especially if you have only been away for a short time.
 2. Your baby is born before the day on which you were planning to start the maternity leave. In this situation, your leave starts automatically on the day after the day of the birth. You must notify us as soon as is reasonably practicable that you have given birth, as well as noting the date of the birth.
1. Having been notified of your intention to take maternity leave and the date on which it will begin, we will give you notice of the date that your maternity leave period shall end. We will provide you with this information within 28 days of you informing us of the date on which the maternity leave period will start, or on which it has started.

Additional maternity leave

2. The second 26 weeks of maternity leave are deemed additional maternity leave. This leave starts on the day after the ordinary maternity leave period finishes. Your employment contract continues during additional maternity leave.
3. You are entitled to return to the same job on the same terms and conditions. If, exceptionally, this is not possible (for example, if the job no longer exists) you are entitled to suitable alternative employment on terms that are not substantially less favourable.
4. The contractual rights of women on additional maternity leave are maintained, except those rights related to pay, as they are during ordinary maternity leave.
5. The only difference between ordinary maternity leave and additional maternity leave is in relation to pension rights. We will maintain your contributions to the pension scheme, if such applies, throughout ordinary maternity leave and throughout a period of paid additional maternity leave (based on the salary you would have received if you weren't on leave). We are not obliged to maintain our contributions for any period of unpaid maternity leave. This means if you are paid for 39 weeks of maternity leave, and you take the full 52 week entitlement to leave, our employer contributions may not be made for the final 13 weeks. If this is any different it will be shown in your contract.
6. Employee contributions to the pension scheme, if applicable, will be based on your actual pay for the period of the maternity pay period (39 weeks).

Compulsory maternity leave

1. It is compulsory for you to take 2 weeks of maternity leave, following the birth of your baby. This is for health and safety reasons.
2. All employment entitlements and conditions that apply during the maternity leave period will also apply during the period of compulsory maternity leave.

Notice of maternity leave

1. You must notify us of your intention to take ordinary maternity leave by the end of the 15th week before the child is due and you must give us the information in writing if we request it. You can notify us earlier than this, and may find it helpful to do so in order to ensure that your health and safety rights are observed, and that you have time off for antenatal care.
2. Your notice must state:
 1. That you are pregnant;
 2. The expected week of childbirth;
 3. The date on which you intend the ordinary maternity leave to start.
1. We can request that you give us a copy of your MAT B1 certificate which you will be given by either the GP or midwife when you are around 20 weeks pregnant and which will state the week that the child is due. We can request that all the notice that you give is in writing.
2. Once you have given notice of the date on which you intend the leave to begin, you can subsequently change your mind and vary the date. In this case, you must notify us of the variation at least 28 days before the new date begins, or if this is not reasonably practicable, as soon as is reasonably practicable.

Notice of return to work

1. You do not need to give any notice that you are returning to work at the end of the maternity leave. However, you might find it helpful to contact us just to make sure that we know you are coming back and the contact would be appreciated.
2. On the other hand, if you wish to return to work earlier than the 52 weeks entitlement, you must give us 56 days/8 weeks' notice of the intention to return early. If you fail to do this, and arrive at work regardless, we can refuse to pay you.

Job role on return



CRESCENT

1. After returning from ordinary maternity leave (OML), you have the right to go back to the same job that you were in before you went on leave.
2. After returning from additional maternity leave (AML) you also have the right to return to the same job unless we can demonstrate that it is not reasonably practicable to have kept that job open for you. In this case, you are still entitled to an alternative job that is both suitable and appropriate for you.

Flexible hours

1. All employees who have worked for us for 26 weeks have the statutory right to request flexible working.

Entitlement to Statutory Maternity Pay

2. You must meet the following three conditions in order to be entitled to SMP:
 1. You must have worked for us for at least 26 weeks by the end of the qualifying week (i.e. by the end of the 15th week before the week the baby is due);
 2. You must still be in the job in the qualifying week;
 3. You must have been paid at least the lower earnings limit on average in the 8 weeks (if you are paid weekly) or the two months (if you are paid monthly) up to the last before the end of the qualifying week.
4. To receive SMP you must give us 28 days' notice of the date on which you want the pay to start and you cannot change your mind about this date.
 1. Your SMP can start on any day of the week that you choose and in most cases will start on the same day as the maternity leave period starts.
 2. To claim your SMP, you must give us a copy of the MAT B1 form – the maternity certificate - which states the expected week of childbirth which the midwife or GP will give you when you are around 20 weeks pregnant. You may find it easier to give the notice for the leave and pay together in the 15th week before the baby is due. However, if you decide to do this then you must remember that you cannot change the date on which the pay starts, even if you decide to vary the date on which the leave begins.

Workers versus employees

1. If you are a worker rather than an employee, for example, because you obtained work through an agency, or are freelance, you will not get the full set of maternity rights, but you will still receive protection, such as health and safety risk assessments that we must carry out, and the right to claim sex discrimination if you should suffer detrimental treatment as a result of your pregnancy.
2. You may qualify for maternity allowance, which is paid by the Government through your local Jobcentre Plus office, rather than through us.

Benefits for those not entitled to SMP

1. If you are not entitled to receive SMP because you are a worker or for other reasons, you may qualify for maternity allowance (MA) and you should seek advice from the local Jobcentre Plus office or Citizens Advice Bureau about your eligibility.

Sickness during pregnancy, maternity leave, or when supposed to return to work

If you are absent through sickness during pregnancy, this will not automatically trigger your maternity leave and you will be treated as every other member of staff who is off sick. You will usually be paid sick pay in the same way as other staff. However, if you are sick during the last four weeks of the pregnancy then we can insist that the maternity leave starts, although we may overlook occasional days of absence.

Once the maternity leave has started, you cannot claim sick pay from us if you become ill. If you are sick when the maternity leave is due to end, our usual sickness procedures will apply to you.

Time off work to attend antenatal classes



CRESCENT

2. An antenatal appointment is an appointment you make on the advice of your doctor, midwife or health visitor.
3. You have the right to reasonable paid time off for antenatal appointments, and this includes the time spent travelling to an appointment and waiting. It does not include entitlement to pay for time where you could reasonably attend work beforehand or return to work afterwards.
4. You cannot be refused time off for the first appointment, but for subsequent appointments we can ask you for written proof of the appointment and a certificate or note from your doctor or midwife, stating that you are pregnant.
5. Antenatal care may include not only medical examinations, but also relaxation and parent craft classes.
6. The right to paid time off for employees begins on the day the job starts and there is no minimum qualifying service.
7. You are not under any obligation to make up the 'lost' hours at a later date.
8. This right only applies if you are classed as an employee rather than a worker.

Antenatal rights for fathers and partners

1. Fathers and partners can take unpaid time off work to accompany a pregnant woman to see a midwife or obstetrician.
2. The right is to time off on up to two occasions for a maximum of 6.5 hours each.
3. You should complete the antenatal appointment request form if you are an expectant father or the partner of the expectant mother and plan to accompany the mother to an antenatal appointment.
4. Time off to attend an antenatal appointments is time off without pay.

Breastfeeding

1. The law does not specifically allow you to delay the return to work from maternity leave or to take special leave once back at work in order to continue breastfeeding.
2. You should ensure that we have written notification that you are breastfeeding so that in the health and safety 'risk assessment' we can take account of the particular risks to you and the baby while you are breastfeeding.

Job ceasing and SMP

1. As long as you are employed in the 'qualifying week', then you are still entitled to receive SMP provided you meet all of the other conditions outlined above. It does not matter if you are off sick or on holiday in the qualifying week.
2. Once you have qualified for SMP you are entitled to receive it for the full 39 weeks. This is the case even if you are made redundant, you leave the job, or a fixed-term contract comes to an end at any time after the 15th week before the baby is due or during the maternity leave.

Part-time working

1. The maternity rights of part-time employees are the same as those of a full-time employee.

If you become pregnant during maternity leave

2. The maternity leave does not break the continuity of employment, so the right to maternity leave for this pregnancy will be based on your total service with us. You will also qualify for SMP, as long as they meet the normal conditions.
3. You must meet the same notice requirements as you did the first time you went on leave.
4. You must physically go back into work for a period – even just one day – in order to maintain all of your rights. You will, however, have the same rights as you would have received on returning from additional maternity leave i.e. the right to return to the same job, or if that is not reasonably practicable, a suitable alternative job on similar terms and conditions.



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5. You should also note that SMP is calculated on average earnings of 8 weeks before the 15th week before the baby is due. Therefore, if you received statutory maternity pay or maternity allowance in this period, then the next maternity leave will be calculated using the actual amount of money you received in this time.
6. Therefore returning to work between the two leave periods may increase the amount of SMP you receive, particularly if you can return 8 weeks before the end of the 15th week before the baby is due.

More than one part-time job and SMP

1. If you work for a second employer, you are allowed to exercise your rights regarding maternity pay and leave with both of us, on a pro rata basis for each job. However, you must ensure that you have worked for each of us for 26 weeks by the qualifying week (15th week before the baby is due), and that you give the correct notification of when the maternity leave will start to both of us.
2. In total you are entitled to receive the same amount of maternity pay as someone who works in a full-time job. However, if you have not worked for us for 26 weeks by the qualifying week, then you will not be entitled to receive maternity pay from us.

Working for us during maternity leave

1. After the first 2 weeks after the baby's birth, you may carry out up to 10 days' work for us (or for each employer who pays you SMP, if you have more than one) without losing any maternity pay or leave. These 'Keeping in Touch' (KiT) days could include attending a training day or staff meeting or actually doing a full day's work. Anything you do on any day will count as though you had worked a full day (even if you only went into work for an hour), but KiT days cannot be used to extend your maternity leave period.
2. Keeping in Touch days have to be an agreement between you and us. If you choose to work on a KiT day, we do not have to pay you normal wages, but your maternity pay is not affected by working such a day.

Reviewed by Marcus Kerridge McColl
December 2025

Medical Emergency Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To support and train employees to deal effectively with medical emergencies.

Scope

2. All medical emergencies.

Policy

3. The competent Carer should contact the GP and if necessary the Emergency Services, before returning to the Service User to ensure their safety and wellbeing.

Procedure

1. Care staff training in medical emergencies and basic first aid will be included in the Training Plan.
2. Care staff will contact the office for advice and guidance in an Emergency situation.
3. Care staff may be requested to summon the Service User's GP or emergency services as instructed by the Care supervisor/Area Co-ordinator/Manager.
4. Care staff will inform next of kin/advocate/family members.

5. **ALL EMERGENCY ACTION MUST BE DOCUMENTED IN THE Service User Care Plan IMMEDIATELY FOLLOWING THE INCIDENT.**

Reviewed by Marcus Kerridge McColl
November 2025

Medications Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To comply with current legislation and best practice in the administration of medicines, including:
 1. Relevant evidence-based guidance and alerts about medicines management and good practice.
 2. Published by appropriate expert and professional bodies, including:
 1. National Patient Safety Agency
 2. National Institute for Health and Clinical Excellence
 3. Medicines and Healthcare products Regulatory Agency
 4. Department of Health
 5. Royal Pharmaceutical Society of Great Britain (RPSGB)
 6. Social Care Institute for Excellence
 7. Medical and other clinical royal colleges, faculties and professional associations
 1. The safe and secure handling of medicines: a team approach (RPSGB, 2005)
 2. Safer management of controlled drugs: Guidance on strengthened governance arrangements (DH, 2007)
 3. Safer management of controlled drugs: Guidance on standard operating procedures for controlled drugs (DH, 2007)
 4. The handling of medicines in social care (RPSGB, 2007)
 5. Research governance framework for health and social care: Second edition (DH, 2005)
 6. The Mental Capacity Act 2005
 7. Mental Capacity Act Code of Practice 2007
 8. Misuse of Drugs Act 1971
 9. Health and Social Care Act 2008
 10. NICE Guidelines – Managing Medicines in Care Homes 2014

Scope

11. All medications, drugs.
12. All persons delivering Care or support, and persons ordering, receiving, managing, administering, disposing and recording medicines.

Policy

1. This policy must be read in conjunction with Royal Pharmaceutical Society - Handling Medicines in Social Care, which takes precedence over this document, and any specific requirements of the Registration Authority. These publications can be found in the Useful Documents section of the QCS web site.

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COMMON POLICY

1. Roles, Responsibilities and arrangements for supervision

34. Management and supervisors are responsible for:
 1. The training and assessment of Care workers in the skills and techniques of medications management consistent with their role;
 2. Instruction of the Care workers in the specific requirements in respect of medications management;
 3. Ensuring that Care workers are at all times provided with administration systems and resources to support their role;
 4. Supervision and auditing of individual Care workers and systems in order to ensure compliance with this policy, as well as with legal requirements and best practice, and to ensure that at all times Care workers are meeting the needs of Service Users;
 5. Regular audit of medications records in order to confirm that the standards of practice and record keeping in respect of medications are as specified to the Care worker. Shortcomings will be reported into the organisation's quality assurance system, and in addition the Care worker(s) involved will be formally supervised and remedial action taken.
6. Care workers are:
 1. Permitted to assist the Service User in taking prescribed medication, within the limits of the Care worker's training and experience;



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2. Care workers will be instructed, through induction and continued training, in the skills, requirements, and level of competence which are required in order to carry out their role. In particular, Care workers who are not a Registered Nurse must have completed a Medications Administration course approved by the Registration Authority before administering medicines, and must also have completed a Medications Management course approved by the Registration Authority if they manage the system through which other Care workers and themselves administer medications;
3. Care workers may purchase, on behalf of the Service User, non-prescribed medicines, and assist in the taking of those homely remedies, only in accordance with homely remedies policy and procedure contained in this policy. Specifically, the Care worker may not purchase or administer homely remedies which have not been specified for use by the Service User's GP.

2. Medications management in a Service User's own home:

4. General:
 1. Wherever possible, Service Users will manage their medications and self-administer them. If an assessment determines that this is not possible, potential Care Plans may include:
 1. A Care worker with appropriate training encouraging the Service User to self-medicate, confirming that the medication has been taken or refused, recording this result, including the time, and taking the action described elsewhere in this policy in the event of a refusal;
 2. A Care worker trained and certified as competent may administer the medication, taking responsibility for correct administration according to this policy and procedure.
 3. Doses can be left out for the Service User to take at a later time. This should be used as a last resort and other options should be discussed before deciding on this action, such as an additional visit for medication prompt. If no other suitable option can be found, then it should be agreed with the Service User, included in the Care Plan, and risk assessed to protect both the Service User and Carers.
4. Refusal of medication:
 5. Identify any special medication administration preferences of the Service User by reading the medication Care Plan.
 6. If a Service User refuses to take their prescribed medication then the reasons for refusal should, if possible, be identified with the Service User, and a second attempt made to offer the medication having addressed the Service User's concerns and requirements.
 7. A Service User MUST NEVER BE FORCED TO TAKE MEDICATION; this act would constitute abuse.
 8. When a Service User refuses medication advice should be obtained from the prescribing GP in order to minimise medication risks.
 9. Medication refusal and consequent action taken by the Care worker must be documented on the medication Care Plan and the medication administration record, and the information communicated in the on-site attendance documentation to the incoming person responsible for medication.

THE REMAINDER OF THIS POLICY DOCUMENT WILL ONLY APPLY IN THE EVENT THAT Care STAFF BECOME INVOLVED IN EITHER SUPERVISING OR ADMINISTERING MEDICATIONS, AND SHOULD BE APPLIED APPROPRIATELY. IT IS RECOMMENDED THAT IN SERVICES WHICH MAY BECOME INVOLVED IN MEDICATIONS SUPERVISION AND/OR ADMINISTRATION, ALL Care STAFF ARE REQUIRED TO READ AND UNDERSTAND THIS POLICY, TO AVOID THE POSSIBILITY THAT A Service User CEASES TO SELF-ADMINISTER BUT THEIR Carer IS NOT APPROPRIATELY TRAINED TO DEAL WITH THAT EVENT.

Mental Capacity Act

1. Assessments in accordance with the Mental Capacity Act 2005 policy must be carried out in order to determine if there are any issues regarding the Service User's mental capacity and understanding e.g. where the Service User may lack the capacity to make a decision regarding taking medication. In certain circumstances section 6 of the Mental Capacity Act allows for the use of reasonable restraint in administering medication, and advice should be sought from health care professionals where restraint may be necessary to prevent harm.

3. Deprivation of Liberty Safeguards

2. This policy should be read in conjunction with the Deprivation of Liberty Safeguards policy, in order to determine where a Service User lacks capacity to make decisions about taking medication, where administration of medication might constitute a deprivation of the person's liberty such as where the medication constitutes a sedative.

4. General

3. Medicines are managed and administered safely to Service Users that need them at a time they need them and by staff who have been trained and assessed as competent to do so.
4. Service Users' rights to choice, independence, privacy and dignity are promoted by staff in the medication processes and procedures in the Care service and Service Users' values and beliefs are respected by staff who are involved with the management and administration of their medication.
5. All legal requirements for the management and administration of Service Users' medication will be adhered to at all times and designated staff will be responsible for the sharing of information regarding medication with the individual Service Users and, where appropriate, with their advocates.
6. Medication will be used to prevent and manage disease and alleviate discomfort.
7. Service Users will be fully involved with the management and administration of their medication.
1. They will be fully involved by the staff about their individual medication and its purpose and will have the medication patient leaflet information provided by the dispensing pharmacist shared with them in a method which promotes their understanding.
1. Before medication is administered to any Service User, **FORMAL CONSENT** must be obtained. Where a Service User is unable to give valid consent due to mental incapacity, best interest meetings will take place and where it is agreed that it is the best interest of the person, including their medical interests, that medication is administered then **FORMAL AUTHORISATION** for medication administration will be obtained and evidenced in the Service User's Care Plan and medication records.

5. Training

2. Care workers must be trained in the handling and use of medication and have their competence assessed:
 1. By supervisor's observation of practice during the first medication handling following initial training completion.
 2. By supervisor's observation of practice at 3 months following initial training completion.
 3. Reviewed at formal supervisions.
 4. By supervisor's observation of practice annually.
5. Each staff member will have an individual record of medication training, competence assessment, and practice skills supervision monitoring.
6. Training must include:
 1. The supply, storage and disposal of medicines
 2. Safe administration of medicines
 3. Quality assurance and record keeping
 4. Accountability, responsibility and confidentiality

Induction Training

1. CERTIFICATION of previous medication training must be supported by a competence assessment of the staff member's knowledge and ability to manage and administer medication before any such staff member is permitted to undertake medication handling.
2. Staff must be given time to read and understand all of the provider policies and procedures related to medication management and administration.
1. **Basic medication training** prepares Care staff:
 1. To give medicines via the mouth (tablets, capsules and liquids); » Drops into the ears, nose and mouth;
 2. Medication by inhalers;
 3. Medicines applied to the skin
1. **Further training is needed for:**

1. Any invasive techniques such as giving suppositories, enemas and injections;
2. Medication competence is assessed formally by the Care manager or Provider where qualifications and skills are appropriate. This is carried out by observation of practice against the medication policies and procedures.
3. **The following key tasks of Care workers are to be observed:**
 1. Checking what medication the Service User takes on the medication administration record and on the medication labels;
 2. Checking it is the right person;
 3. Asking whether the person wants the medication;
 4. Checking that the medication has not already been given;
 5. Preparing the correct dose for the time of day;
 6. Giving a suitable drink, with the medication, to the Service User;
 7. Signing the medication record entering the relevant data (for example time, where specific time is required or for a medication which is taken only when needed e.g. for pain management).

Further details of the skills and knowledge required for medicines handling can be found at the Skills for Care website www.skillsforcare.org.uk.

8. Care workers are permitted, if trained, to:
 1. Give capsules tablets and oral medicines;
 2. Apply external creams and lotions;
 3. Insert drops to ears nose or eyes;
 4. Administer inhaled medication.
5. Specialist additional training is needed in order to enable Care staff to carry out the following:
 1. Rectal administration. Suppositories;
 2. Injectable drugs such as insulin;
 3. Administration via gastrostomy tubes;
 4. Administration of oxygen.
5. **Timing of medication** administration can be crucial and **adherence to medication prescription instructions must be followed**. This must be clearly indicated in the medication Care Plan and in the medication administration record.
6. **Medicines must be given only to the person for whom they are prescribed**, following the prescription instructions.
7. **Medicines must be given only from the container in which they are supplied by the Pharmacy or GP. DOSES OF MEDICATION MUST NOT BE PUT OUT IN ADVANCE OF ADMINISTRATION**; this can lead to errors and accidents.

Ongoing updating

1. Health and social care practitioners should be able to access reliable and up to date information about medicines. Resources may include the patient information leaflet supplied with the medicine and the following websites:
 1. Medicines and Healthcare Products Regulatory Agency
 2. NHS choices
 3. NICE Evidence
 4. Patient.co.uk
5. Health professionals may also use the:
 6. British National Formulary (BNF)
 7. British National Formulary for Children (BNFC)
 8. Clinical Knowledge Summaries Electronic Medicines Compendium

Particular staff training issues for services delivered in the Service User's own home:

1. Where service staff only supervise Service Users' self-medication:



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1. There may be occasions when the Care service needs to offer support with medication due to Service Users' mental health issues;
2. Training must include:
 1. How to support Service User to take responsibility for their own medicines;
 2. Action to be taken if a Service User becomes unwell and unable to take full responsibility for medicines;
 3. Obtaining Service User formal consent if Care workers give medication;
 4. Which medication the Care worker can give, having been trained to do so;
 5. By what method the Care worker keeps records;
 6. Providing medication storage on an individual Service User basis.
 7. Policies and procedures to guide homecare staff in the processes of medication handling in the person's own home including what to do if a person decides not to take their medication.
 8. What to do if the Service User's mental or physical health state changes significantly.
 9. How to communicate between Care workers and other agencies.
 10. How medicines that are no longer in use are disposed of.
11. Where service staff administer medications to Service Users:
 1. Homecare providers must ensure that their insurance policy provides suitable indemnification for the policies and procedures that are followed.
 2. Homecare workers who work alone must be appropriately trained and regularly assessed for their competence in the safe handling of Service Users' medication.
 3. The Care provider must have clear plans on the individualised support for Service Users in their own homes who use medication. This must be detailed in their Care Plan and actively reviewed and monitored by the designated Care staff, who will have the appropriate skills and knowledge to do so safely and effectively.
 4. Communications between Care workers, supervisors, and prescribers must be effective in order to ensure that the Care staff are kept up to date with Service User medication changes and associated support needs.
 5. Important points for domiciliary Care staff:
 1. The amount of help and support the Service User needs with their medications;
 2. Which medications the Care worker may administer after being trained to do so;
 3. The way that the Care worker keeps records.
 4. What the Care worker MUST NOT DO e.g. OFFER ADVICE ON TREATMENT OF MINOR AILMENTS.
1. It is not appropriate for a Care worker to influence:
 1. How a person chooses to obtain medicines;
 2. How and where the person chooses to keep medicines in The Agency;
 3. How medicines that are no longer in use are disposed of;
 4. The choice of over the counter medicines the person/parent of the child Service User may wish to buy;

6. Confidentiality

5. All medication records for the Service User must be treated as confidential, and permission obtained from the Service User (or their advocate where appropriate) for the sharing of medication records, when this is needed to contribute to their health and wellbeing.
6. Where Service Users wish to be given their medication in private this must be accommodated to support their personal dignity and promote their autonomy.

7. Self- administration

1. All Service Users that have the ability and mental capacity to self-medicate should be given the opportunity and support to do so.
2. In order to protect the safety of the Service User and others it is essential to assess the Service User's ability to manage their medications independently and safely. This assessment should include:
 3. Whether the Service User wishes to self-medicate;
 4. Identify that the Service User knows the medication they are taking, what it is for, and how and when to take it;
 5. Understand how important it is, not to leave the medicines lying around where someone else may take it accidentally.
1. A full assessment of their ability and mental capacity to self-medicate must be carried out and documented.

2. In a Service User's own home, the assumption is that the GP supports self-medication. However, if there is any doubt then self-medication by the Service User should be authorised by the prescribing GP, following a risk assessment.
3. The assessment and documentation will be stored in the individual Service User Care Plan, and a copy held with the Service User's medication record when Service Users are self-medicating. This will help to remind staff of the need to monitor any associated risks.
4. The assessment will be reviewed if there is a change in the Service User's mental or physical health state, and on a routine basis timed according to self-medication Care Plan instructions (as the timing of review will depend on individual needs and risks).
5. The Agency will have an individual medication record for Service Users who self-medicate and their medication supply will be checked in and out, as for other Service Users.
6. Any Service Users who share their home with other people and self-medicate will have a locked facility in their room for the safe storage of medicines. Where the Service User is taking controlled drugs they are also kept in their personal locked storage area together with other medications.
7. Consent must be obtained from the Service User to enable designated Care staff to monitor the medication supply being used by the Service User and to monitor the Service User's continuing ability to manage their medication safely.
8. A full audit of the medications used in self-medication will need to support the tracking of the medication from the point of ordering, receipt into The Agency, the date given to the self-medicating Service User, the usage of the medication by the Service User and the destruction or disposal of any medications that are not required.
9. When Care staff have to prompt self-medicating Service Users to take their medication this should be documented and closely monitored by all staff involved in medication management and administration. If this situation is reoccurring a review of the risk assessment for self-medication for the individual should be carried out and appropriate action taken and documented in the Service User's Care Plan and medication records.

8. Cultural requirements and medication

1. Vegetarians and people from certain religious groups will not want to take gelatin capsules, because they are made from animal products.
2. Some people may prefer to have medications given to them by people of the same gender.
3. Some religious festivals include fasting and some people prefer not to have medicines given at certain times.
4. Followers of the Jewish and Islamic faiths may be concerned about medicines containing substances which are unclean according to the tenets of their faith.

9. Obtaining medication supplies

1. Care staff should either support the Service User in ordering their own repeat prescriptions, or take responsibility for doing it themselves, in which case the responsibility must be clearly noted in the Care Plan.
2. Amounts to be ordered must be checked against the supply required in consideration of any residual medications which are prescribed for intermittent or irregular administration.
3. Prescriptions received from GP practices should be checked against the requested medications before submitting the prescriptions to the pharmacy.
4. Any unexpected medication changes should be checked with the GP before use.
5. A formal process for the arrangements with the dispensing pharmacist should be organised and documented in order to specify the internal medication training and competence assessment programme, and to be available for staff reference.
6. On receipt of the medication the designated staff member must check the dispensed and received medication against the list of prescribed medications and the order, checking the amounts in each individual medication container against that stated on the contained label. Boxed medication should be opened and checked for the amounts contained to minimise error potential and support stock audit.

7. The prescriptions must always be ordered by the person designated for that purpose.
8. The pharmacy should be notified of a person's medication when:
 1. The service begins;
 2. The Service User returns from a period of hospitalisation;
 3. There is any change to the medication.
1. The Care service must also have a formal process for the obtaining of medication prescribed to deal with acute health problems. Medication prescribed as a one-off should be obtained quickly and efficiently and new medication should be supplied as soon as possible, with no delay beyond 24 hours at the latest.
2. Staff ordering medicines must be provided with protected time sufficient to carry out the task accurately.

10. Administration of medication

1. Medication must be given safely to maximise the benefits to Service Users' health and wellbeing.
2. Medication must be managed and administered by procedures and processes which promote Service User independence, choice, privacy, and dignity.
3. Medication management and administration procedures and processes must take account of Service Users' cultural and religious values and beliefs.
4. Service Users must be given an opportunity to self-medicate and be given the range of support and Care needed to enable them to self-medicate when they have been assessed and proved to be able to self-medicate.
5. Where Service Users have consented to their medication to be administered by staff, they should be assured that the staff responsible have been trained and assessed as competent to do so.
6. All Care staff, including those who are not directly concerned with the administration of medicines, should be trained in the understanding of medications, the main types of medications in use, their administration procedures, and how to look for and report possible adverse reactions, including changes which may require review of the Service User's medication prescription.
7. Only staff who have had certificated medication management and administration training and who have been assessed by the manager as competent should be involved in the administration of medication.
8. Medication must be checked, administered and signed for on the medication record by one person who is accountable for all stages of medication with respect to individual Service Users.
9. MEDICATION MUST NOT BE TAKEN FROM ITS ORIGINAL CONTAINER AND GIVEN TO ANOTHER MEMBER OF STAFF TO GIVE THE MEDICATION TO THE Service User as the person checking the right dose for the right person must also witness the person taking the medication, and must be sure that the medication has been taken properly by the Service User.
10. Carers must not administer insulin injections.
11. Staff administering medicines must be provided with protected time sufficient to carry out the task accurately.

11. Storage of medication

1. Medications must be stored in safe place.
2. Where medicines requiring refrigeration are in use, the accommodation must have a fridge and the daily temperature recordings will monitor the temperature requiring MIN 2⁰C and MAX 8⁰C. The fridge will be large enough to store all the medications requiring refrigeration.
3. External medications such as creams and lotions should be stored separately from other medications.
4. Dressings, catheters, and ostomy products will be stored separately from other medications.



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5. There will be adequate storage facilities to ensure all medications are stored correctly and that no medications will be stored on the floor.
6. If required, there will be, on the premises where they are used, a suitable waste disposal unit for paper waste, and a sharps clinical waste disposal bin for any used or waste needles and sharps.
7. Individual locked medication cupboards in Service User rooms may be used for self-medication and for administration by Care staff.

12. Record keeping

Record what you do when you do it. As medicines are given they should be recorded immediately and signed for by the person responsible immediately following the administration, and before moving on to the next Service User.

1. Record any medications not taken, with reason.
2. Correct mistakes with a single line through the text, accompanied by a signature and date and time. Never use correction fluid.
3. Record medicines given by other professionals, such as a visiting health professional.
4. Records will be clear, complete, legible, written in black ink dated and signed to say who has made the record.
5. The intention of the medications records is to allow a full stock quantity reconciliation to be carried out without warning at any time. Therefore records must include:
 1. Receipt;
 2. Dispensing;
 3. Destruction or disposal for whatever reason;
 4. Transfer to another person or facility.
1. The medication administration record for an individual Service User will include the name of the Service User, date of birth, weight, name of the drug, the dose, and time to be given, and any special requirements, e.g. with food only.
2. Hand written medication administration records should be produced only in exceptional circumstances, and created by a member of staff with the appropriate medicines administration training for the setting. The hand-written record must be checked and verified by a second member of staff with the same training before first use.
3. For self-medicating Service Users the date and the name of the Care staff who gave the Service User their supply of medication must be recorded in their medication administration record, as well as in the Care Plan, and must be accounted for by their signature.
4. Where records are held on computer these must comply with data protection regulations, must be tamper-proof, and must give clear indications of who has made the record entry.
5. ALL STAFF INVOLVED IN MEDICATION MANAGEMENT AND ADMINISTRATION ARE RESPONSIBLE FOR ACCURATE RECORD KEEPING.
6. Medication administration records must record:
 1. The medications are prescribed for the person;
 2. Where, when and the quantity of any medications received;
 3. The time they must be given;
 4. The dose of the medication;
 5. Any special administration requirements;
 6. The name and designation of the person making the record;
 7. Copies of emails, texts, faxes, and transcriptions of phone messages must be kept and stored with the medications Care Plan.
1. Disposal of medication records must record:
 1. Date of disposal/return to the pharmacy;

2. Name and strength of medication;
3. Quantity being disposed of;
4. Person for whom medication was prescribed or purchased;
5. Signature of the person disposing the medication and a witness signature for the disposal.
6. **Hand-over.** Staff must ensure at shift handover that all information regarding all changes to medications administration during that shift is communicated to the incoming staff, and they have received and understood the message.

13. External medication application

1. Medication applied to the Service User's skin should be applied by a staff member wearing disposable gloves. Creams and lotions must be applied according to the prescription instructions.
2. Instructions for creams and lotions must be clear.
3. Before Care staff members apply creams and lotions they must be trained and assessed for their knowledge and competence related to the application of external medication.
4. Clear information must be available to inform Care staff as to what the Service User's cream is for how much to apply, where precisely to apply the cream, the frequency of application and for how long the application is to continue.
5. There must be a Care Plan for the application of the cream and a daily account of its application.
6. Creams can be stored in a locked cabinet in the Service User's room unless the cream requires refrigeration.

14. Controlled drugs

1. There are special legal requirements for the ordering, storage and disposal of all controlled drugs.
2. Controlled drugs are prescribed differently.
3. Controlled drugs if collected from a pharmacy must be accounted for in the pharmacy controlled drugs register.
4. Anyone collecting controlled drugs from a pharmacy will need to produce personal identification and sign for receipt of the medication.
5. Separate records for receipt administration and disposal of controlled drugs must be kept.
6. All controlled drugs must be accounted for on the Service User's medication administration record for receipt, times of administration and destruction or disposal. They must also be fully accounted for in the hardback controlled drug register.
7. The register must have numbered pages and each controlled drug for each person must have a separate page to account for receipt, administration, disposal, and stock audit, which must be accounted for each time the medication is given.
8. Where a Service User is self-medicating and taking prescribed controlled drugs these can be kept in the Service User's locked medication facility. Receipt of, and the date that, the controlled drugs are given to the Service User should be accounted for in the controlled drugs register and in the Service User's medication record.

Disposal of controlled drugs

1. In a Care Agency without nursing the controlled drugs to be returned to the pharmacy must be accounted for in the controlled drug register, the receiving pharmacist must account in the register for the receipt of the drug to be disposed. This must also take place with a dispensing GP where appropriate.
2. In a Care Agency with nursing arrangements for controlled drugs disposal should be discussed with the community pharmacist and an authorised member of the Clinical Commissioning Group contacted regarding disposal.

Storage of Temazepam and Fentanyl



1. Both of these drugs require storage within the Controlled Drugs storage facility, and accounting for in the same way as Controlled Drugs, even though they are not Controlled Drugs. This is a requirement of the Registration Authority, which overrides any otherwise legally compliant advice given by a Community Pharmacist.

15. Non-prescribed controlled drugs

2. If a Service User is found to have non-prescribed drugs in their possession then the local police should be contacted for their advice.

16. Covert medication

3. This is the term used when medications are hidden and given without the consent of the Service User.
4. Covert medication must not be given to a Service User who has the capacity to give their consent or refusal with respect to receiving medical treatment.
5. GIVING MEDICATION BY DECEPTION IS POTENTIALLY AN ASSAULT.
6. The covert administration of medication should only take place within the context of existing legal and best practice frameworks to protect the person receiving the medication and the Care staff involved in giving the medication.
7. Covert medication must not be used without, firstly, a formal assessment of the Service User's capacity and, where they lack or may lack capacity, that administration of medication is in the person's best interest. Where covert administration of medication may constitute a deprivation of the person's liberty, a referral to the supervisory body responsible for authorizing Deprivation of Liberty Safeguards should be sought. Authorisation for covert medication must be obtained before covert administration is used.
8. Before medication can be given covertly, the Service User's doctor and pharmacist must be consulted and must give their written authority for the medication prescription and method of administration to be used.
9. Where covert medication is formally agreed there must be a clear assessment and Care Plan to demonstrate the need for covert medication and the individual review must be stipulated by the responsible doctor and professional authorisation teams. Reviews must be carried out following any change in the Service User's capacity.
10. A useful publication is "Covert Medication", published by the Mental Welfare Commission for Scotland.

17. Adverse drug reactions

1. If a Service User becomes unwell after taking a new medication then the prescribing GP should be notified immediately, or it may be necessary to contact the emergency ambulance service if the reaction is severe.
2. An incident report must be completed and submitted to the Registered Manager for review.

18. Inappropriate use of medication

1. MEDICATION MUST NOT BE USED AS A FORM OF RESTRAINT TO SEDATE PEOPLE FOR THE CONVENIENCE OF THE CARE SERVICE STAFF. THIS IS ABUSE.
2. Medication must only be administered to the person who has been prescribed that medication.
3. Partly used medication dispensed for an individual, and no longer required, must not be used for any other person.
4. Where several Service Users have the same medication, the medication must only be administered from the container marked with the Service User's name. This must be clearly accounted for in the drug stock audit.
5. An incident report must be completed and submitted to the Registered Manager for review.
6. Administration of medicines inappropriately, whether by design or accident, is potentially abuse and must be reported immediately to the Registered Manager, and the formal safeguarding process initiated.

19. Medication errors



1. Any medication error must be immediately reported to the GP of the Service User affected by the medication error and any advice given acted upon to protect the safety of the Service User. If a GP cannot be contacted immediately then NHS Direct and the local pharmacy should be contacted for advice.
2. The incident and all action taken must be documented in the Service User's Care Plan, medication record, and an appropriate incident form completed by the person responsible.
3. The Service User and their personal advocate must be fully informed by the service manager of the incident and the actions taken in order to minimise the risk to the Service User.
4. An incident report should be made to the regulatory authorities regarding the incident and actions taken by the service management following the medication incident to investigate causal factors and future medication error prevention strategies to be acted upon.
5. All staff involved in medication error incidents should have an immediate formal supervision by their line manager to offer support and to support investigation of the incident.
6. All medication errors should be documented on an incident report, reported to the manager and investigated to prevent further errors occurring.
7. Any mistake with medication management or administration should be treated as an incident or error and recorded as such, and brought to the attention of the person in charge of the service at the time, and to the attention of the service manager.
8. Examples of medication errors:
 1. Medication given to the wrong person;
 2. The wrong dose is given, i.e. too much or too little;
 3. Medication is not given.
1. All staff should be encouraged to promptly report medication errors
2. All medication errors should be investigated, with any necessary changes to training and procedures being made immediately.
3. All actions taken should be recorded.
4. All serious incidents should be reported to the professional regulatory body.

20. Nutritional supplements

1. There must be a Care Plan for any nutritional supplements used for the Service User. These must be supported by an assessment of need and a monitoring and review record to document when the supplements are given, by whom and the measured quantity taken by the Service User accurately recorded and accounted for.
2. The medication record must be signed when the supplement has been taken by the Service User and accounted for by the person who is responsible for giving and witnessing the Service User taking the supplement. A daily record of nutritional supplements taken or refused by the Service User must be recorded in the Care Plan daily record.
3. Refusal to accept nutritional supplements must be treated as other medication refusal and reported to the prescribing professionals and recorded as refused on the medication record by the accountable staff member.
4. A regular review of the use of the nutritional supplements should be requested by the Care service and this will be made by the prescribing GP or the dietician.
5. All medications given must be recorded at the time they are given and also when a person refuses their medication.
6. If a person cannot swallow then their medication advice must be obtained from a health Care professional and alternative liquid medication could be prescribed. **MEDICATION MUST NOT BE CRUSHED OR CAPSULES SPLIT TO GIVE TO Service Users** as this may affect the way medicines work and can be potentially harmful to the Service User.

21. Homely remedies

1. Where a Service User wishes to take homely remedies the GP should be contacted for advice and to arrange their agreement and consent in consideration of other medicines currently being taken.
2. When a provider decides to use homely remedies for minor ailments this must be formalised in a policy about which non prescribed medications will be used and which minor ailments are to be treated by these. This policy must be formally shared with all staff responsible for medication management and administration. QCS are unable to provide this policy because it can only be locally determined, therefore a blank form is provided with this Policy for local completion; the completed document can be uploaded to the QCS system for publication within the provider organisation.
3. Relatives who buy homely remedies for Service Users should be encouraged to contact the GP or pharmacist for their advice regarding proposed homely remedy medication.
4. Advice regarding the use of homely remedies must be obtained from a doctor, pharmacist, or nurse.
5. Homely remedy policy for the Care agency must be shared with GPs, Service Users and their personal representatives.
1. Records must be kept of the purchase, administration and disposal of medication.

22. Advice from a pharmacist

2. All services should have an arrangement with a local Pharmacy or dispensing GP to provide advice to staff concerning medication management and administration.
3. Medication supply from a community pharmacist should be of appropriate quality and suitably labelled for its intended purpose.
4. Dispensing services should be:
 1. Accurate;
 2. Accessible;
 3. Prompt;
 4. Reliable.

23. Changing a pharmacy supplier

1. Agree the change of date with the new pharmacy.
2. Agree the procedure for requesting repeat prescription orders and any documentation by the pharmacy or the GP practice.
3. Arrange for prescription repeats a few weeks prior to commencement of the new supply.
4. Ensure that current medication stocks are used before reordering.
5. Dispose of any medication from the previous supplier that is no longer required by the Service User before the changeover date. Service User consent to medication disposal must be gained before doing so.

24. Monitored dosage systems and compliance aids

1. MDS do improve some procedures including:
 1. The system of organising repeat prescriptions for Service Users;
 2. A visual check whether medicines have been prepared and given to the Service User.
3. Medication NOT to be packaged in a monitored dosage system:
 1. Medicines that are sensitive to moisture, e.g. effervescent tablet;
 2. Light-sensitive medicines, e.g. chlorpromazine;

3. Medicines that should only be dispensed in glass bottles, e.g. glyceryl trinitrate (GTN);
4. Medicines that may be harmful when handled, e.g. cytotoxic products like methotrexate;
5. Medicines that should only be taken when required, e.g. painkillers;
6. Medicines whose dose may vary depending on test results, e.g. warfarin.

7. Liquid medicines, creams, eye drops, inhalers must be supplied in traditional containers. Therefore, any Care agency that uses MDS will have two different systems operating.

8. Some Care providers who have been unable to get medicines in MDS have allowed Care workers to re-package medicines in compliance aids. This is also known as 'secondary dispensing'.

9. Repackaging of medicines by Care workers should NOT take place. The risk of making a mistake is too great.

25. Ordering, storage and administration of “when required” (PRN) medication

Medication with a “when required” (PRN) dose is usually prescribed to treat short term or intermittent medical conditions, i.e. it is **not** to be taken regularly. Therefore, it should be approached differently to regular repeat medication.

1. Recording PRN Medication in residents’ Care Plans

Staff administering PRN medication must ensure the medication is given as intended by recording a specific plan in the residents’ Care Plan which should be kept with the MAR chart. The details of which should include:

1. The details of the medication, i.e. name of drug and strength.
2. Signs and symptoms – specific to the individual
3. The precise reason for dispensing, for instance 'in the event of lower back pain'.
4. Why it has been prescribed and how to give it?
5. How long for? If advised by the prescriber.
6. When the medication should be given, e.g. before, with or after food.
7. The maximum daily amount or the time to leave between doses **must** be recorded. E.g. Paracetamol – 2 tabs every 4-6hrs when necessary (**max 4 doses in 24hrs**).

8. Ordering PRN Medication

PRN medication by its nature should not need to be included in the monthly repeat medication, this will help to minimise waste.

1. PRN medication should always be supplied in an original box rather than a monitored dosage system (MDS). This allows for a check on the expiry date and reduces waste.
2. PRN medication that is still in use and in date should be carried over from one month to the next and **not** disposed of.
3. A record of the quantity carried over must be recorded on the new MAR chart so there is an accurate record of the quantity in stock.

4. Storing PRN Medication

Please follow The Agency's medication policy for storage of medicines.

5. Administration of PRN Medication

Staff administering medication should have the appropriate training and follow the procedures set out in The Agency's medication policy. However, when administering PRN medication the following points need to be considered:-



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1. PRN medication should not be offered or given only at the times listed on the MAR chart or at specific medication rounds. As it is for occasional use the person should be offered the medication at the times they are experiencing the signs and symptoms either by telling a member of staff or by staff identifying the residents' need as **outlined in the Care Plan**.
2. The exact time the medication was given and the amount given (e.g. 1 or 2 tablets if prescribed as 1-2 tablets prn,) should be recorded on the MAR.
3. A record does not have to be made at each medical round to show the person has been offered the medication. However, the Care Plan should demonstrate that staff know what the medication is for and have made an assessment on whether the person requires the medication.
4. Consideration should be given to the residents' capacity to refuse the medication.
5. The Care Plan should show the needs of the person e.g. If signs of pain are expressed in a non-verbal way.
6. **Other areas to be considered**
 1. If a PRN medicine is administered on a regular basis a referral to the prescriber should be considered for a review of the resident's medication. This action must be clearly recorded in the resident's Care Plan.
 2. Should the PRN medication not have the expected effects the prescriber should be contacted. This action must be clearly recorded in the residents' Care Plan.
 3. Paracetamol: If body weight is <39kgs consider giving 1 or 2 tablets up to four times a day, in order to stay within safe toxicity limits of a maximum dose of 100mg/kg bodyweight per 24 hours.
4. **The return of unused/ unwanted PRN Medication**
 1. Please refer to The Agency's policy for the control of medicines for homes.

26. Essential Oils

Use of essential oils.

2. Always check the safety of every essential oil on each Service User with that Service User's General Practitioner before use.
3. Essential oils should be used only by a practitioner qualified in their use.
4. Never take essential oils internally.
5. Keep essential oils in locked storage, separate from any other medications.
6. Always dilute essential oils with suitable carrier oil before applying it to the skin.
7. If you are using a new, untried essential oil, first do a skin patch test before using extensively.
8. If irritation occurs with a specific oil or formula, discontinue the use of such oil or formula.
9. Take care not to get any essential oils into eyes or on to any mucus membranes.
10. A wise practice is to always wash hands after handling pure undiluted essential oils.
11. When using essential oils that cause sun sensitivity people should take Care to avoid exposure to the sun.
12. People with a sensitive skin should always perform a skin patch test before using untried essential oils.

MEDICATION FOLLOWING SERVICE COMMENCEMENT

27. Medication assessment

1. Pre-commencement assessments of medication taken by the Service User should be made to include medication name, dose, time of taking, reason for taking and Service User understanding regarding their medication.
2. On commencement, the Service User's formal consent will be sought, and recorded, in order to allow the designated Care staff to support the Service User with the management of their medication supply.
3. Medication assessments and associated Care Plans will be carried out by staff who have been trained and assessed as competent for the management and administration of Service Users' medication.
4. All Service Users will have a formal, documented assessment for self-medication.
5. All Service Users will have a formal, documented assessment of their individual needs and specialist requirements in relation to their medications.
6. The assessment will include any potential side effects or known allergies associated with medication for the individual Service User.
7. On commencement day all medications will be recorded on the Service User's medication assessment.
8. Following the assessment the information will be used to formulate a medication Care Plan which will stipulate medication names, doses, times of administration, route of medication, and any special requirements the Service User has in relation to individual medications and their safe management.
9. Any changes made by the GP will be noted and appropriate changes made in the records and in the medication supply. This will be discussed with the Service User where appropriate, and with their consent any medication no longer required by the Service User will be returned to the pharmacy.
10. Service User will be advised that they should seek the advice of an independent advocate if they have concerns regarding their medication.

28. Medication review

1. Medication review can be made by the prescribing GP, community pharmacist, or prescribing specialist community nurse.
1. Medication review should take place:
 1. As soon as possible following commencement;
 2. When medication side effects are noted;
 3. Where the Service User's health state changes;
 4. When there are any recommendations by the Department of Health regarding particular medications taken by the Service User;
 5. When the Service User is discharged from hospital and there have been changes in their medication.

29. Medication on temporary transfer or absence

6. The supplying pharmacist will be able to advise the Care service and be able to offer support by providing a special container for medication at the time when the Service User is likely to be on leave.
7. If a Service User is likely to be out of their home for one or more medication administration times then the Service User should be given the correct medication, clearly named and with clear written instructions stipulating the medication dose and time to be taken:
 1. These medications should be entered onto the medication record as given to the Service User but not administered by the staff;
 2. A risk assessment of the Service User's ability to manage the medication should be documented in the Care Plan by the staff responsible for the medication arrangements at the time the Service User leaves the building;



3. Where a Service User is unable to self-medicate the staff member escort will be trained and competent in the administration of medication, will take responsibility for the medication, and will complete the medication records on return to The Agency.
1. Ensure that any person responsible for administering or assisting with self-administration of medicines while off the premises is aware of:
 1. What medicines the Service User needs to take;
 2. Clear directions on how, when and how much of each medicine to take;
 3. What time the last dose of each was taken;
 4. Contact numbers for queries, such as The Agency, pharmacy, or GP.
5. On return to The Agency the senior designated staff should check with the Service User to ensure that their medication has been taken correctly and record the answer on the medication administration record.

30. Medication on permanent transfer

6. On permanent transfer to another care facility a copy of the current assessment, most recent review and the current medication administration chart must be transferred together with the Service User.
7. Record the transfer of information in the Service User's documentation.
8. Original copies of all documents relating to medications for that Service User must be retained in accordance with normal practice for storage and retention.

31. Sharing of information on medications

1. Medications information regarding a Service User is confidential and must only be shared with persons or organisation who may have need to temporarily administer medicines to the Service User, or is responsible for reviewing the medications, such as a hospital.
2. The Service User must be informed of the proposed information sharing and authorise it.

32. Disposal of medication

1. Medicines must be disposed of as soon as they cease to be currently prescribed, or the person for whom they are prescribed is no longer present.
2. Where the Service User has left the service or died, see the 'Medication on discharge' or 'Medication on death' sections of this policy.
3. Where medications for existing Service Users require disposal:
 1. A complete written record of all medicines to be disposed of should be made;
 2. Services providing Care in Service Users' own homes are able to return medication to the dispensing service which must have an appropriate arrangement for medication disposal in order to meet current regulatory requirements for waste management;
 3. All unused medications will be returned to the pharmacy, covered by a written record of the returns on the form provided in this policy, with signatures against each item.

33. Verbal telephoned, video link or online medication instructions

1. Changes of drug doses are sometimes ordered by GPs over the telephone. This must be by exception only. There must be a formal procedure for this and all staff must be made aware of the process by internal training and supervision.
2. The prescriber must establish the capacity and consent of the Service User, and the training and capacity of the staff before prescribing when not present.
3. Records of telephone medication messages should include:
 1. Who took the call;
 2. The time of the call;

3. The name of the person who called;
 4. Record of changes made.
-
1. Before ending the call:
 1. Read back the information that has been written down to reduce the chance of misunderstandings;
 2. Spell out the name of the medicine(s);
 3. Request written confirmation by fax, letter or by issue of a new prescription, without delay, and before the next administration of the medicine.
 4. Staff must change the Service User's medications records immediately on receipt of a "not present" prescription.
 5. Staff must record the time and date of the change, and ensure that a second administration of that medicine does not take place before written confirmation is received.

PROCEDURE FOR MEDICATION ADMINISTRATION Remember the 5 x R's:

1. **Right Medication**
 2. **Right Dose**
 3. **Right Way**
 4. **Right Person**
 5. **Right Time**
-
1. **These are the main five rights, some publications may list more, such as 'Right Documentation' or 'Right Reason' which you may choose to incorporate into your procedure.**
 2. Check you are giving the medication to the right person. Ask them for their name, check recent photograph, and cross reference name and, if appropriate, their room number on the medication record. Make sure that you know the person by name. If in any doubt check with another member of the Care team or contact the administration office who will know the Service User well.
 3. Select all of the correct medication for the time of day for the individual Service User. ALWAYS CHECK THE MEDICATION RECORD. DO NOT RELY ON MEMORY OR ADMINISTER ONLY WHAT IS IN FRONT OF YOU.
 4. Ask the person if they are experiencing the condition for which the medication has been prescribed.
 5. Give the medication with a tumbler of water (or milk if required by the prescription) and encourage the person to sit in an upright position whilst swallowing the medication.
 6. If the tablets or capsules are in a monitored dosage pack, open the appropriate section and empty the tablets/capsules into a medicine pot and hand it directly to the Service User.
 7. Transfer the medication from the bottle or pack into a medication pot and give this directly to the Service User.
 8. Liquid medication must be measured into a clearly graduated and marked medication pot or by using an appropriate sized syringe which clearly identifies individual millilitre markings.
 9. Medication must not be handled but transferred to the medication pot in a non-handling clean method.
 10. Some medications may be harmful if handled by the Care worker and disposable plastic gloves must be available and worn where there is an identified risk. This may also apply where the Service User is unable to handle medication and they require extra support, although the handling of drugs should be avoided and medication spoons used to aid administration where difficulties are identified.
 11. Medication doses affected by the latest blood results should have a copy of the latest results kept with the medication administration record.
 12. Ask if they want to take their medication before removing them from the pack. If they refuse try again a little later. Refusal must be documented and the GP or pharmacist telephoned for advice. NEVER FORCE ANYONE TO TAKE MEDICATION AND IT MUST NOT BE HIDDEN IN FOOD OR DRINK.
 13. Medicines to be taken when required (PRN) should be offered to people experiencing pain, and the time and dosage noted in the medication administration record.

Reviewed by Marcus Kerridge McColl
November 2025

Mental Capacity Act 2005 Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To meet the provisions of the Mental Capacity Act 2005.

Scope

2. All Service Users who lack mental capacity as defined under the Act.
3. The Mental Capacity Act 2005 protects both the Service User and the Care worker provided the law is adhered to.

Policy

4. All Service Users assessed as lacking mental capacity will be subject to the provisions of the Act. §
5. The organisation will have access to:
 1. The Act;
 2. The Code of Practice.
3. Each of these documents is available in the Useful Documents section of your QCS management system.

Mental Capacity Act 2005 – Summary

4. The Mental Capacity Act came into force in April 2007 and was implemented in October 2007.
5. The Mental Capacity Act was substantially amended by the Mental Health Act 2007 to provide for a procedure to authorise the deprivation of the liberty of a mentally incapacitated person. This part of the Act is called the Deprivation of Liberty Safeguards (DOLS). The policy and procedure regarding this part of the Act is contained in the Deprivation of Liberty Safeguards Policy and Procedure.
6. The Mental Capacity Act 2005 provides a statutory framework to empower and protect vulnerable people who are not able to make their own decisions. It makes it clear who can take decisions, in which situations, and how they should go about this. It enables people to plan ahead for a time when they may lose capacity.
7. Guidance on the Act is provided in a Code of Practice. Everyone caring for, or working with an adult who may lack capacity to make specific decisions must comply with this Act. There is a Code of Practice which provides guidance and information as to how the Act works.
8. The whole Act is underpinned by a set of five key principles:
 1. A presumption of capacity – Every adult has the right to make his or her own decisions and must be assumed to have the capacity to do so unless it is proved otherwise;
 2. The right for individuals to be supported to make their own decisions – People must be given all appropriate help before anyone concludes that they cannot make their own decisions;

3. That individuals must retain the right to make what might be seen as eccentric or unwise decisions;
4. Best interests – Anything done for or on behalf of people without capacity must be in their best interests; and
5. Least restrictive intervention – Anything done for or on behalf of people without capacity should be the least restrictive of their basic rights and freedoms.

What does the Act do?

6. The Act enshrines in statute best practice and common law principles concerning people who lack mental capacity and those who take decisions on their behalf. It replaced previous statutory schemes for enduring powers of attorney and Court of Protection receivers with reformed and updated schemes.
7. **Assessing lack of capacity** - The Act sets out a single clear test for assessing whether a person lacks capacity to take a particular decision at a particular time. It is a 'decision-specific' test. No one can be labelled 'incapable' as a result of a particular medical condition or diagnosis. Section 2 of the Act makes it clear that a lack of capacity cannot be established merely by reference to a person's age, appearance, or any condition or aspect of a person's behaviour which might lead others to make unjustified assumptions about capacity. A form to be used in conjunction with the assessment test for capacity is included towards the end of this policy. The test for capacity is a two stage test. Firstly, is there an impairment of or disturbance in the functioning of a person's mind or brain? Secondly, is the impairment or disturbance sufficient that the person lacks the capacity to make a particular decision? In deciding this you should establish if the person can do all of the following. If not then we should say the person lacks capacity to make that particular decision.
 1. Understand information about the decision to be made?
 2. Retain that information in their mind?
 3. Weigh that information as part of the decision-making process?
 4. Communicate their decision?
1. **Best Interests** - Everything that is done for or on behalf of a person who lacks capacity must be in that person's best interests. The Mental Capacity Act does not define what it means to make a decision in someone's best interests. In considering what is in someone's best interest we would consider the factors outlined in the section of this policy headed 'Best interest'. The Act provides a checklist of factors that decision-makers must work through in deciding what is in a person's best interests. A person can put his/her wishes and feelings into a written statement if they so wish, which the person making the determination must consider. This statement is an expression of wishes, and is not the same as an advance decision to refuse treatment which is referred to later in the policy.
2. Acts in connection with Care or treatment: Section 5 clarifies that, where a person is providing Care or treatment for someone who lacks capacity, then the person can provide the Care without incurring legal liability. The key will be proper assessment of capacity and best interests. This will cover actions that would otherwise result in a civil wrong or crime if someone has to interfere with the person's body or property in the ordinary course of caring, for example, by giving an injection or by using the person's money to buy items for them.
3. **Restraint/deprivation of liberty** - Section 6 of the Act defines restraint as the use or threat of force where an incapacitated person resists, and any restriction of liberty or movement whether or not the person resists. Restraint is only permitted if the person using it reasonably believes it is necessary to prevent harm to the incapacitated person, and if the restraint used is proportionate to the likelihood and seriousness of the harm.
4. Section 4A prohibits the deprivation of the liberty of the person unless deprivation is authorised by the Deprivation of Liberty Safeguards in Part 2 of the Mental Capacity Act. Where a person may be being accommodated in circumstances that amount to a deprivation of liberty, authorisation under the DOLS arrangements should be sought. The authorisation process is described in the Deprivation of Liberty Safeguards Policy and Procedure.
5. The Act deals with two situations whereby a designated decision-maker can act on behalf of someone who lacks capacity:
 1. **Lasting powers of attorney (LPAs)** – The Act allows a person to appoint an attorney to act on their behalf if they should lose capacity in the future. There are two types of LPA, one to make and health and welfare decisions, and the other to make finance and property decisions. The provision replaces the previous role of Enduring Power of Attorney (EPA). An LPA can only be appointed by someone who has the capacity to do this. Staff should be aware of any LPA in place for service users in their care.
 2. **Court appointed deputies** – The Act provides for a system of court appointed deputies to replace the previous system of receivership in the Court of Protection. Deputies are able to take decisions on welfare, healthcare and financial matters as authorised by the Court but are not able to refuse consent to life- sustaining treatment. They are only appointed if the Court cannot make a one-off decision to resolve the issues, and if the person has already lost



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capacity to make these decisions. Staff should be aware of any Court appointed deputies in place for service users in their care.

1. The Act created two new public bodies to support the statutory framework, both of which are designed around the needs of those who lack capacity:
 1. **A new Court of Protection** – The new Court will have jurisdiction relating to the whole Act and is the final arbiter for capacity matters. It has its own procedures and nominated judges.
 2. **A new Public Guardian** – The Public Guardian and his/her staff are the registering authority for LPAs and deputies. They supervise deputies appointed by the Court and provide information to help the Court make decisions. They also work together with other agencies, such as the police and social services, to respond to any concerns raised about the way in which an attorney or deputy is operating.
3. The Act also includes three further key provisions to protect vulnerable people:
 1. **Independent Mental Capacity Advocate (IMCA)** - An IMCA is someone appointed to support a person who lacks capacity but has no one to speak for them. The IMCA makes representations about the person's wishes, feelings, beliefs and values, at the same time as bringing to the attention of the decision-maker all factors that are relevant to the decision. Staff should be aware of how to refer to an IMCA. This information can be found out from the Local Authority or from the Citizens' Advice Bureau in the area.
 2. **Advance decisions to refuse treatment** - Statutory rules with clear safeguards confirm that people may make a decision in advance to refuse treatment if they should lose capacity in the future. It is made clear in the Act that an advance decision will have no application to any treatment which a doctor considers necessary to sustain life unless strict formalities have been complied with. These formalities are that the decision must be in writing, signed and witnessed. In addition, there must be an express statement that the decision stands "even if life is at risk". An advance decision to refuse treatment that is not life sustaining does not have to be recorded in writing.
 3. **A criminal offence** - The Act introduces a new criminal offence of ill treatment or neglect of a person who lacks capacity. A person found guilty of such an offence may be liable to imprisonment for a term of up to five years.

Best Interests

1. It is important not to make assumptions about someone's best interests merely on the basis of the person's age or appearance, condition or any aspect of their behaviour.
2. The decision-maker must consider all the relevant circumstances relating to the decision in question.
3. The decision-maker must consider whether the person is likely to regain capacity (e.g. after receiving medical treatment). If so, can the decision that is being made on their behalf wait?
4. If the decision concerns the provision or withdrawal of life-sustaining treatment, the decision-maker must not be motivated by a desire to bring about the person's death.
5. The decision maker must in particular consider:
 1. The person's past and present wishes and feelings (in particular if they have been written down).
 2. Any beliefs and values (e.g. religious, cultural or moral) that would be likely to influence the decision in question and any other relevant factors.
 3. As far as possible, the decision-maker must consult other people if it is appropriate to do so and take into account their views as to what would be in the best interests of the person lacking capacity, especially:
 1. Anyone previously named by the person lacking capacity as someone to be consulted.
 2. Carers, close relatives or close friends or anyone else interested in the person's welfare. ☒
 3. Any attorney appointed under a Lasting Power of Attorney.
 4. Any deputy appointed by the Court of Protection to make decisions for the person.

Procedure

5. Maintain awareness of the Mental Capacity Act principles and the DOLS.



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6. If the registration of the service may allow for admission of persons assessed as lacking mental capacity, train key staff using the DOH-provided training resources which can be accessed at:
<http://www.scie.org.uk/publications/guides/guide44/workingbettertogether/training.asp>.
7. Seek advice from the QCS Customer Care Team (**0333 405 33 33 option 2**) in the event of any questions arising.
8. Documents and resources about the Act, including training resources, are available under Useful Documents section of your QCS management system.

Four Steps to remember when assessing a person's mental capacity

1. Bear in mind the key principles which emphasise the fundamental concepts and core values of the Act (noted previously in this document under summary of Mental Capacity Act 2005).
2. Complete the attached MCA Assessment with the Service User. Having a member of staff or a family member sit with the Service User whilst you complete the assessment is intended to help them relax and feel comfortable with you.
3. Make sure the form is completed fully and that it is signed by the assessor and that it is dated. This form should be kept with the Care Plan so it is readily available.
4. If it is determined that the Service User does not have the mental capacity to make a particular decision, any action taken or any decision made must be in his or her best interest.

What do I do if there is a dispute about best interests?

1. Family and friends will not always agree about what is in the best interests of an individual.
2. If you are the decision-maker you will need to clearly demonstrate in your record keeping that you have made a decision based on all available evidence and taken into account all conflicting views.
3. If there is a dispute, the following things might assist you in determining what is in the person's best interests:
 1. Involve an advocate who is independent of all the parties involved;
 2. Get a second opinion;
 3. Hold a formal or informal case conference;
 4. Go to mediation;
 5. An application could be made to the Court of Protection for a ruling;
 6. You should ensure that all documents you complete are both signed and dated.

Reviewed by Marcus Kerridge McColl
November 2025

Minimum Wage Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with legislation.

Scope

- All workers above school-leaving age.

Policy

- The organisation will comply with the minimum wage regulations.

Guidance to the Service Provider

General

Entitlement to the NMW



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- Most workers in the UK who are over the compulsory school-leaving age are legally entitled to be paid at least the NMW, and all employers have to pay it if the employee is entitled to it. It makes no difference:
 - If the employee is paid weekly or monthly, by cheque, in cash or in another way;
 - If the employee works full time, part time or any other working pattern;
 - If the employee works at the employer's own premises or elsewhere;
 - What size the employer is;
 - Where the employee works in the UK.
- The employee is entitled to the NMW even if they sign a contract agreeing to be paid at a lower rate, either of the employee's own free will or because the employer persuades or makes the employee sign. The contract will have no legal effect and the employee must still be paid the proper rate.

How to check pay

- The average hourly pay has to be at least the NMW, worked out over a period called the 'pay reference period'.
- To find out whether an employee is receiving the NMW there is a need to know what counts towards it and what does not. There is also a need to know which hours are entitled to be paid for. This depends on which of the four types of work under the NMW rules the employee is doing.

Time Work

- If the employee is paid according to the number of hours they are at work, they are doing time work – so anybody whose pay goes up or down depending on the actual hours they work is likely to be doing time work. This includes workers who are paid piece rates but have to be at a factory for a set numbers of hours a day to produce their work.
- Time workers must be paid the NMW for hours spent:
 - At work and required to be working or on standby near the workplace (but not on rest breaks);
 - When kept at the workplace but unable to work because of machine breakdown;
 - Travelling on business during normal working hours;
 - Training or travelling to training during normal working hours;
 - Awake and working, *and* during 'sleeping time' if they are required to remain on the premises either by their employment contract or to fulfill other regulations.
- Sleeping time means any time when the employee is allowed to sleep, if the employer arranges for the employee to sleep at or near where the employee works and provides them with suitable facilities for doing so.
- Time workers do not need to be paid the NMW for hours spent:
 - Travelling between home and work;
 - Away from work on rest breaks, holidays (Different arrangements apply, which may nonetheless amount to the NMW.), sick leave (Statutory Sick Pay may be payable) or maternity/paternity/adoption leave (Statutory Maternity / Paternity / Adoption Pay may apply);
 - Away from work because of industrial action.

Salaried Hours Work

- If the employee is paid under the contract for a set basic number of hours a year, and receives an annual salary paid in equal weekly or monthly instalments, the employee is a salaried hours worker. The contract does not have to state the hours as an annual figure (e.g., 2,000 hours a year), but it must be possible to work out from the contract what the basic annual hours are.

- Salaried hours workers must be paid the NMW for hours spent:
 - At work and required to be working;
 - On standby, or on call, at or near the place of work;
 - When kept at the workplace but unable to work because of machine breakdown;
 - Travelling on business during normal working hours;
 - Training or travelling to training during normal working hours;
 - Awake, and working, during 'sleeping time' if they are required to remain on the premises either by their employment contract or to fulfill other regulations;
 - Absences such as rest breaks, lunch breaks, holidays, sick and maternity leave will need to be counted *if* they are included in the basic minimum hours under the contract. (So, if the contract says the employee is employed for 35 hours per week and that includes their lunch break, then you must count that time into the number of hours they work every week or month for calculating the minimum wage).
- Salaried hours workers do not need to be paid the NMW for hours spent:
 - When paid less than the normal pay because the worker is absent from work, for instance if the employee only receives half-pay while on sick leave;
 - On any unpaid leave which the employer allows the employee to take;
 - Away from work because of industrial action.

The pay reference period

- This is usually the period of time that the employee is actually paid for. For example, if employees are paid weekly the pay reference period is one week; if they are paid monthly it is one month. Employees must be paid, on average, at least the NMW during each pay reference period.
- Under the NMW rules, the pay reference period cannot be longer than a month. If the employer pays at longer intervals (e.g. once a quarter) employees must still get the NMW on average each month during that quarter.
- When working out whether the employee is receiving the NMW, the pay that counts may not just be the pay the employee receives during the pay reference period, it also includes pay which the employee earns during that period, but does not receive until the next one. For example, if the employee is paid monthly and does some overtime near the end of July they might not be paid for it until August. The overtime pay will still count towards the July pay.

Pay that counts towards the NMW

- The pay the employee receives that can be used to calculate the NMW is known as the NMW pay.
- NMW pay is calculated on gross pay (before tax and National Insurance have been taken off). The gross pay includes the basic pay for the work done and other types of pay which count towards the NMW. For example, sales commission, performance-related pay or other payments based on how well the employee does the job.
- Some payments do not count towards NMW pay. These should be deducted from the total pay before working out whether the employee is receiving the NMW. Payments which do not count include:
 - Loans;
 - Advances of wages;
 - Pension payments;
 - Retirement lump sums;
 - Redundancy payments;
 - Rewards under staff suggestion schemes;



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- Any premium element (in other words, anything the employee is paid on top of basic pay) for working at special times, e.g. overtime or on bank holidays;
- Allowances on top of basic pay, for example for working unsocial hours, in a particular area (e.g. London weighting), in dangerous conditions, 'on call', or performing special duties over and above normal one;
- Tips, gratuities, service charges and cover charges (until 1 October 2009 these did count towards NMW pay as long as the employer paid them through the employee's payroll, however they no longer count, regardless of how the employee receives them).

Deductions from the employee's pay and payments to the employer

- Some deductions and payments reduce NMW pay while others do not. However, the employer cannot make any deductions from the employee's pay, or take payments from the employee, unless certain conditions are met. The Wage Deductions Agreement in the Recruitment Pack C – Induction and Job Start provides the basis for most deductions, if signed by the employee.

Deductions and payments which do not reduce NMW pay

- Some deductions from pay, or payments the employee make to the employer, are ignored in working out NMW pay. Therefore, if the employee is paid £7.00 an hour and deductions or payments for any of the reasons listed below reduce this to £6.00, the question is whether £7 an hour, not £6, meets the NMW in the employee's case.
- Deductions and payments which do not reduce NMW pay are:
 - Penalties for misconduct – as long as these penalties are in the employment contract;
 - Paying back an advance of wages;
 - Paying back an accidental overpayment of wages;
 - The costs of any shares or securities which employees have chosen to buy in the firm;
 - Money the employee chooses to have deducted from pay (e.g. a pension contribution or trade union subscription), as long as the deduction is not required as part of the employee's work and is not for the employer's own use and benefit;
 - Payments the employee chooses to make to the employer to buy goods or services from them, for example, if the employee spends some of their wages on meals in the staff canteen.

Deductions and payments which reduce NMW pay

- Some deductions from pay, or payments made to the employer, are taken into account in working out NMW pay. Therefore, if the employee is paid £7 an hour and deductions or payments for any of the reasons listed below reduce this to £5, the question is whether £5 an hour, not £7, meets the NMW in that case.
- Deductions and payments which reduce NMW pay are:
 - Refunds of any money that the employee spent in connection with the work, for example the cost of purchasing tools or uniform;
 - Refunds of any expenses incurred doing the employee's job, for example travel costs;
 - Deductions to cover the cost of items the employer supplied to employees that are needed for the job, for example, tools or uniform;
 - Deductions for the employer's own use and benefit for goods and services, for example transport provided to and from work, regardless of whether the employee has the option of using the goods or services.
- The employer does not have to be making a profit from a deduction for it to be 'for their own use and benefit'. For example, if the employer provides transport at a loss, any deductions the employer makes for providing it help to reduce their loss. The reduction is part of the 'use and benefit' they will have had from the deductions.

Do 'benefits in kind' count towards the NMW?



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- Benefits in kind are anything the employer provides for employees' benefit apart from pay. There is nothing to stop employers offering employees benefits in kind. However, an equivalent value in money for a benefit in kind cannot be counted towards NMW pay, except for a set amount for accommodation.
- Examples of benefits in kind which do not count towards NMW pay are:
 - Meals;
 - Fuel;
 - A car;
 - The employer's contribution to the employee's pension fund;
 - Help with removals;
 - Medical insurance;
 - Lunch vouchers;
 - Child care vouchers.

Current Rates

- Please note that rates change on an annual basis, usually every October. (See <http://www.hmrc.gov.uk/payee/rates-thresholds.htm#2> for confirmation of the most recent figures).
- From 1st October 2025 the rates will be:
 - For workers 21 and over - **£6.70** per hour (previously £6.50).
 - For workers from 18 to 20 - **£5.30** per hour (previously £5.13).
 - For workers from 16 to 17 - **£3.87** per hour (previously £3.79).
 - For apprentices under 19 or 19 and over who are in the first year of apprenticeship - **£3.30** per hour (previously £2.73).

Living Accommodation

- The amount that can be taken into account for living accommodation is **£5.35** per day from 1st October 2025 (previously £5.08 per day).

On-Call

- Being on-call at the business premises is 'work', and is subject to the minimum wage regulations and working time regulations.
- Time on-call away from the business premises, where the worker is free to pursue other activities, is not 'work', and not subject to the regulations.

Volunteers

- The minimum wage does not apply to volunteers who:
 - Receive only reimbursement for genuine or reasonable out-of-pocket expenses, and;
 - Receive no benefits other than training necessary for them to carry out the work.
- It also does not apply to volunteers who:
 - Receive accommodation and/or meals because it is necessary in order for them to carry out their work, or;
 - Receive a payment towards subsistence. However, this only stands if (a) the volunteer is working for a charity, voluntary organisation, associated fundraising body or statutory body, and (b) the volunteer has been placed with that body by a charity acting in pursuance of its charitable objects, or;



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- Are residential members of a charitable religious community (provided the community is not an independent school and does not provide further or higher education courses).
- The minimum wage is likely to apply to volunteers who receive payment above genuine reimbursement. This could include, for example, lump sum expenses, sessional fees, one-off payments, 'honoraria' etc. It is also likely to apply where volunteers receive training beyond what is needed for them to do their work, or where they receive other benefits.

National Living Wage

- This is not to be confused with the National Minimum Wage, but is a minimum "living wage" to be paid to workers aged 25 years or over with effect from 1st April 2016. The rate is £7.20 per hour.

Information given here is guidance only and cannot be taken as an authoritative interpretation of the law. Specific circumstances can vary; the law is complex and is continually changing.

Reviewed Marcus Kerridge-McColl

December 2025

Missing Service User Policy & Procedure
Crescent community Care Services Limited

Purpose

1. To comply with regulations and statutes and quality standards
2. Crescent Community Care Services Limited will respond in an effective and structured manner to missing Service users

Scope

1. All Employees
2. All service users and families and other professionals involved in the service

Policy

1. In the event that a service user is deemed missing as they are not answering the door, the telephone and no family member or neighbour has no good understanding or reason for them not to be answering or if there is an expression of concern for the wellbeing of a service user. The following procedure will take effect immediately
2. All service users have the choice to leave their home whenever they wish, however it is necessary that members of staff respond to concerns about the whereabouts of a Service User in the event that contact can not be made
3. All staff should be aware that NO service user can be held at their own premises against their will unless they are subject to a Mental Health Act or a DOLS (Deprivation of Liberty) restraint order. However Safety and Risk management will always be our primary concern
4. In the event that a Service User is deemed missing the following procedure must be adhered to and that all staff recognise their own responsibilities for the safety and security of all service users in their care.
5. Should a Service User wish to leave their home and the care plan states that there is either a Mental Health reason or Deprivation of Liberty in place then intervention must take place either by management, professionals and / or family members

Procedure

1. If the care worker should find that a service user is not where they are expected to be at the visit or that entry to a property is not available then they should contact the office or the On Call immediately and wait for further instruction
2. The office and On Call will ask the care worker to try neighbours and look through windows and doors to ascertain that the service user has not fallen or is unwell and unable to answer the door
3. The office or On Call will try telephoning the service user and then the Next of Kin to ascertain if there is a reason that entry can not be made
4. Other professionals and hospitals will be called to ascertain if the service user has been either admitted to hospital or to another site due to illness or concern



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5. Social Services will be informed and worked with to ensure that all parties are aware of the situation and the actions taken
6. In the event that there is no clue as to the whereabouts of the service user the Police may be called to gain entry to the service user's home to ensure that they are not incapacitated for reasons of illness etc
7. In the event that the service user is still not found then the Police will take over with a missing persons investigation
8. All actions must be recorded so that the information can be passed over to other professionals to assist the finding of the service user
9. CQC have to be notified under a statutory notice
10. When the service user has been found a professionals meeting must be called to ensure that this does not happen again and if there requires intervention

Reviewed Marcus Kerridge-McColl
October 2025

Moonlighting Policy and Procedure
Crescent Community Care Services Limited

Purpose

1. To control potential conflicts of interest from moonlighting, and to maintain the health and safety of employees.

Scope

2. All employees.

Policy

3. The organisation recognises that from time-to-time employees may take up separate employment with another employer or pursue outside business interests whilst still remaining in the organisation's employ.
4. Although the organisation has no desire to unreasonably restrict an employee's external activities, it must seek to protect its own interests and those of all its employees.
5. To this end, the organisation's policy is that employees will not be permitted to undertake business activities or other work where the organisation considers that this is incompatible with its interests and, in any event, unless employees have obtained the prior written authorisation of the department manager.
6. Private work for Service Users:
 1. Private work for Service Users is strongly discouraged, as it conflicts with our contract of employment, and will very probably be regarded as financial abuse. In exceptional circumstances the organisation may authorise such work where it is clearly for the benefit of the Service User, subject to the approval of the organisation funding the support to that individual Service User, and subject to controls on charging and quality.

Procedure

2. When an employee proposes taking up additional employment or external business interests, the employee:
 1. Must request an interview with the department manager with a view to establishing the likely impact of these activities on the organisation;
 2. The employee will be asked to provide full details of the proposed work and specific consideration should be given to the following areas:
3. Working hours: is the employee proposing to conduct their affairs entirely outside their contractual hours of work, or is there likely to be some overlap?



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4. Competition: is the employee intending to work in competition with the organisation, either in their own right or for a competing organisation? If so, is there a real risk of a conflict of interest and/or confidential information being used to the organisation's detriment?
5. Health, safety and welfare: is the employee proposing to carry out work which is inherently hazardous and where the risk of injury is high (should the employee become injured or fall sick as a result of other work, organisation sick-pay entitlement will be affected) and is the extra work likely to cause undue fatigue, stress, etc. which will affect job performance with the organisation?
1. If, after investigation, the department manager considers that the proposed activities are incompatible with the individual's obligations to the organisation, permission will be refused.
2. This should be notified to the employee in writing detailing the reasons for refusal.
3. Employees should be advised that if they are dissatisfied with the decision they should make use of the organisation's grievance procedure.
4. Where employees' proposed work does not significantly affect their employment with the organisation, permission should be given in writing to the employee.
5. This, though, should include provisions reminding employees of their obligations to the organisation and stating that should their external activities subsequently have an effect on their employment, permission may be withdrawn.
6. Additionally, employees should be advised that any material changes to the circumstances of their outside interests must be brought to the attention of the organisation.

Reviewed by Marcus Kerridge McColl
December 2025

Moving & Handling Policy & Procedure
Crescent Community Care Services limited

Purpose

- To ensure that a standardised, practical and knowledge-based approach is taken by workers ("workers" means anyone working for the employer, including employees, agency staff and contractors) on moving and handling in line with current legislation and company standards. This will facilitate a safe system of work.
- This policy has been produced in line with requirements specified in the Manual Handling Regulations 1992 (revised in 2004) and The Guide to the Handling of People (current revised 6th edition).

Scope

- All workers ("workers" means anyone working for the employer, including employees, agency staff and contractors).

Policy

- There are many situations in which staff may be required to move or handle Service Users or loads during their working day. During this activity there is potential for injury for both the worker and the Service User.
- The Moving and Handling Operations Regulations 1992 (amended in 2004) require employers to:
 - AVOID the need for their workers to carry out moving and handling operations whenever possible.
 - ASSESS if moving and handling is unavoidable a **SUITABLE AND SUFFICIENT** assessment must be undertaken.
 - REDUCE risk to workers and Service Users, as far as is reasonably practicable, to the lowest possible level.
 - REVIEW assessments every time the situation/condition of the Service User changes.

- Initial appraisals of risk and risk assessments will be carried out by qualified Senior Carers for each individual in order to ensure that workplace assessments are undertaken before service delivery.
- Crescent Community Care Services Ltd is required to nominate an appropriate number of risk assessors, who will be trained to carry out risk assessments within the workplace or a Service User's home environment as part of the Care Plan.
- "The Guide to the Handling of People", 6th Edition, published by BackCare in collaboration with the Royal College of Nursing, will be integral to the training process and should only be used in conjunction with the approved basic moving and handling training.
- An initial appraisal of risk must be undertaken and reported to the appropriate person.
- A Risk Assessment must be carried out **on any** moving and handling task whereby a risk has been identified. This risk must be eliminated, or if that is not possible, reduced to the lowest level as is reasonably practicable.
- Crescent Community Care Services Ltd's Moving & Handling risk assessment form will be the only outcome-accepted evaluation of level of risk.
- Moving and handling of Service Users must be carried out as part of the Care planning process, using the documentation contained in the QCS Care Plan format. Where Service Users lack mental capacity to consent to this, decisions must be made in the person's best interest.
- Where workers may move or handle Service Users, those workers must have initial training which has a theoretical and practical assessment of competence. This training is to be delivered by a Moving and Handling trainer. Staff must also have an annual assessment of their competence conducted by a Moving and Handling trainer, and be given remedial training and be reassessed if any assessment indicates that this is required.
- Staff should be aware of mental capacity legislation in relation to decisions made in a person's interest, including where those decisions may involve restraint. Where the level of restraint necessary constitutes a deprivation of someone's liberty, this would be contrary to Article 5 of the Human Rights Act, unless the deprivation was lawfully authorised by the Court of Protection.
- This policy applies to all, inclusive of employees, volunteers, agency workers, students, and relief staff and should be implemented by Line Managers, both during induction and at regular intervals.
- Special consideration should be given to vulnerable workers:
 - For pregnant staff, a specific risk assessment should be completed, this should include a re-assessment upon return to work following maternity leave.
 - Any other workers with an identified medical condition, should have a specific risk assessment supported in writing by the organisation's occupational health provider and signed off by the Registered Manager.
- Induction will include moving and handling instruction, practice and assessment as an essential component before any moving and handling task is undertaken in the workplace, and this will be subject to regular review.

Procedure

- It is recognised that Managers require a broad understanding and working knowledge of risk assessments and moving and handling principles and they should establish working practices in consultation with the Health & Safety Adviser.
- It is the responsibility of the organisation, delegated to the Registered Manager, to identify risks and subsequently manage them. Managers must not rely solely on risk analysis forms, which may not recognise risks particular to one situation or location.
- The Risk Assessment should be a collaborative process between all parties involved in any daily working task. In any moving and handling task - inclusive of people moving - it is recognised that the worker requires adequate training and the Assessors require detailed knowledge of the following:

- **L.I.T.E.E;**

7. Load - Typically the Service User.

8. Individual Capability - Are they suitably trained?

9. Task - What is the activity?

10.Environment - Where will the activity take place?

11.Equipment - What will be required to reduce the risks?

- Final decisions on the risk assessment outcome will be made by the respective Manager in conjunction with the Health & Safety requirements.
- The employer may be liable if it can be proved that a risk assessment of the task or the standard and regularity of training has not been upheld, in order for the worker to carry out their duties related to moving and handling of inanimate loads and Service Users (i.e. litigation from injured workers).
- The worker could also be liable if found to be negligent in the undertaking of their duties, i.e. failure to observe Lifting Operations and Lifting Equipment Regulations 1998 (LOLER) which requires the carer to check that a hoist has been serviced within the last six months and upon visual inspection is safe to use prior to using it. The employer has vicarious liability for actions taken by their workers on their behalf i.e. risk assessments.
- It should be remembered that Service User Care is paramount and the resulting impact of the risk assessment must be sensitively managed by all involved.
- Where there are disputes or conflicts between risk assessors, agencies, Service Users, professionals and any others, the senior manager (in consultation with the HSA) would make a decision in line with the respective current practice and legislation.
- The policy will be reviewed, expanded or modified periodically, in accordance with changing conditions or legislation, on an annual basis.
- All people involved in Moving & Handling tasks as part of their daily work must be appropriately clothed in either uniform (if provided) or clothing which facilitates the principles of safe handling. Refer to the following policy and procedure – Employee Appearance Policy and Procedure, in particular:
 - Care staff will wear the appropriate organisation uniform, not wear jewellery of any kind, have long hair tied back, and wear appropriate footwear.
 - Staff with Service User contact:
 - 12. Staff members are expected to wear the uniform appropriate to their role; where a uniform is not specified casual clothing is acceptable, but it must be neat and clean and appropriate to the work which is undertaken.
 - Uniform clothing must be clean, odour-free, in good repair and well pressed.
 - Hair must be kept clean and tidy.
 - Staff must keep their hair secure in such a way that it does not pose a safety hazard.
 - Jewellery, including ear-rings and studs which may come into contact with a Service User should not be worn.
 - Nails must be clean and short, and not pose a scratching risk to Service Users, especially those with frail skin.
 - Footwear must be appropriate to the situation, providing protection from dropped objects, slip protection, not be a scratch or other danger to Service Users. Open toe or sling back shoes are not acceptable.
- The results of the risk assessment must be adhered to, except in instances whereby any of the following emergency situations apply:
 - A person is in imminent danger of drowning;
 - A person is in an area that is on fire or filling with smoke;
 - A person is in danger of a life threatening situation;

- A person is in danger of a collapsing building or other structure.
- In any other emergency situation time must be taken to assess the situation, plan the manoeuvre and obtain equipment if necessary.
- Prospective workers will be made aware that care and consideration must be given to the tasks to be undertaken, and they must be suitable for the chosen place of work, i.e. Service User bedroom or bathroom.
- The risk assessments should be reviewed by senior managers if tasks alter at any time. Modification of any job description must also be undertaken only with the express permission of senior management.
- After completing a written risk assessment the assessor must make relevant workers and other relevant staff aware of its content and any identified remedial actions should be dealt with, e.g. obtaining equipment, identifying additional training, etc. Staff should follow the risk assessment guidance and instructions given; failure to do so could result in disciplinary action.
- Managers and all users must be conversant with safe use of moving and handling equipment and must ensure that maintenance and use of equipment complies with the Provision and Use of Work Equipment Regulations 1992 (PUWER as amended in 2002), the manufacturer's instructions, and safety precautions. Selection of equipment must be undertaken by a qualified Occupational Therapist (OT) or Physiotherapist.
- All privately purchased equipment must be accompanied by the supplier or manufacturers documentation on safe use, and all relevant workers must receive training on the safe use of the equipment before it is first used.
- All equipment used in moving and handling must be stored in a safe manner and must not pose a hazard in itself and be protected against malicious damage.
- Accidents to staff during moving and handling must be reported and documented as per Crescent Community Care Services Ltd accident reporting and investigation procedure. Those accidents, which cause a worker to be incapacitated for over 7 days not including the day on which the accident happened, are reported under RIDDOR. Remedial action should be taken to limit a recurrence following investigation by senior managers.
- The employer is insured for personal risks undertaken by the worker as described in their job description.
- Safety disclaimers are not legally binding and cannot be upheld by courts.
- The successful implementation of this policy requires total commitment from all employees. Each individual has a legal obligation to take reasonable care of their own health and safety, and for the safety of other people who may be affected by their actions or omissions. No carer should manually handle people or objects until they have completed training and been assessed as competent to do so.

Risk Assessments - General

- A risk assessment format is supplied as part of this policy. Users must note that it is the responsibility of the organisation, delegated to the departmental manager, to identify risks and subsequently manage them. Managers must not rely solely on risk assessment forms, which may not recognise risks particular to one situation or location.
- For general guidance on moving and handling, and charts to assess some generic risks, see the Health and Safety Executive publication available at: <http://www.hse.gov.uk/pubns/indg383.pdf>

Risk Assessments - Lifts and Hoists

- Where risks associated with lifts and hoists are to be undertaken, recognition of environmental factors is important. The Royal Institute of British Architects publish guidance on room design relevant to safe manual handling. These recommendations are usually incorporated within the lift or hoist manufacturers' safe usage guidelines. The guidelines may preclude some rooms for use with lifts and hoists due to lack of room for safe handling.

Other Relevant Documents:

- Crescent Community Care Services Ltd Health & Safety Policy and Procedure.
- Manual Handling Operations Regulations 1992 revised 1998 and updated in 2004 (MHOR).
- Management of Health and Safety at Work Regulations 1999
- The Guide to the Handling of People", 6th Edition, published by BackCare (current revised 6th edition 2011).



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- HSE A Brief Guide to Manual Handling at Work, <http://www.backcare.org.uk/wp-content/uploads/2025/02/HSE-Manual-Handling-at-Work.pdf>.
- Work Equipment Regulations 1992.
- RIDDOR - Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013.
- LOLER - Lifting Operations and Lifting Equipment Regulations 1998, as amended in 2002.
- PUWER - Provision and Use of Work Equipment Regulations 1998, as amended in 2002.
- Human Rights Act 1998.
- Health & Safety at Work Act 1974.
- The Mental Capacity Act 2005 incorporating the Deprivation of Liberty Safeguards.

Reviewed Marcus Kerridge-McColl
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